

G.E.H.A Indemnity Benefit Plan

Government Employees Health Association

geha.com/Get-Support

Customer Care: 800-821-6136



2026

A Fee-for-Service Plan (Elevate Plus and Elevate Options) with a Preferred Provider Organization

This plan's health coverage qualifies as minimum essential coverage and meets the minimum value standard for the benefits it provides. See page 7 for details. This plan is accredited. See page 12.

Sponsored and administered by:
Government Employees Health Association, Inc.

Who may enroll in this Plan: All Federal Employees and Annuitants who are eligible to enroll in the Federal Employees Health Benefits (FEHB) Program may become members of Government Employees Health Association, Inc. (G.E.H.A). You must be, or must become a member of G.E.H.A (herein referred to as "Us", "We", or "Our").

To become a member: You join simply by signing a completed Standard Form 2809, Health Benefits Registration Form, evidencing your enrollment in the Plan.

Membership dues: There are no membership dues for the Year 2026.

Postal Employees and Annuitants are no longer eligible for this plan, unless currently under Temporary Continuation of Coverage.

Enrollment codes for these Plans:

- 251 Elevate Plus Option - Self Only
- 253 Elevate Plus Option - Self Plus One
- 252 Elevate Plus Option - Self and Family
- 254 Elevate Option - Self Only
- 256 Elevate Option - Self Plus One
- 255 Elevate Option - Self and Family

IMPORTANT

- Rates: Back Cover
- Changes for 2026: Page 14
- Summary of Benefits: Page 123



Authorized for distribution by the:



United States
Office of Personnel Management

Healthcare and Insurance
<http://www.opm.gov/insure>

Important Notice

Important Notice from Government Employees Health Association, Inc.

About Our Prescription Drug Coverage and Medicare

The Office of Personnel Management (OPM) has determined that the Government Employees Health Association, Inc. prescription drug coverage is, on average, expected to pay out as much as the standard Medicare prescription drug coverage will pay for all plan participants and is considered Creditable Coverage. This means you do not need to enroll in Medicare Part D and pay extra for prescription drug coverage. If you decide to enroll in Medicare Part D later, you will not have to pay a penalty for late enrollment as long as you keep your FEHB coverage.

However, if you choose to enroll in Medicare Part D, you can keep your FEHB coverage, and your FEHB plan will coordinate benefits with Medicare.

Remember: If you are an annuitant and you cancel your FEHB coverage, you may not re-enroll in the FEHB Program.

Please be advised

If you lose or drop your FEHB coverage and go 63 days or longer without prescription drug coverage that is at least as good as Medicare's prescription drug coverage, your monthly Medicare Part D premium will go up at least 1% per month for every month that you did not have that coverage. For example, if you go 19 months without Medicare Part D prescription drug coverage, your premium will always be at least 19% higher than what many other people pay. You will have to pay this higher premium as long as you have Medicare prescription drug coverage. In addition, you may have to wait until the next Annual Coordinated Election Period (October 15 through December 7) to enroll in Medicare Part D.

Medicare's Low-Income Benefits

For people with limited income and resources, extra help paying for a Medicare prescription drug plan is available. Information regarding this program is available through the Social Security Administration (SSA) online at www.socialsecurity.gov, or call the SSA at 800-772-1213, TTY: 800-325-0778.

Additional Premium for Medicare's High-Income Members

Income-Related Monthly Adjustment Amount (IRMAA)

The Medicare Income-Related Monthly Adjustment Amount (IRMAA) is an amount you may pay in addition to your FEHB premium to enroll in and maintain Medicare prescription drug coverage. **This additional premium is assessed only to those with higher incomes, based on the income reported on your IRS tax return.** You do not make any IRMAA payments to your FEHB plan. Refer to the Part D-IRMAA section of the Medicare website: <https://www.medicare.gov/drug-coverage-part-d/costs-for-medicare-drug-coverage/monthly-premium-for-drug-plans> to see if you would be subject to this additional premium.

You can get more information about Medicare prescription drug plans and the coverage offered in your area from these places:

- Visit www.medicare.gov for personalized help.
- Call Medicare at 800-633-4227, TTY 877-486-2048.

Table of Contents

Table of Contents	1
Introduction	3
Plain Language.....	3
Stop Healthcare Fraud!	3
Discrimination is Against the Law	4
Preventing Medical Mistakes.....	5
FEHB Facts	7
Coverage information	7
• No pre-existing condition limitation.....	7
• Minimum essential coverage (MEC).....	7
• Minimum value standard (MVS).....	7
• Where you can get information about enrolling in the FEHB Program	7
• Enrollment types available for you and your family	7
• Family member coverage	8
• Children’s Equity Act	9
• When benefits and premiums start	10
• When you retire	10
When you lose benefits	10
• When FEHB coverage ends.....	10
• Upon divorce	10
• Temporary Continuation of Coverage (TCC).....	11
• Converting to individual coverage.....	11
• Health Insurance Marketplace	11
Section 1. How This Plan Works	12
• General features of our Elevate Plus and Elevate Options	12
• How we pay providers	12
• Health education resources	13
• Your rights and responsibilities	13
• Your medical and claims records are confidential	13
Section 2. Changes for 2026	14
Section 3. How You Get Care	15
Identification cards.....	15
Where you get covered care.....	15
Balance Billing Protection	15
• Covered providers.....	15
• Covered facilities	16
• Transitional care	17
• If you are hospitalized when your enrollment begins.....	18
You need prior Plan approval for certain services	18
• Inpatient facility admission	18
• Other services that require preauthorization.....	20
• Non-urgent care claims.....	21
• Urgent care claims	21
• Concurrent care claims	21
• Emergency inpatient admission.....	22
• Maternity care	22
• If your facility stay needs to be extended	22

• If your treatment needs to be extended.....	22
If you disagree with our pre-service claim decision	22
• To reconsider a non-urgent care claim.....	22
• To reconsider an urgent care claim	22
• To file an appeal with OPM.....	23
Overseas claims.....	23
Section 4. Your Costs for Covered Services	24
Cost-sharing	24
Copayments.....	24
Deductible	24
Coinsurance.....	25
If your provider routinely waives your cost.....	25
Waivers.....	25
Differences between our allowance and the bill	25
Your catastrophic protection out-of-pocket maximum	26
Carryover	27
If we overpay you	27
When Government facilities bill us	27
Important Notice About Surprise Billing - Know Your Rights.....	27
The Federal Flexible Spending Account Program - FSAFEDS.....	28
Section 5. Elevate Plus and Elevate Benefits.....	29
Elevate Plus and Elevate Overview	31
Non-FEHB Benefits Available to Plan Members.....	95
Section 6. General Exclusions - Services, Drugs and Supplies We Do Not Cover	96
Section 7. Filing a Claim for Covered Services.....	97
Section 8. The Disputed Claims Process.....	100
Section 9. Coordinating Benefits with Medicare and Other Coverage.....	103
When you have other health coverage or auto insurance.....	103
• TRICARE and CHAMPVA.....	103
• Workers' Compensation.....	104
• Medicaid	104
When other Government agencies are responsible for your care	104
When others are responsible for injuries.....	104
When you have Federal Employees Dental and Vision Insurance Plan (FEDVIP).....	105
Clinical trials	106
When you have Medicare	106
• The Original Medicare Plan (Part A or Part B).....	106
• Tell us about your Medicare coverage.....	108
• Private contract with your physician	108
• Medicare Advantage (Part C)	108
• Medicare prescription drug coverage (Part D)	109
When you have the Original Medicare Plan (Part A, Part B, or both).....	112
Section 10. Definitions of Terms We Use in This Brochure	113
Index.....	122
Summary of Benefits for the Elevate Plus Option of the Government Employees Health Association, Inc. 2026	123
Summary of Benefits for the Elevate Option of the Government Employees Health Association, Inc. 2026	125
2026 Rate Information for Government Employees Health Association, Inc. Benefit Plan	130

Introduction

This brochure describes the benefits of the Indemnity Benefit Plan under contract (CS 2962) between Government Employees Health Association, Inc. (G.E.H.A) and the United States Office of Personnel Management, as authorized by the Federal Employees Health Benefits (FEHB) law here in referred to as the "Plan." This Plan is underwritten by Government Employees Health Association, Inc. (G.E.H.A, Inc.) and Surety Life Insurance Company and administered by G.E.H.A, Inc. The Carrier of the Plan is a voluntary association comprised of G.E.H.A, Inc. and Surety Life Insurance Company. Customer Care may be reached at 800-821-6136 or through our website: geha.com/Get-Support. The address for the Government Employees Health Association, Inc. administrative offices is:

Government Employees Health Association, Inc.
310 NE Mulberry St.
Lee's Summit, MO 64086

This brochure is the official statement of benefits. No verbal statement can modify or otherwise affect the benefits, limitations, and exclusions of this brochure. It is your responsibility to be informed about your health benefits.

If you are enrolled in this Plan, you are entitled to the benefits described in this brochure. If you are enrolled in Self Plus One or Self and Family coverage, each eligible family member is also entitled to these benefits. You do not have a right to benefits that were available before January 1, 2026, unless those benefits are also shown in this brochure.

OPM negotiates benefits and rates for each plan annually. Benefit changes are effective January 1, 2026, and changes are summarized in Section 2. Rates are shown at the end of this brochure.

Plain Language

All FEHB brochures are written in plain language to make them easy to understand. Here are some examples:

- Except for necessary technical terms, we use common words. For instance, “you” means the enrollee and each covered family member; “we” means Government Employees Health Association, Inc.
- We limit acronyms to ones you know. FEHB is the Federal Employees Health Benefits Program. OPM is the United States Office of Personnel Management. If we use others, we tell you what they mean.
- Our brochure and other FEHB plans’ brochures have the same format and similar descriptions to help you compare plans.

Stop Healthcare Fraud!

Fraud increases the cost of healthcare for everyone and increases your Federal Employees Health Benefits Program premium.

OPM’s Office of the Inspector General investigates all allegations of fraud, waste, and abuse in the FEHB Program regardless of the agency that employs you or from which you retired.

Protect Yourself From Fraud – Here are some things that you can do to prevent fraud:

- Do not give your plan identification (ID) number over the phone or to people you do not know, except for your healthcare provider, authorized health benefits plan, or OPM representative.
- Let only the appropriate medical professionals review your medical record or recommend services.
- Avoid using healthcare providers who say that an item or service is not usually covered, but they know how to bill us to get it paid.
- Carefully review Explanation of Benefits (EOBs) statements that you receive from us.
- Periodically review your claim history for accuracy to ensure we have not been billed for services you did not receive.
- Do not ask your doctor to make false entries on certificates, bills, or records in order to get us to pay for an item or service.

- If you suspect that a provider has charged you for services you did not receive, billed you twice for the same service, or misrepresented any information, do the following:
 - Call the provider and ask for an explanation. There may be an error.
 - If the provider does not resolve the matter, call us at 800-356-5803 and explain the situation.
 - If we do not resolve the issue:

CALL THE HEALTHCARE FRAUD HOTLINE

877-499-7295

OR go to www.opm.gov/our-inspector-general/hotline-to-report-fraud-waste-or-abuse/complaint-form

The online reporting form is the desired method of reporting fraud in order to ensure accuracy, and a quicker response time.

You can also write to:

**United States Office of Personnel Management
Office of the Inspector General Fraud Hotline
1900 E Street NW Room 6400
Washington, DC 20415-1100**

- Do not maintain family members on your policy:
 - Your former spouse after a divorce decree or annulment is final (even if a court order stipulates otherwise);
 - Your child over age 26 (unless they were disabled and incapable of self-support prior to age 26).

A carrier may request that an enrollee verify the eligibility of any or all family members listed as covered under the enrollee's FEHB enrollment.

- If you have any questions about the eligibility of a family member, check with your personnel office if you are employed, with your retirement office (such as OPM) if you are retired, or with the National Finance Center if you are enrolled under Temporary Continuation of Coverage (TCC).
- Fraud or intentional misrepresentation of material fact is prohibited under the Plan. You can be prosecuted for fraud, and your agency may take action against you. Examples of fraud include falsifying a claim to obtain FEHB benefits, trying to or obtaining service or coverage for yourself or for someone else who is not eligible for coverage, or enrolling in the Plan when you are no longer eligible.
- If your enrollment continues after you are no longer eligible for coverage (i.e., you have separated from Federal service) and premiums are not paid, you will be responsible for all benefits paid during the period in which premiums were not paid. You may be billed by your provider for services received. You may be prosecuted for fraud for knowingly using health insurance benefits for which you have not paid premiums. It is your responsibility to know when you or a family member is no longer eligible to use your health insurance coverage.

Discrimination is Against the Law

We comply with applicable Federal nondiscrimination laws and do not discriminate on the basis of race, color, national origin, age, disability, religion, sex, pregnancy, or genetic information. We do not exclude people or treat them differently because of race, color, national origin, age, disability, religion, sex, pregnancy, or genetic information.

The health benefits described in this brochure are consistent with applicable laws prohibiting discrimination. All coverage decisions will be based on nondiscriminatory standards and criteria. An individual's protected trait or traits will not be used to deny health benefits for items, supplies, or services that are otherwise covered and determined to be medically necessary.

Preventing Medical Mistakes

Medical mistakes continue to be a significant cause of preventable deaths within the United States. While death is the most tragic outcome, medical mistakes cause other problems such as permanent disabilities, extended hospital stays, longer recoveries, and even additional treatments. Medical mistakes and their consequences also add significantly to the overall cost of healthcare. Hospitals and healthcare providers are being held accountable for the quality of care and reduction in medical mistakes by their accrediting bodies. You can also improve the quality and safety of your own healthcare and that of your family members by learning more about and understanding your risks. Take these simple steps:

1. Ask questions if you have doubts or concerns.

- Ask questions and make sure you understand the answers.
- Choose a doctor with whom you feel comfortable talking.
- Take a relative or friend with you to help you take notes, ask questions, and understand answers.

2. Keep and bring a list of all the medications you take.

- Bring the actual medication or give your doctor and pharmacist a list of all the medications and dosages that you take, including non-prescription (over the counter) medications and nutritional supplements.
- Tell your doctor and pharmacist about any drug, food, and other allergies you have, such as latex.
- Ask about any risks or side effects of the medication and what to avoid while taking it. Be sure to write down what your doctor or pharmacist says.
- Make sure your medication is what the doctor ordered. Ask the pharmacist about your medication if it looks different than you expected.
- Read the label and patient package insert when you get your medication, including all warnings and instructions.
- Know how to use your medication. Especially note the times and conditions when your medication should and should not be taken.
- Contact your doctor or pharmacist if you have any questions.
- Understanding both the generic and brand names of all of your medication(s) is important. This helps ensure you do not receive double dosing from taking both a generic and a brand of the same medication. It also helps you avoid taking a medication to which you are allergic

3. Get the results of any test or procedure.

- Ask when and how you will get the results of tests or procedures. Will it be in person, by phone, mail, through the Plan or Provider's portal?
- Don't assume the results are fine if you do not get them when expected. Contact your healthcare provider and ask for your results.
- Ask what the results mean for your care.

4. Talk to your doctor about which hospital or clinic is best for your health needs.

- Ask your doctor about which hospital or clinic has the best care and results for your condition if you have more than one hospital or clinic to choose from to get the healthcare you need.
- Be sure you understand the instructions you get about follow-up care when you leave the hospital or clinic.

5. Make sure you understand what will happen if you need surgery.

- Make sure you, your doctor, and your surgeon all agree on exactly what will be done during the operation.
- Ask your doctor, "Who will manage my care when I am in the hospital?"
- Ask your surgeon:

- “Exactly what will you be doing?”
 - “About how long it will take?”
 - “What will happen after surgery?”
 - “How can I expect to feel during recovery?”
- Tell the surgeon, anesthesiologist, and nurses about any allergies, bad reactions to anesthesia, and any medications or nutritional supplements you are taking.

Patient Safety Links

For more information on patient safety, please visit:

- www.jointcommission.org/speakup.aspx. The Joint Commission's Speak Up™ patient safety program.
- www.jointcommission.org/topics/patients_safety.aspx. The Joint Commission helps healthcare organizations to improve the quality and safety of the care they deliver.
- www.ahrq.gov/patients-consumers. The Agency for Healthcare Research and Quality makes available a wide-ranging list of topics not only to inform consumers about patient safety but to help choose quality healthcare providers and improve the quality of care you receive.
- <https://psnet.ahrq.gov/issue/national-patient-safety-foundation>. The National Patient Safety Foundation has information on how to ensure safer healthcare for you and your family.
- www.bemedwise.org. The National Council on Patient Information and Education is dedicated to improving communication about the safe, appropriate use of medications.
- www.leapfroggroup.org. The Leapfrog Group is active in promoting safe practices in hospital care.
- www.ahqa.org. The American Health Quality Association represents organizations and healthcare professionals working to improve patient safety.

Preventable Healthcare Acquired Conditions (“Never Events”)

When you enter the hospital for treatment of one medical problem, you do not expect to leave with additional injuries, infections, or other serious conditions that occur during the course of your stay. Although some of these complications may not be avoidable, patients do suffer from injuries or illnesses that could have been prevented if doctors or the hospital had taken proper precautions. Errors in medical care that are clearly identifiable, preventable, and serious in their consequences for patients can indicate a significant problem in the safety and credibility of a healthcare facility. These conditions and errors are sometimes called “Never Events” or “Serious Reportable Events.”

We have a benefit payment policy that encourages hospitals to reduce the likelihood of hospital-acquired conditions such as certain infections, severe bedsores, and fractures, and to reduce medical errors that should never happen. When such an event occurs, neither you nor your FEHB plan will incur costs to correct the medical error.

You will not be billed for inpatient services related to treatment of specific hospital-acquired conditions or for inpatient services needed to correct “Never Events.” “Never Event” is defined by your claim's administrator using national standards. This policy helps to protect you from preventable medical errors and improve the quality of care you receive.

FEHB Facts

Coverage information

- **No pre-existing condition limitation**

We will not refuse to cover the treatment of a condition you had before you enrolled in this Plan solely because you had the condition before you enrolled.
- **Minimum essential coverage (MEC)**

Coverage under this plan qualifies as minimum essential coverage. Please visit the Internal Revenue Service (IRS) website at www.irs.gov/uac/Questions-and-Answers-on-the-Individual-Shared-Responsibility-Provision for more information on the individual requirement for MEC.
- **Minimum value standard (MVS)**

Our health coverage meets the minimum value standard of 60% established by the ACA. This means that we provide benefits to cover at least 60% of the total allowed costs of essential health benefits. The 60% standard is an actuarial value; your specific out-of-pocket costs are determined as explained in this brochure.
- **Where you can get information about enrolling in the FEHB Program**

See www.opm.gov/healthcare-insurance/healthcare for enrollment information as well as:

 - Information on the FEHB Program and plans available to you
 - A health plan comparison tool
 - A list of agencies that participate in Employee Express
 - A link to Employee Express, and
 - Information on and links to other electronic enrollment systems.

Also, your employing or retirement office can answer your questions, give you other plans' brochures and other materials you need to make an informed decision about your FEHB coverage. These materials tell you:

- When you may change your enrollment;
- How you can cover your family members;
- What happens when you transfer to another Federal agency, go on leave without pay, enter military service, or retire;
- What happens when your enrollment ends; and
- When the next Open Season for enrollment begins.

We do not determine who is eligible for coverage and, in most cases, cannot change your enrollment status without information from your employing or retirement office. For information on your premium deductions, you must also contact your employing or retirement office.

Once enrolled in your FEHB Program Plan, you should contact your carrier directly for updates and questions about your benefit coverage.

- **Enrollment types available for you and your family**

Self Only coverage is only for the enrollee. Self Plus One coverage is for the enrollee and one eligible family member. Self and Family coverage is for the enrollee and one or more eligible family member. Family members include your spouse and your children under age 26, including any foster children authorized for coverage by your employing agency or retirement office. Under certain circumstances, you may also continue coverage for a disabled child 26 years of age or older who is incapable of self-support.

If you have a Self Only enrollment, you may change to a Self Plus One or Self and Family enrollment if you marry, give birth, or add a child to your family. You may change your enrollment 31 days before to 60 days after that event. The Self Plus One or Self and Family enrollment begins on the first day of the pay period in which the child is born or becomes an eligible family member. When you change to Self Plus One or Self and Family because you marry, the change is effective on the first day of the pay period that begins after your employing office receives your enrollment form. Benefits will not be available to your spouse until you are married. A carrier may request that an enrollee verify the eligibility of any or all family members listed as covered under the enrollee's FEHB enrollment.

Contact your employing or retirement office if you want to change from Self Only to Self Plus One or Self and Family. If you have a Self and Family enrollment, you may contact us to add a family member.

Your employment or retirement office will **not** notify you when a family member is no longer eligible to receive benefits. Please tell us immediately of changes in family member status including your marriage, divorce, annulment, or when your child reaches age 26. We will send written notice to you 60 days before we proactively disenroll your child on midnight of their 26th birthday unless your child is eligible for continued coverage because they are incapable of self-support due to a physical or mental disability that began before age 26.

If you or one of your family members is enrolled in one FEHB plan, you or they cannot be enrolled in or covered as a family member by another enrollee in another FEHB plan.

If you have a qualifying life event (QLE) - such as marriage, divorce, or the birth of a child - outside of the Federal Benefits Open Season, you may be eligible to enroll in the FEHB Program, change your enrollment, or cancel coverage. For a complete list of QLEs, visit the FEHB website at www.opm.gov/healthcare-insurance/life-events. If you need assistance, please contact your employing agency, Tribal Benefits Officer, retirement office or personnel/payroll office.

- **Family member coverage**

Family members covered under your Self and Family enrollment are your spouse (including your spouse by a valid common-law marriage from a state that recognizes common-law marriage) and children as described below. A Self Plus One enrollment covers you and your spouse, or one other eligible family member as described below.

Natural children, adopted children, and stepchildren

Coverage: Natural children, adopted children, and stepchildren are covered until their 26th birthday.

Foster children

Coverage: Foster children are eligible for coverage until their 26th birthday if you provide documentation of your regular and substantial support of the child and sign a certification stating that your foster child meets all the requirements. Contact your human resources office or retirement system for additional information.

Children incapable of self-support

Coverage: Children who are incapable of self-support because of a mental or physical disability that began before age 26 are eligible to continue coverage. Contact your human resources office or retirement system for additional information.

Married children

Coverage: Married children (but NOT their spouse or their own children) are covered until their 26th birthday.

Children with or eligible for employer-provided health insurance

Coverage: Children who are eligible for or have their own employer-provided health insurance are covered until their 26th birthday.

Newborns of covered children are insured only for routine nursery care during the covered portion of the mother's maternity stay.

You can find additional information at www.opm.gov/healthcare-insurance.

• Children's Equity Act

OPM implements the Federal Employees Health Benefits Children's Equity Act of 2000. This law mandates that you be enrolled for Self Plus One or Self and Family coverage in the FEHB Program if you are an employee subject to a court or administrative order requiring you to provide health benefits for your child(ren).

If this law applies to you, you must enroll in Self Plus One or Self and Family coverage in a health plan that provides full benefits in the area where your children live or provide documentation to your employing office that you have obtained other health benefits coverage for your children.

If you do not do so, your employing office will enroll you involuntarily as follows:

- If you have no FEHB coverage, your employing office will enroll you for Self Plus One or Self and Family coverage, as appropriate in the lowest-cost nationwide plan option as determined by OPM;
- If you have a Self Only enrollment in a fee-for-service plan or in an HMO that serves the area where your children live, your employing office will change your enrollment to Self Plus One or Self and Family, as appropriate, in the same option of the same plan; or
- If you are enrolled in an HMO that does not serve the area where the children live, your employing office will change your enrollment to Self Plus One or Self and Family, as appropriate, in the lowest-cost nationwide plan option as determined by OPM.

As long as the court/administrative order is in effect, and you have at least one child identified in the order who is still eligible under the FEHB Program, you cannot cancel your enrollment, change to Self Only, or change to a plan that does not serve the area in which your children live, unless you provide documentation that you have other coverage for the children.

If the court/administrative order is still in effect when you retire, and you have at least one child still eligible for FEHB coverage, you must continue your FEHB coverage into retirement (if eligible) and cannot cancel your coverage, change to Self Only, or change to a plan that does not serve the area in which your children live as long as the court/administrative order is in effect. Similarly, you cannot change to Self Plus One if the court/administrative order identifies more than one child. Contact your employing office for further information.

- **When benefits and premiums start**

The benefits in this brochure are effective January 1. If you joined this Plan during Open Season, your coverage begins on the first day of your first pay period that starts on or after January 1. **If you changed plans or plan options during Open Season and you receive care between January 1 and the effective date of coverage under your new plan or option, your claims will be processed according to the 2026 benefits of your prior plan or option.** If you have met (or pay cost-sharing that results in your meeting) the out-of-pocket maximum under the prior plan or option, you will not pay cost-sharing for services covered between January 1 and the effective date of coverage under your new plan or option. **However**, if your prior plan left the FEHB Program at the end of 2025, you are covered under that plan's 2025 benefits until the effective date of your coverage with your new plan. Annuitants' coverage and premiums begin January 1. If you joined at any other time during the year, your employing office will tell you the effective date of coverage.

If your enrollment continues after you are no longer eligible for coverage (i.e., you have separated from Federal service) and premiums are not paid, you will be responsible for all benefits paid during the period in which premiums were not paid. You may be billed for services received directly from your provider. You may be prosecuted for fraud for knowingly using health insurance benefits for which you have not paid premiums. It is your responsibility to know when you or a family member are no longer eligible to use your health insurance coverage.

- **When you retire**

When you retire, you can usually stay in the FEHB Program. Generally, you must have been enrolled in the FEHB Program for the last five years of your Federal service. If you do not meet this requirement, you may be eligible for other forms of coverage, such as Temporary Continuation of Coverage (TCC).

When you lose benefits

- **When FEHB coverage ends**

You will receive an additional 31-days of coverage, for no additional premium, when:

- Your enrollment ends, unless you cancel your enrollment; or
- You are a family member no longer eligible for coverage.

Any person covered under the 31-day extension of coverage who is confined in a hospital or other institution for care or treatment on the 31st day of the temporary extension is entitled to continuation of the benefits of the Plan during the continuance of the confinement but not beyond the 60th day after the end of the 31-day temporary extension.

You may be eligible for spouse equity coverage or assistance with enrolling in a conversion policy (a non-FEHB individual policy).

- **Upon divorce**

If you are an enrollee and your divorce or annulment is final, your ex-spouse cannot remain covered as a family member under your Self Plus One or Self and Family enrollment. You must contact us to let us know the date of the divorce or annulment and have us remove your ex-spouse. We may ask for a copy of the divorce decree as proof. In order to change enrollment type, you must contact your employing or retirement office. A change will not automatically be made.

If you were married to an enrollee and your divorce or annulment is final, you may not remain covered as a family member under your former spouse's enrollment. This is the case even when the court has ordered your former spouse to provide health coverage for you. However, you may be eligible for your own FEHB coverage under either the spouse equity law or Temporary Continuation of Coverage (TCC). If you are recently divorced or are anticipating a divorce, contact your ex-spouse's employing or retirement office to get additional information about your coverage choices, www.opm.gov/healthcare-insurance/life-events/memy-family/im-separated-or-im-getting-divorced/#url=Health. We may request that you verify the eligibility of any or all family members listed as covered under the enrollee's FEHB enrollment.

- **Temporary Continuation of Coverage (TCC)**

If you leave Federal service, Tribal employment, or if you lose coverage because you no longer qualify as a family member, you may be eligible for Temporary Continuation of Coverage (TCC). For example, you can receive TCC if you are not able to continue your FEHB enrollment after you retire, if you lose your Federal or Tribal job, or if you are a covered child and you turn age 26, regardless of marital status, etc.

You may not elect TCC if you are fired from your Federal or Tribal job due to gross misconduct.

Enrolling in TCC. Get the RI 79-27, which describes TCC, from your employing or retirement office or from www.opm.gov/healthcare-insurance. It explains what you have to do to enroll.

Alternatively, you can buy coverage through the Health Insurance Marketplace where, depending on your income, you could be eligible for a tax credit that lowers your monthly premiums. Visit www.HealthCare.gov to compare plans and see what your premium, deductible, and out-of-pocket costs would be before you make a decision to enroll. Finally, if you qualify for coverage under another group health plan (such as your spouse's plan), you may be able to enroll in that plan, as long as you apply within 30 days of losing FEHB coverage.

- **Converting to individual coverage**

If you leave Federal or Tribal service, your employing office will notify you of your right to convert. You must contact us in writing within 31 days after you receive this notice. However, if you are a family member who is losing coverage, the employing or retirement office will not notify you. You must contact us in writing within 31 days after you are no longer eligible for coverage. Your benefits and rates will differ from those under the FEHB Program; however, you will not have to answer questions about your health, a waiting period will not be imposed, and your coverage will not be limited due to pre-existing conditions. When you contact us, we will assist you in obtaining information about health benefits coverage inside or outside the Affordable Care Act's Health Insurance Marketplace in your state. For assistance in finding coverage, please contact us at 800-821-6136 or visit our website at geha.com/Get-Support.

- **Health Insurance Marketplace**

If you would like to purchase health insurance through the ACA's Insurance Marketplace, please visit www.HealthCare.gov. This is a website provided by the U.S. Department of Health and Human Services that provides up-to-date information on the Marketplace.

Section 1. How This Plan Works

This Plan is a fee-for-service (FFS) plan. OPM requires that FEHB plans be accredited to validate that plan operations and/or care management meet nationally recognized standards. G.E.H.A holds the following accreditations: Health Plan Accreditation with Accreditation Association for Ambulatory Health Care (AAAHC) and Dental Network Accreditation with URAC. To learn more about this plan's accreditations, please visit the following websites: Accreditation Association for Ambulatory Health Care (www.aaahc.org); URAC (www.urac.org). You can choose your own physicians, facilities, and other healthcare providers. We give you a choice of enrollment in the Elevate or Elevate Plus plan.

We reimburse you or your provider for your covered services, usually based on a percentage of the amount we allow. The type and extent of covered services, and the amount we allow, may be different from other plans. Read brochures carefully.

General features of our Elevate Plus and Elevate Options

We have a Preferred Providers Organization (PPO) Network

Our fee-for-service plan offers in-network benefits through a PPO. This means that certain hospitals and other healthcare providers are “preferred providers.” When you use our PPO providers, you will receive services at reduced cost. Providers in the network accept a contracted payment from us, and you will only be responsible for your cost-sharing (copayments, coinsurance, deductibles, and non-covered services and supplies).

- On the Elevate Option, you also have benefits to receive covered services from non-participating providers; however, out-of-network benefits may have higher out-of-pocket costs than the in-network benefits.
- Elevate Plus does not provide benefits for non-participating providers, except in cases of emergency medical care.

To find in-network providers, use the provider search tool on the geha.com/Locate-Care website or call G.E.H.A at 800-821-6136. When you call your provider for an appointment, please remember to verify that the physician is still an in-network provider. G.E.H.A in-network providers are required to meet licensure and certification standards established by State and Federal authorities; however, inclusion in the network does not represent a guarantee of professional performance nor does it constitute medical advice.

On the Elevate Option, the out-of-network benefits are the standard benefits of this Plan. Elevate Plus does not provide out-of-network benefits. On both Elevate and Elevate Plus, in-network benefits apply only when you use an in-network provider. Provider networks may be more extensive in some areas than others. We cannot guarantee the availability of every specialty in all areas. If no in-network provider is available, or you do not use an in-network provider, the standard out-of-network benefits apply on the Elevate Option. However, if the services are rendered at an in-network facility, the professionals who provide services to you may not all be preferred providers. If the services are rendered by out-of-network providers at an in-network facility, we will pay up to the Plan allowance according to the No Surprises Act.

Note: Members residing outside the United States and accessing providers within their area of residence without access to an in-network providers will be paid at the in-network level of benefits.

How we pay providers

Fee-for-service plans reimburse you or your provider for covered services. They do not typically provide or arrange for healthcare. Fee-for-service plans let you choose your own physicians, facilities and other healthcare providers.

We offer benefits through the UnitedHealthcare Choice Plus network of individual physicians, medical groups, and facilities. These Plan providers accept a negotiated payment from us, and you will only be responsible for your cost-sharing (copayments, coinsurance, deductibles, and non-covered services and supplies), which may vary by plan.

We utilize Optum's Ingenix Claim Editing System (iCES) for United Health Network providers and Optum's Claims Editing System (CES) for non-United Health Network providers to review claims for bundling, unbundling, upcoding and other billing and coding edits using criteria that includes, but is not limited to, National Correct Coding Initiative (NCCI) guidelines, Centers for Medicare and Medicaid Services (CMS) guidelines, and Commercial (UHC) guidelines.

We reserve the right to audit medical expenses to ensure that the provider's billed charges match the services that you received.

Health education resources

Our website, at geha.com/Health-Wellness-Library, offers access to our Healthy Living resources for a wide range of information on general topics, pregnancy and childbirth, diabetes management and gym discounts.

Your rights and responsibilities

OPM requires that all FEHB plans provide certain information to their FEHB members. You may get information about us, our networks, and our providers. OPM's FEHB website (www.opm.gov/insure) lists the specific types of information that we must make available to you. Some of the required information is listed below:

- G.E.H.A was founded in 1937 as the Railway Mail Hospital Association. For over 88 years, G.E.H.A has provided health insurance benefits to Federal employees and retirees.
- G.E.H.A is incorporated as a General Not-For-Profit Corporation pursuant to Chapter 355 of the Revised Statutes of the State of Missouri.

You are also entitled to a wide range of consumer protections and have specific responsibilities as a member of this Plan. You can view the complete list of these rights and responsibilities by visiting our website, geha.com/Member-Rights. You can also contact us to request that we mail a copy to you.

If you wish to make a suggestion, file a formal complaint, require language translation services, or if you want more information about us, call 800-821-6136, or write to G.E.H.A Enrollment, PO Box 21262, Eagan, MN 55121. You may also visit our website at geha.com/Submit-Feedback.

By law, you have the right to access your protected health information (PHI). For more information regarding access to PHI, visit our website at geha.com/Your-Data-Protection to obtain our Notice of Privacy Practices. You can also contact us to request that we mail you a copy of that Notice.

Your medical and claims records are confidential

We will keep your medical and claims records confidential. Please note that we may disclose your medical and claims information (including your prescription drug utilization) to any of your treating physicians or dispensing pharmacies.

Section 2. Changes for 2026

Do not rely only on these change descriptions; this Section is not an official statement of benefits. For that, go to Section 5, *Benefits*. Also, we edited and clarified language throughout the brochure; any language change not shown here is a clarification that does not change benefits.

Changes to Elevate Option

- **Premium:** Your share of the premium rate will increase for Self Only, Self Plus One and Self and Family. See back cover.
- **Deductible:** The Plan will increase the calendar year deductible for in-network services for Self Only to \$750 from \$500 and for Self Plus One and Self and Family to \$1,500 from \$1,000. For out-of-network services, the calendar year deductible will increase for Self Only to \$2,000 from \$1,000 and for Self Plus One and Self and Family to \$4,000 from \$2,000.
- **Out-of-Pocket-Maximum:** The Plan will increase the in-network catastrophic out-of-pocket maximum for Self Only to \$10,600 from \$8,500 and for Self Plus One and Self Plus One and Self and Family to \$21,200 from \$17,000.
- **Emergency Room:** The Plan will increase the emergency room cost share to 35% of the Plan allowance, after the deductible from 25% of the Plan allowance, after the deductible for in and out-of-network services.
- **Prescription Drugs:** The Plan will increase the cost share network retail generic drugs to a \$10 copay from a \$4 copay (up to a 30-day supply), to \$20 from \$8 (31 to 60-day supply), and to \$30 from \$12 (61 to 90-day supply). See Section 5(f), *Prescription Drug Benefits*, page 83.

Changes to Elevate Plus Option

- **Premium:** Your share of the premium rate will increase for Self Only, Self Plus One and Self and Family. See back cover.
- **Coinsurance:** The Plan will increase the in-network coinsurance amount to 20% of the Plan allowance, after the deductible. from 15% of the Plan allowance, after the deductible.
- **Emergency Room:** The Plan will increase the emergency room cost share to 25% of the Plan allowance, after the deductible from 15% of the Plan allowance, after the deductible for in and out-of-network services.
- **Prescription Drugs:** The Plan will increase the cost share for network retail generic drugs to a \$15 copay from a \$10 copay (up to a 30-day supply), to \$30 from \$20 (31 to 60-day supply), and to \$45 from \$30 (61 to 90-day supply); network retail preferred drugs to a \$100 copay from an \$80 copay (up to a 30-day supply), to a \$200 copay from a \$160 copay (31 to 60-day supply), and to a \$300 copay from a \$240 copay (61 to 90-day supply). In addition, the cost share maximum for specialty CVS pharmacy generic and preferred drugs will increase from 40% of Plan allowance to \$700 from \$500 (up to a 30-day supply), to \$1,400 from \$1,000 (31 to 60-day supply) and to \$2,100 maximum from \$1,500 (61 to 90-day supply). See Section 5(f), *Prescription Drug Benefits*, page 83.

Changes to both Elevate and Elevate Plus Options

- **Breast Cancer Screening:** The Plan will expand preventive breast cancer screening from ages 50-74 to age 40-74. See Section 5(a), *Preventive care/adult*, page 37.
- **Mental Health and Substance Abuse:** The Plan will expand coverage to include no cost share on one primary care physician or specialist visit for the management of a mental health condition as a follow up within 30 days of an **emergency room visit for mental health and substance use disorder**. Previously, there was no cost share for the first follow-up PCP or specialist visit within 30 days of a mental health inpatient confinement only. See Section 5(e), *Mental Health and Substance Use Disorder Benefits*, page 78.
- **Urine Drug Screening:** The Plan will remove limits on urine drug testing. Previously, it was covered up to 16 tests for urine drug testing per person per calendar year. See Section 5(a), *Lab, x-ray, and other diagnostic tests*, page 35.
- **Reconstructive Surgery:** The Plan will remove coverage for surgery for Sex-Trait Modification to treat gender dysphoria. See Section 5(a), *Reconstructive surgery*, page 57.
- **Prescription Drugs:** The Plan will remove coverage for drugs prescribed in connection with sex-trait modification for treatment of gender dysphoria. See Section 5(f), *Non-covered medications and supplies*, page 94.

Section 3. How You Get Care

Identification cards

We will send you an identification (ID) card when you enroll. You should carry your ID card with you at all times. You must show it whenever you receive services from a Plan provider or fill a prescription at a Plan pharmacy. Until you receive your ID card, use your copy of the Health Benefits Election Form, SF-2809, your health benefits enrollment confirmation (for annuitants), or your electronic enrollment system (such as Employee Express) confirmation letter.

If you do not receive your ID card within 30 days after the effective date of your enrollment, or if you need replacement cards, call us at 800-821-6136 or write to us at:

G.E.H.A Enrollment
PO Box 21262
Eagan, MN 55121

You may also request replacement cards through our website: geha.com/My-Member-Portal.

Where you get covered care

You can get care from any "covered providers" and "covered facilities." How much we pay - and you pay - depends on the type of covered provider or facility you use and how the services are billed. If you use our preferred providers, you will pay less.

Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and any applicable medical or payment policies. Policies may be found on our website geha.com/Provider-Policies. In some instances, additional services may be rendered, and additional cost shares may apply.

Benefits are applied based on how your provider bills us. You must reference all Sections in this Brochure depending on where your care is obtained. For example, but not limited to, any procedure, injection, diagnostic service, laboratory, or x-ray service done in a hospital in conjunction with a physician office examination may apply separate cost share depending on where you obtain your care.

Balance Billing Protection

FEHB Carriers must have clauses in their in-network (participating) provider agreements. These clauses provide that, for a service that is a covered benefit in the plan brochure or for services determined not medically necessary, the in-network provider agrees to hold the covered individual harmless (and may not bill) for the difference between the billed charge and the in-network contracted amount. If an in-network provider bills you for covered services over your normal cost share (deductible, copay, co-insurance), contact your Carrier to enforce the terms of its provider contract.

• Covered providers

Covered providers are medical practitioners who perform covered services when acting within the scope of their license or certification under applicable state law and who furnish, bill or are paid for their healthcare services in the normal course of business. Covered services must be provided in the state in which the practitioner is licensed or certified.

Benefits are provided under this Plan for the services of covered providers, in accordance with Section 2706(a) of the Public Health Service Act. Coverage of practitioners is not determined by your state's designation as a medically underserved area.

We list network-contracted covered providers in our network provider directory, which we update periodically, and make available on our website.

Benefits described in this brochure are available to all members meeting medical necessity guidelines regardless of race, color, national origin, age, disability, religion, sex, pregnancy, or genetic information.

This plan provides Care Coordinators for complex conditions and can be reached at 800-821-6136.

- **Covered facilities**

Covered facilities include:

- **Freestanding Ambulatory Facility**
 - A facility which is licensed by the state as an ambulatory surgery center or has Medicare certification as an ambulatory surgical center, has permanent facilities and equipment for the primary purpose of performing surgical and/or renal dialysis procedures on an outpatient basis; provides treatment by or under the supervision of doctors and nursing services whenever the patient is in the facility; does not provide inpatient accommodations; and is not, other than incidentally, a facility used as an office or clinic for the private practice of a doctor or other professional.
 - If the state does not license Ambulatory Surgical Centers and the facility is not Medicare certified as an ambulatory surgical center, then they must be accredited with AAAHC (Accreditation Association for Ambulatory Health Care), AAAASF (American Association for Accreditation for Ambulatory Surgery Facilities), IMQ (Institute for Medical Quality) or TJC (The Joint Commission).
 - Ambulatory Surgical Facilities in the state of California do not require a license if they are physician owned. To be covered, these facilities must be accredited by one of the following: AAAHC (Accreditation Association for Ambulatory Health Care), AAAASF (American Association for Accreditation for Ambulatory Surgery Facilities), IMQ (Institute for Medical Quality) or TJC (The Joint Commission).
- **Hospital**
 - An institution or distinct portion of an institution that is primarily engaged in providing: (1) general inpatient acute care and treatment of sick and injured persons through medical, diagnostic, and major surgical facilities; or (2) specialized inpatient acute medical care and treatment of sick or injured persons through medical and diagnostic facilities (including x-ray and laboratory); or (3) comprehensive specialized services relating to the individual's specific medical, physical, mental health, and/or substance use disorder therapy needs, and has, for each patient, an individualized written treatment plan, which includes diagnostic assessment of the patient and a description of the treatment to be rendered, and provides for follow-up assessments by, or under, the direction of the supervising doctor.
 - All services must be provided on its premises, under its control, or through a written agreement with a hospital or with a specialized provider of those facilities.
 - A hospital must be operated pursuant to law, accredited as a hospital under the Hospital Accreditation Program of The Joint Commission (TJC) or meet the states' applicable licensing or certification requirements for a hospital, and is operating under the supervision of a staff of physicians with 24-hour-a-day registered nursing services.
 - The term hospital does not include a convalescent home, extended care facility, skilled nursing facility, or any institution or part thereof which: (1) is used principally as a convalescent facility, nursing facility, or long-term care facility; (2) furnishes primarily domiciliary or custodial care, including training in the routines of daily living; or (3) is operating as or is licensed as a school or residential treatment facility (except as listed in Section 5(e)).
- **Hospice** - A facility which meets all of the following:
 - Primarily provides inpatient hospice care to terminally ill persons;

- Is certified by Medicare as such, or is licensed or accredited as such, by the jurisdiction it is in;
- Is supervised by a staff of M.D.'s or D.O.'s, at least one of whom must be on call at all times;
- Provides 24-hour-a-day nursing services under the direction of an R.N. and has a full-time administrator; and
- Provides an ongoing quality assurance program.
- Skilled Nursing Facility licensed by the state or certified by Medicare if the state does not license these facilities. See limitations in Section 5(c), *Services Provided by a Hospital or Other Facility, and Ambulance Services*.
- Birth Center
 - A birth center is a health facility that is not a hospital or physician's office, where childbirth is planned to occur away from the pregnant woman's residence, that is licensed or otherwise approved by the state to provide prenatal, labor and delivery, or postpartum care that is covered by the plan.
- Residential Treatment Centers (RTCs)
 - An institution that is primarily engaged in providing: (1) 24-hour residential evaluation, treatment, and comprehensive specialized services relating to the individual's specific mental health, and/or substance use disorder therapy needs, all under the active participation and direction of a licensed physician who is practicing within the scope of the physician's license; and (2) specialized programs for persons who need short-term services designed to achieve predicted outcomes focused on fostering improvement or stability in mental health and/or substance use disorder, recognizing the individuality, strengths, and needs of the persons served; and (3) care that meets evidence-based treatment guidelines or criteria as determined by the plan.
 - The services are provided for a fee from its patients and include both: (1) room and board; and (2) 24-hour-a-day registered nursing services. Additionally, the RTC keeps adequate patient records which include: (1) the individualized treatment plan; and (2) the person's progress; and (3) discharge summary; and (4) follow-up programs. Benefits are available for services performed and billed by RTCs, as described in Section 5(e), *Mental Health and Substance Use Disorder Benefits*.
 - RTCs must be: (1) operated pursuant to law; and (2) accredited by a nationally recognized organization, and licensed by the state, district or territory to provide residential treatment for mental health conditions and/or substance use disorder; or (3) credentialed by a network partner.
 - The term RTC does not include a convalescent home, extended care facility, skilled nursing facility, group home, halfway house, sober home, transitional living center or treatment, or any institution or part thereof which: (1) is used principally as a convalescent facility, nursing facility, or long-term care; (2) furnishes primarily domiciliary or custodial care, including training in the routines of daily living; or (3) is operating or licensed as a school.

- **Transitional care** **Specialty care:** You may continue seeing your specialist and receiving any PPO benefits for up to 90 days if you are undergoing treatment for a chronic or disabling condition and you lose access to your specialist because:
 - we drop out of the Federal Employees Health Benefits (FEHB) Program and you enroll in another FEHB Plan, or
 - we terminate our contract with your specialist for reasons other than for cause.

Contact us at 800-821-6136 or, if we drop out of the Program, contact your new plan.

If you are in the second or third trimester of pregnancy and you lose access to your specialist based on the above circumstances, you can continue to see your specialist and your PPO benefits will continue until the end of your postpartum care, even if it is beyond the 90 days.

Note: If you lose access to your specialist because you changed your carrier or plan option enrollment, contact your new plan.

Sex-Trait Modification: If you are mid-treatment under this Plan, within a surgical or chemical regimen for Sex-Trait Modification for diagnosed gender dysphoria, for services for which you received coverage under the 2025 Plan brochure, you may seek an exception to continue care for that treatment. Ongoing sex-trait modification hormone treatment is not a covered benefit under this plan as of 1/1/2026. Our exception process is as follows: To initiate the exception process, contact Customer Care at 800-821-6136, Monday-Friday, 8 a. m. - 8 p.m. E.T. If you disagree with our decision on your exception, please see Section 8 of this brochure for the disputed claims process.

Individuals under age 19 are not eligible for exceptions related to services for ongoing surgical or hormonal treatment for diagnosed gender dysphoria.

- **If you are hospitalized when your enrollment begins**

We pay for covered services from the effective date of your enrollment. However, if you are in the hospital when your enrollment in our Plan begins, call our Customer Care department immediately at 800-821-6136. If you are new to the FEHB Program, we will reimburse you for your covered services while you are in the hospital beginning on the effective date of your coverage.

If you changed from another FEHB plan, your former plan will pay for the hospital stay until:

- You are discharged, not merely moved to an alternative care center;
- The day your benefits from your former plan run out; or
- The 92nd day after you become a member of this Plan, whichever happens first.

These provisions apply only to the benefits of the hospitalized person. If your plan terminates participation in the FEHB in whole or in part, or if OPM orders an enrollment change, this continuation of coverage provision does not apply. In such cases, the hospitalized person's benefits under the new plan begin on the effective date of enrollment.

You need prior Plan approval for certain services

The pre-service claim approval processes for inpatient hospital admissions (called precertification) and for other services, are detailed in this Section. A **pre-service claim** is any claim, in whole or in part, that requires approval from us in advance of obtaining medical care or services. In other words, a pre-service claim for benefits (1) requires precertification, prior approval or a referral and (2) will result in a reduction of benefits if you do not obtain precertification, prior approval or a referral.

- **Inpatient facility admission**

Precertification is the process by which – prior to your inpatient admission – we evaluate the medical necessity of your proposed stay, the location in which services are intended to be rendered, and the number of days required to treat your condition. Unless we are misled by the information given to us, we won't change our decision on medical necessity.

In most cases, your physician or facility will take care of requesting precertification. Because you are still responsible for ensuring that your care is precertified, you should always ask your physician or facility whether or not they have contacted us.

Avoid paying providers for services prior to precertification. It is important to assure services are authorized by a covered provider or facility.

We only cover a private room if we determine it to be medically necessary. Otherwise, we will pay the hospital's average charge for semi-private accommodations. The remaining balance is not a covered expense. If the hospital only has private rooms, we will cover the private room rate.

For out-of-network facilities on the Elevate Option, you are responsible for obtaining precertification for these services.

- **Warning** **In-network:** Network Plan physicians must provide or arrange your in-network care and get precertification or preauthorization for certain services and/or procedures, including but not limited to those noted below in *Other services that require preauthorization*.

Note: Avoid paying providers for services prior to preauthorization. It is important to assure services are authorized and provided by a covered provider or facility.

Out-of-network (Elevate only):

You must get precertification for certain services prior to admission. Failure to do so will result in the following penalties, unless due to a medical emergency:

- We will reduce our benefits for the Inpatient Hospital stay, Long Term Acute Care, Residential Treatment Center (RTC), Skilled Nursing Facility (SNF), or Rehabilitation Facility stay by **\$500 per day for each day that is not precertified prior to admission, limited to a maximum penalty of \$5,000 per admission.** If the stay is not medically necessary, we will only pay for any covered medical services and supplies that are otherwise payable on an outpatient basis.

- **Exceptions** You do not need precertification in these cases:
 - You have another group health insurance policy that is the primary payor for the facility stay or other services; or
 - Medicare Part A is primary payor for the facility stay. If you exhaust your Medicare hospital benefits and do not want to use your Medicare lifetime reserve days, then we will become the primary payor, and you **do** need precertification.

Other services Some surgeries and procedures, drugs, tests, services and equipment require precertification or preauthorization. Specific services requiring preauthorization or precertification are indicated in Section 5, when applicable. Please note this list is subject to change, please call to verify if your procedure requires preauthorization.

Services requiring preauthorization or medical necessity determination may be reviewed with guidelines as described at geha.com/Coverage-Criteria. G.E.H.A has coverage policies for many services and procedures; refer to geha.com/Provider-Policies for a complete list of policies.

Transplants services require more extensive reviews. The transplant must be performed in a designated transplant facility to receive maximum benefits.

If precertification is not obtained or a Plan-designated transplant facility is not used, our allowance will be limited for hospital and surgery expenses up to a maximum of \$100,000 per transplant. If we cannot refer a member in need of a transplant to a designated facility, the \$100,000 maximum will not apply.

- Tandem bone marrow transplants approved as one treatment protocol are limited to \$100,000 when not performed at a Plan-designated facility. All treatment within 120 days following the transplant is subject to the \$100,000 limit. Outpatient prescription drugs are not a part of the \$100,000 limit.
- Simultaneous transplants such as kidney/pancreas, heart/lung, heart/liver are considered as one transplant procedure and are limited to \$100,000 when not performed at a Plan-designated transplant facility.

- Chemotherapy and procedures related to bone marrow transplantation must be performed only at a Plan-designated transplant facility to receive maximum benefits.

If benefits are limited to \$100,000 per transplant, included in the maximum are all charges for hospital, medical and surgical care incurred while the patient is hospitalized for a covered transplant surgery and subsequent complications related to the transplant. Outpatient expenses for chemotherapy and any process of obtaining stem cells or bone marrow associated with bone marrow transplant (stem cell support) are included in benefits limit of \$100,000 per transplant.

• **Other services that require preauthorization**

- Applied behavioral analysis (ABA) therapy;
- Arthroplasty;
- Bariatric (obesity surgery);
- Bone growth stimulators;
- Cellular and gene therapy;
- Certain prescription drugs
- Cochlear and auditory implants and implant procedures;
- Durable medical equipment (DME) over \$1,000;
- Facility admissions (including hospitals, long term acute care, rehabilitation facilities, residential treatment centers, and skilled nursing facilities; see above*);
- Fertility procedures and drugs;
- Genetic testing;
- Hysterectomy except for diagnosis of cancer;
- Neurostimulation, including devices and implantation procedures for cranial, gastric, peripheral, spinal, or vagus nerve stimulation;
- Non-emergency air ambulance transportation;
- Orthognathic surgery (jaw), including TMJ;
- Orthopedic and prosthetic devices over \$1,000;
- Prostate implants, destruction, and removal;
- Psychological testing and neuropsychological testing exceeding 8 hours/calendar year;
- Reconstructive procedures;
- Sleep studies (in-lab) attended or performed in a healthcare facility;
- Transplants

How to precertify an admission for Other services

First, you, your representative, your physician, or your facility must call to obtain preauthorization before an inpatient facility admission, Residential Treatment Center (RTC) admission, or for services requiring precertification are rendered. Refer to the back of your member ID card under the heading Prior Authorization for the contact information or contact us at 800-821-6136.

Next, provide the following information:

- enrollee's name and plan identification number;
- patient's name, birth date, and phone number;
- reason for hospitalization, proposed treatment, or surgery;
- name and phone number of admitting doctor;
- name of hospital or facility; and
- number of days requested for hospital stay.

- **Non-urgent care claims**

We will then tell the doctor and/or facility the number of approved inpatient days and we will send written confirmation of our decision to you, your doctor, and the facility.

For non-urgent care claims, we will tell the physician and/or facilities the number of approved inpatient days, or the care that we approve for other services that must have precertification. We will make our decision within 15 days of receipt of the pre-service claim.

If matters beyond our control require an extension of time, we may take up to an additional 15 days for review and we will notify you of the need for an extension of time before the end of the original **15-day** period. Our notice will include the circumstances underlying the request for the extension and the date when a decision is expected.

If we need an extension because we have not received necessary information from you, our notice will describe the specific information required and we will allow you up to 60 days from the receipt of the notice to provide the information.

- **Urgent care claims**

If you have an **urgent care claim** (i.e., when waiting for the regular time limit for your medical care or treatment could seriously jeopardize your life, health, or ability to regain maximum function, or in the opinion of a physician with knowledge of your medical condition, would subject you to severe pain that cannot be adequately managed without this care or treatment), we will expedite our review and notify you of our decision within 72 hours. If you request that we review your claim as an urgent care claim, we will review the documentation you provide and decide whether or not it is an urgent care claim by applying the judgment of a prudent layperson that possesses an average knowledge of health and medicine.

If you fail to provide sufficient information, we will contact you within 24 hours after we receive the claim to let you know what information we need to complete our review of the claim. You will then have up to 48 hours to provide the required information. We will make our decision on the claim within 48 hours of (1) the time we received the additional information, or (2) the end of the time frame, whichever is earlier.

We may provide our decision orally within these time frames, but we will follow up with written or electronic notification within three days of oral notification.

You may request that your urgent care claim on appeal be reviewed simultaneously by us and OPM. Please let us know that you would like a simultaneous review of your urgent care claim by OPM either in writing at the time you appeal our initial decision, or by calling us at 800-821-6136. You may also call OPM's Health Insurance 2 at 202-606-3818 between 8 a. m. and 5 p.m. Eastern Time to ask for the simultaneous review. We will cooperate with OPM so they can quickly review your claim on appeal. In addition, if you did not indicate that your claim was a claim for urgent care, then call us at 800-821-6136. If it is determined that your claim is an urgent care claim, we will expedite our review (if we have not yet responded to your claim).

- **Concurrent care claims**

A concurrent care claim involves care provided over a period of time or over a number of treatments. We will treat any reduction or termination of our pre-approved course of treatment before the end of the approved period of time or number of treatments as an appealable decision. This does not include reduction or termination due to benefit changes or if your enrollment ends. If we believe a reduction or termination is warranted, we will allow you sufficient time to appeal and obtain a decision from us before the reduction or termination takes effect.

If you request an extension of an ongoing course of treatment at least 24 hours prior to the expiration of the approved time period and this is also an urgent care claim, we will make a decision within 24 hours after we receive the claim.

- **Emergency inpatient admission** If you have an emergency admission due to a condition that you reasonably believe puts your life in danger or could cause serious damage to bodily function, you, your representative, the physician, or the facility must notify us within two business days following the day of the emergency admission, even if you have been discharged from the facility.
- **Maternity care** You do not need precertification of a maternity admission for a routine delivery. However, if your medical condition requires you to stay more than 48 hours after a vaginal delivery or 96 hours after a cesarean section, then your physician or the facility must contact us for precertification of additional days. Further, if your baby stays after you are discharged, your physician or the facility must contact us for precertification of additional days for your baby.

Note: When a newborn requires definitive treatment during or after the mother's stay, the newborn is considered a patient in their own right. If the newborn is eligible for coverage, regular medical or surgical benefits apply rather than maternity benefits.
- **If your facility stay needs to be extended** If your facility stay - including for maternity care - needs to be extended, you, your representative, your doctor or the facility must ask us to approve the additional days. If you remain in the facility beyond the number of days we approved and did not get the additional days precertified, then
 - for the part of the admission that was medically necessary, we will pay inpatient benefits, but
 - for the part of the admission that was not medically necessary, we will pay only medical services and supplies otherwise payable on an outpatient basis and will not pay inpatient benefits.
- **If your treatment needs to be extended** If you request an extension of an ongoing course of treatment at least 24 hours prior to the expiration of the approved time period and this is also an urgent care claim, we will make a decision within 24 hours after we receive the claim.
- If you disagree with our pre-service claim decision** If you have a **pre-service claim** and you do not agree with our decision regarding precertification of an inpatient admission or preauthorization of other services, you may request a review in accord with the procedures detailed below. If your claim is in reference to a contraceptive, call 844-443-4279.

If you have already received the service, supply, or treatment, then you have a **post-service claim** and must follow the entire disputed claims process detailed in Section 8.
- **To reconsider a non-urgent care claim** Within 6 months of our initial decision, you may ask us in writing to reconsider our initial decision. Follow Step 1 of the disputed claims process detailed in Section 8 of this brochure.

In the case of a pre-service claim and subject to a request for additional information, we have 30 days from the date we receive your written request for reconsideration to:
 1. Precertify your hospital stay or, if applicable, arrange for the healthcare provider to give you the care or grant your request for preauthorization for a service, drug, or supply; or
 2. Ask you or your provider for more information. You or your provider must send the information so that we receive it within 60 days of our request. We will then decide within 30 more days. If we do not receive the information within 60 days, we will decide within 30 days of the date the information was due. We will base our decision on the information we already have. We will write to you with our decision.
 3. Write to you and maintain our denial.
- **To reconsider an urgent care claim** In the case of an appeal of a pre-service urgent care claim, within 6 months of our initial decision, you may ask us in writing to reconsider our initial decision. Follow Step 1 of the disputed claims process detailed in Section 8 of this brochure.

Unless we request additional information, we will notify you of our decision within 72 hours after receipt of your reconsideration request. We will expedite the review process, which allows oral or written requests for appeals and the exchange of information by phone, electronic mail, facsimile, or other expeditious methods.

- **To file an appeal with OPM**

After we reconsider your **pre-service claim**, if you do not agree with our decision, you may ask OPM to review it by following Step 3 of the disputed claims process detailed in Section 8 of this brochure.

Overseas claims

For covered services you receive by physicians and hospitals outside the United States and Puerto Rico, send a completed Overseas Claim Form and the itemized bills to:

G.E.H.A Medical Claims
PO Box 21172
Eagan, MN 55121

You may obtain overseas claim forms from geha.com/Health-Overseas-Access.

Eligibility and/or medical necessity review is required when procedures are performed, or you are admitted to a hospital outside of the United States. Review includes the procedure and/or service to be performed, the number of days required to treat your condition, and any other applicable benefit criteria.

If you have questions about the processing of overseas claims, contact us at 800-821-6136. Covered providers outside the United States will be paid at the in-network level of benefits, subject to the deductible, copays and/or coinsurance. We will provide translation and currency conversion for claims for overseas (foreign) services. The conversion rate will be based on the date services were rendered.

When members living abroad are stateside and seeking medical care, contact us at 800-821-6136, or visit geha.com/Health-Overseas-Access to locate an in-network provider. When you are seeking treatment stateside, all precertification and authorization requirements are applicable. If you utilize an out-of-network provider, out-of-network benefits would apply on the Elevate Option. The Elevate Plus Option does not provide benefits for out-of-network providers, except in cases of emergency medical care.

Section 4. Your Costs for Covered Services

This is what you will pay out-of-pocket for your covered care:

Cost-sharing

Cost-sharing is the general term used to refer to your out-of-pocket costs (e.g., deductible, coinsurance, and copayments) for the covered care you receive.

Copayments

A copayment is a fixed amount of money you pay to the provider, facility, pharmacy, etc., when you receive certain services.

Example: When you see your in-network Primary Care Provider (PCP), under Elevate Plus, you pay a copayment of \$30 per visit.

Note: If the billed amount (or the Plan allowance that providers we contract with have agreed to accept as payment in full) is less than your copayment, you pay the lower amount.

Deductible

A deductible is a fixed amount of covered expenses you must incur for certain covered services and supplies before we start paying benefits for them. Copayments and coinsurance amounts do not count toward any deductible. When a covered service or supply is subject to a deductible, only the Plan allowance for the service or supply counts toward the deductible.

Elevate Plus Option

In-Network: Under a Self Only enrollment, the deductible is considered satisfied, and benefits are payable for you when your covered expenses applied to the calendar year deductible for your enrollment reach \$200. Under the Self Plus One and the Self and Family enrollments, once the calendar year deductible amount of \$200 is satisfied for an individual, covered benefits are payable for that individual; the calendar year deductible is met for all family members when the covered expenses accumulated to the calendar year deductible for any combination of family members reaches the Self Plus One or the Self and Family limit of \$400. Only plan allowance paid for services or supplies from in-network providers counts toward this amount.

Out-of-Network: Elevate Plus does not provide out-of-network benefits.

Elevate Option

In-Network: Under a Self Only enrollment, the deductible is considered satisfied, and benefits are payable for you when your covered expenses applied to the calendar year deductible for your enrollment reaches \$750. Under the Self Plus One and the Self and Family enrollments, once the calendar year deductible amount of \$750 is satisfied for an individual, covered benefits are payable for that individual; the calendar year deductible is met for all family members when the covered expenses accumulated to the calendar year deductible for any combination of family members reaches the Self Plus One or the Self and Family limit of \$1,500. Only plan allowance paid for services or supplies from in-network providers counts toward this amount.

Out-of-Network: Under a Self Only enrollment, the deductible is considered satisfied, and benefits are payable for you when your covered expenses applied to the calendar year deductible for your enrollment reach \$2,000. Under the Self Plus One and the Self and Family enrollments, once the calendar year deductible amount of \$2,000 is satisfied for an individual, covered benefits are payable for that individual; the calendar year deductible is met for all family members when the covered expenses accumulated to the calendar year deductible for any combination of family members reaches the Self Plus One or the Self and Family limit of \$4,000. Only plan allowance paid for services or supplies from out-of-network providers counts toward this amount.

If the billed amount (or the Plan allowance that providers we contract with have agreed to accept as payment in full) is less than the remaining portion of your deductible, you pay the lower amount.

Example: If the billed amount is \$100, the in-network provider has an agreement with us to accept \$80, and you have not paid any amount toward meeting your calendar year deductible, you must pay \$80. We will apply \$80 to your deductible. We will begin paying benefits once the remaining portion of your calendar year deductible has been satisfied.

Note: If you change plans during Open Season and the effective date of your new plan is after January 1 of the next year, you do not have to start a new deductible under your prior plan between January 1 and the effective date of your new plan. If you change plans at another time during the year, you must begin a new deductible under your new plan.

If you change enrollment options in this Plan during the year, we will credit the amount of covered expenses already applied toward the deductible of your old option to the deductible of your new option.

Coinsurance

Coinsurance is the percentage of our allowance that you must pay for your care. Coinsurance does not begin until you have met your deductible. We will base this percentage on either the billed charge or the Plan allowance, whichever is less.

Example: Under the Elevate Option, you pay 25% (or 20% under the Elevate Plus Option) of our allowance for certain in-network services.

If your provider routinely waives your cost

If your provider routinely waives (does not require you to pay) your copayments, deductibles, or coinsurance, the provider is misstating the fee and may be violating the law. In this case, when we calculate our share, we will reduce the provider's fee by the amount waived.

For example, if your physician ordinarily charges \$100 for a service but routinely waives your 30% coinsurance, the actual charge is \$70. We will pay \$49 (70% of the actual charge of \$70).

Waivers

In some instances, a provider may ask you to sign a "waiver" prior to receiving care. This waiver may state that you accept responsibility for the total charge for any care that is not covered by your health plan. If you sign such a waiver, whether or not you are responsible for the total charge depends on the contracts that the Plan has with its providers. If you are asked to sign this type of waiver, please be aware that if benefits are denied for the services, you could be legally liable for the related expenses. If you would like more information about waivers, please contact us at 800-821-6136 or write to G.E.H.A Enrollment, PO Box 21262, Eagan, MN 55121.

Differences between our allowance and the bill

Our "Plan allowance" is the amount we use to calculate our payment for covered services. Fee-for-service plans arrive at their allowances in different ways, so their allowances vary. For more information about how we determine our Plan allowance, see the definition of Plan allowance in Section 10.

Often, the provider's bill is more than a fee-for-service plan's allowance. Whether or not you have to pay the difference between our allowance, and the bill will depend on the provider you use.

- **In-network providers** agree to limit what they will bill you. Because of that, when you use a preferred provider, your share of covered charges consists only of your deductible and coinsurance or copayment. Here is an example about coinsurance: You see an in-network physician who charges \$150, but our allowance is \$100. If you have met your deductible, you are only responsible for your coinsurance. That is, with the Elevate Option, you pay just 25% of our \$100 allowance (\$25). Because of the agreement, your in-network physician will not bill you for the \$50 difference between our allowance and the bill.

- **Out-of-network providers**, on the other hand, have no agreement to limit what they will bill you. When you use an out-of-network provider, you will pay your deductible and coinsurance - plus any difference between our allowance and charges on the bill. Here is an example: You see an out-of-network physician who charges \$150, and our allowance is again \$100. Because you've met your deductible, you are responsible for your coinsurance, so with the Elevate Option you pay 50% of our \$100 allowance (\$50). Plus, because there is no agreement between the out-of-network physician and us, the physician can bill you for the \$50 difference between our allowance and the bill. Elevate Plus Option does not provide benefits for out-of-network providers, so you would pay 100% of the provider's billed charges.

The following illustrates the examples of how much you have to pay out-of-pocket, under the Elevate Option, for services from an in-network physician vs. an out-of-network physician. This is an example of a service for which the physician charges \$150 and our allowance is \$100. The example shows the amount you pay if you have met your calendar year deductible.

EXAMPLE - Elevate Option

In-network physician

Physician's charge: \$150
 Our allowance: We set it at: \$100
 We pay: 75% of our allowance: \$75
 You owe: 25% of our allowance: \$25
 +Difference up to charge: No: \$0
TOTAL YOU PAY: \$25

Out-of-network physician

Physician's charge: \$150
 Our allowance: We set it at: \$100
 We pay: 50% of our allowance: \$50
 You owe: 50% of our allowance: \$50
 +Difference up to charge: Yes: \$50
TOTAL YOU PAY: \$100

Your catastrophic protection out-of-pocket maximum

After your (deductible, copayments and coinsurance) reaches the out-of-pocket maximum you do not have to pay any more for covered services, with the exception of certain cost sharing for the services below which do not count toward your catastrophic protection out-of-pocket maximum.

- **In-network**
 - For the Elevate Plus Option, the out-of-pocket maximum is \$7,000 for Self Only enrollment; \$14,000 when enrollment is Self Plus One or Self and Family when you use in-network providers.
 - For the Elevate Option, the out-of-pocket maximum is \$10,600 for Self Only enrollment; \$21,200 when enrollment is Self Plus One or Self and Family if you use in-network providers.
 - Only eligible out-of-pocket expenses from in-network providers count toward these limits.
 - An individual under Self Plus One and Self and Family enrollment will never have to satisfy more than what is required for the out-of-pocket maximum under a Self only enrollment.
- **Out-of-network**
 - The Elevate Plus Option does not provide out-of-network benefits.
 - For the Elevate Option, the out-of-pocket maximum is \$17,000 for Self Only enrollment; \$34,000 when enrollment is Self Plus One or Self and Family if you use out-of-network providers. Only eligible out-of-pocket expenses from out-of-network providers count toward these limits.
 - An individual under Self Plus One or Self and Family enrollment will never have to satisfy more than what is required for the out-of-pocket maximum under a Self only enrollment.

Out-of-pocket expenses for in-network and out-of-network benefits are the expenses you pay for covered services.

The following cannot be counted toward catastrophic protection out-of-pocket expenses:

- Expenses you pay for non-covered services;
- Expenses in excess of our allowable amount or maximum benefit limitations;
- Charges incurred by failure to obtain pre-certification, when using non-network facilities, and other amounts you pay because benefits have been reduced/denied for non-compliance with the Plans requirements (see Section 3);
- Expenses in excess of plan limits for dental;
- The cost for non-approved medications and drugs that we exclude; and
- The difference between the cost of the generic and brand name medication.

Carryover

If you changed to this Plan during Open Season from a plan with a catastrophic protection benefit and the effective date of the change was after January 1, any expenses that would have applied to that plan's catastrophic protection benefit during the prior year will be covered by your prior plan if they are for care you received in January before your effective date of coverage in this Plan. If you have already met your prior plan's catastrophic protection benefit level in full, it will continue to apply until the effective date of your coverage in this Plan. If you have not met this expense level in full, your prior plan will first apply your covered out-of-pocket expenses until the prior year's catastrophic level is reached and then apply the catastrophic protection benefit to covered out-of-pocket expenses incurred from that point until the effective date of your coverage in this Plan. Your prior plan will pay these covered expenses according to this year's benefits; benefit changes are effective January 1.

Note: If you change options in this Plan during the year, we will credit the amount of covered expenses already accumulated toward the catastrophic out-of-pocket limit of your old option to the catastrophic protection limit of your new option.

If we overpay you

We will make diligent efforts to recover benefit payments we made in error but in good faith. We may reduce subsequent benefit payments to offset overpayments.

When Government facilities bill us

Facilities of the Department of Veteran Affairs, the Department of Defense, and the Indian Health Service are entitled to seek reimbursement from us for certain services and supplies they provide to you or a covered family member. They may not seek more than their governing laws allow. You may be responsible to pay for certain services and charges. Contact the government facility directly for more information.

Important Notice About Surprise Billing - Know Your Rights

The No Surprises Act (NSA) is a federal law that provides you with protections against "surprise billing" and "balance billing" for out-of-network emergency services; out-of-network non-emergency services provided with respect to a visit to a participating facility; and out-of-network air ambulance services.

A surprise bill is an unexpected bill you receive for:

- emergency care - when you have little or no say in the facility or provider from whom you receive care, or for
- non-emergency services furnished by non-participating providers with respect to patient visits to participating health care facilities, or for
- air ambulance services furnished by non-participating providers of air ambulance services.

Balance billing happens when you receive a bill from the non-participating provider, facility, or air ambulance service for the difference between the non-participating provider's charge and the amount payable by your health plan.

Your health plan must comply with the NSA protections that hold you harmless from surprise bills.

For specific information on surprise billing, the rights and protections you have, and your responsibilities, go to geha.com/Plan-Transparency or contact the health plan at 800-821-6136.

**The Federal Flexible
Spending Account
Program - FSAFEDS**

- **Healthcare FSA (HCFSA)** - Reimburses an FSA participant for eligible out-of-pocket healthcare expenses (such as copayments, deductibles, over-the-counter drugs and medications, vision and dental expenses, and much more) for the participant and, their tax dependents, and their adult children (through the end of the calendar year in which they turn 26).
- FSAFEDS offers paperless reimbursement for your HCFSA through a number of FEHB and FEDVIP plans. This means that when you or your provider files claims with your FEHB or FEDVIP plan, FSAFEDS will automatically reimburse your eligible out-of-pocket expenses based on the claim information it receives from your plan.

Section 5. Elevate Plus and Elevate Benefits

See Section 2, *Changes for 2026* for how our benefits changed this year. See the *Summary of Benefits* sections for both the Elevate Plus and Elevate options in the back of the brochure. Make sure that you review the benefits that are available under the option in which you are enrolled.

Section 5. Elevate Plus and Elevate Benefits.....	29
Elevate Plus and Elevate Overview	31
Section 5(a). Medical Services and Supplies Provided by Physicians and Other Healthcare Professionals	32
Diagnostic and treatment services.....	33
Telehealth services	33
Lab, x-ray and other diagnostic tests	34
Preventive care, adult.....	35
Preventive care, children.....	36
Maternity care	38
Family planning	40
Infertility services	40
Allergy care.....	42
Treatment therapies.....	42
Physical, occupational, speech, habilitative and rehabilitative therapy.....	44
Cognitive rehabilitation.....	44
Hearing services (testing, treatment, and supplies).....	45
Vision services (testing, treatment, and supplies).....	45
Foot care.....	46
Orthopedic and prosthetic devices	46
Durable medical equipment (DME).....	47
Home health services	49
Chiropractic services.....	50
Alternative treatments	50
Educational classes and programs.....	51
Section 5(b). Surgical and Anesthesia Services Provided by Physicians and Other Healthcare Professionals	52
Surgical procedures.....	53
Reconstructive surgery.....	54
Oral and maxillofacial surgery.....	55
Organ/tissue transplants	56
Anesthesia	63
Section 5(c). Services Provided by a Hospital or Other Facility, and Ambulance Services	64
Inpatient hospital	65
Outpatient hospital or ambulatory surgical center	66
Extended care benefits/Skilled nursing care facility benefits	68
Hospice/End of life care.....	68
Ambulance	69
Section 5(d). Emergency Services/Accidents	70
Accidental injury.....	71
Medical emergency	71
Urgent Care Facility.....	71
Ambulance	71
Section 5(e). Mental Health and Substance Use Disorder Benefits.....	73
Professional services	73

Applied Behavioral Analysis Therapy	75
Diagnostics	75
Telehealth services	76
Inpatient hospital or other covered facilities	76
Outpatient hospital or other covered facilities	76
Section 5(f). Prescription Drug Benefits	78
Covered medications and supplies	83
Specialty drug benefits	86
Preventive care medications	87
Section 5(g). Dental Benefits	90
Accidental injury benefits	90
Section 5(h). Wellness and Other Special Features	92
Flexible Benefits Option	92
Impaired Hearing Services	92
Wellness Incentives	92
24-Hour Nurse Advice Line	92
Telehealth	92
Member Portal	93
Weight Management	93
G.E.H.A Maternity Program	93
Personal Health Record	93
Value Added Programs and Services	93
Family Planning Care Program	93
Summary of Benefits for the Elevate Plus Option of the Government Employees Health Association, Inc. 2026	123
Summary of Benefits for the Elevate Option of the Government Employees Health Association, Inc. 2026	125

Elevate Plus and Elevate Overview

This Plan offers both an Elevate Plus and an Elevate Option. Both benefit packages are described in Section 5. Make sure that you review the benefits that are available under the option in which you are enrolled.

The Elevate Plus and Elevate Options Section 5 is divided into subsections. Please read *Important things you should keep in mind* at the beginning of each subsection. For more information about services, see Section 1, *We have a Preferred Provider Network*. Also read the *General Exclusions* in Section 6; they apply to the benefits in the following subsections. To obtain claim forms, claims filing advice, or more information about Elevate Plus and Elevate benefits, contact us at 800-821-6136 or on our website at geha.com/Get-Support.

Each option offers unique features:

Elevate Plus

- You must use in-network providers for your care to be eligible for benefits, except in certain circumstances, such as emergency care.
- No requirement to choose a doctor as your primary physician.
- No referral needed to see a specialist. However, you might need preauthorization for certain services.
- Generic drugs: \$15 copay at an in-network retail pharmacy for a 30-day supply.
- Calendar year deductible of \$200 for Self Only enrollment and \$400 Self Plus One or Self and Family enrollment.
- 20% Coinsurance of our allowance after the deductible for certain inpatient and outpatient services.
- Predictable out-of-pocket copayments for many in-network healthcare services:
 - Telehealth visits at \$0, geha.com/Telehealth-Visit.
 - MinuteClinic® copay of \$10/visit.
 - Office visit copay of \$30 to any primary care provider including family or general practitioners, pediatricians, OB/GYN and medical internists; specialist provider copay is \$50.
 - Urgent Care Facility copay of \$50.

Elevate

- Extensive provider network and freedom to choose any doctor with extra savings when you see a preferred provider.
- No requirement to choose a doctor as your primary physician.
- No referral needed to see a specialist. However, you might need preauthorization for certain services.
- Generic drugs: \$10 copay at an in-network retail pharmacy for a 30-day supply. This plan does not include a benefit for mail order.
- Low-cost access to a wide variety of in-network providers for wellness and acute care visits:
 - Telehealth visits at \$0, geha.com/Telehealth-Visit.
 - Copay of \$10/visit for:
 - MinuteClinic®
 - Primary care provider including family or general practitioners, pediatricians, OB/GYN and medical internists
 - Chiropractic and acupuncture services
 - Specialist office visit copay is \$30.
 - Urgent Care Facility copay of \$50.

Section 5(a). Medical Services and Supplies Provided by Physicians and Other Healthcare Professionals

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
 - The in-network benefits are the standard benefits of the Plan.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). Applicable coinsurance applies after you met your Deductible until you've reached your Out-of-Pocket Maximum. We added "(no deductible)" to show when the calendar year deductible does not apply.
 - In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply.
- When you visit an in-network facility, the professionals who provide services to you in a facility may not all be in-network providers. If you receive out-of-network services at an in-network facility, we will pay up to the plan allowance according to the No Surprises Act.
- We will provide in-network benefits if you are admitted to an out-of-network hospital due to a medical emergency. We will also provide in-network benefits for professionals who provide services in a non-network hospital, when admitted due to a medical emergency.
- Medications may be available under the Prescription drug benefit and may require prior authorization.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- **YOU MUST GET PREAUTHORIZATION FOR SOME SERVICES.** Please refer to the preauthorization information shown in Section 3 to be sure which services require preauthorization.
- Be sure to read Section 4, Your Costs for Covered Services, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- The coverage and cost-sharing listed below are for services provided by physicians and other healthcare professionals for your medical care. See Section 5(c), for cost-sharing associated with the facility (i.e., hospital, surgical center, etc.).

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Note: The calendar year deductible applies to almost all benefits in this Section. We say “(No deductible)” when it does not apply.		
Diagnostic and treatment services	Elevate Plus	Elevate
Professional services of physicians • In physician's office • Office medical consultations • Second surgical opinions • Advance care planning • At home • Telehealth services provided by your physician Note: If additional services (e.g., lab, x-ray) are provided during your office visit, separate cost shares may apply.	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
• During a hospital stay Note: Services not related to your inpatient stay, may apply separate cost share.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
MinuteClinic® MinuteClinic® is available in several states and the District of Columbia. Walk-in medical clinics are located inside select CVS pharmacy locations and no appointment is necessary. MinuteClinic® is staffed by certified family nurse practitioners and physician assistants who diagnose, treat and write prescriptions for common illnesses, injuries and skin conditions. MinuteClinic® also offers physical exams, routine vaccinations and screenings for disease monitoring. To locate a MinuteClinic®, visit https://www.cvs.com/minuteclinic/clinic-locator or call 866-389-2727.	\$10 copayment for office visit (no deductible) Note: Other services rendered may take additional cost share.	\$10 copayment for office visit (no deductible) Note: Other services rendered may take additional cost share.
Telehealth services	Elevate Plus	Elevate
Telehealth professional services through the Plan's administrative partner for: • Minor acute conditions, see Section 10, <i>Definitions of Terms</i> • Dermatology conditions, see Section 10, <i>Definitions of Terms</i> Note: For telehealth services provided by your physician, see Section 5(a), <i>Diagnostic and treatment services</i>. Note: • For more information on telehealth vendor, see Section 5(h), <i>Wellness and Other Special Features</i>, or visit www.geha.com/Telehealth-Visit.	Nothing (no deductible)	Nothing (no deductible)

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Lab, x-ray and other diagnostic tests	Elevate Plus	Elevate
Tests, such as but not limited to: • Blood tests • Urinalysis • Non-routine Pap tests • Pathology	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Tests, such as but not limited to: • X-rays • Non-routine mammogram • Ultrasound • Electrocardiogram and EEG • Neurological testing	In-network: \$50 copayment (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Diagnostic tests, such as but not limited to: • Double-contrast barium enemas • Bone density tests • Diagnostic genetic testing and screening • Sleep studies (Polysomnography) Notes: • Preauthorization may be required for these tests • See Section 5(c) for any applicable outpatient facility charges.	In-network: 20% of the Plan allowance (deductible applies) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance (deductible applies) Out-of-network: 50% of the Plan allowance (deductible applies) and any difference between our allowance and the billed amount
Tests such as, but not limited to: • CT/CAT scan • MRI • Nuclear Cardiology • PET scan Notes: • Preauthorization is required for these tests. • See Section 5(c) for any applicable outpatient facility charges.	In-network: \$100 copayment (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Professional fees for automated lab tests.</i> • <i>Physical, psychiatric, or psychological exams and testing required for obtaining or continuing employment or insurance, attending schools or camps, sports physicals, travel, related to judicial or administrative proceedings or orders, or required to obtain or maintain a license of any type.</i> • <i>Home test kits including but not limited to HIV and drug home test kits, except as specified by the brochure.</i> • <i>Testing ordered by or on behalf of third parties (e.g., schools, courts, employers, etc.).</i>	<i>All charges</i>	<i>All charges</i>

Benefits Description	You pay After the calendar year deductible...	
Preventive care, adult	Elevate Plus	Elevate
Routine physical every year. The following preventive services are covered at the time intervals recommended at each of the links below. Please consult with your healthcare provider to determine if the services below are appropriate for you: • U.S. Preventive Services Task Force (USPSTF) A and B recommend screenings such as cancer, osteoporosis, depression, diabetes, high blood pressure, total blood cholesterol, HIV, and colorectal cancer. For a complete list of screenings, age and frequency limitations go to the website at uspreventiveservicestaskforce.org/uspstf/recommendation-topics/uspstf-a-and-b-recommendations • Individual counseling on prevention and reducing health risks • Preventive care benefits for women such as Pap smears, gonorrhea prophylactic medication to protect newborns, annual counseling for sexually transmitted infections, contraceptive methods, and screening for interpersonal and domestic violence. For a complete list of preventive care benefits for women go to the Health and Human Services (HHS) website at www.hrsa.gov/womens-guidelines • Routine mammogram - covered, including 3D mammograms - This coverage will include breast ultrasound performed after inconclusive breast cancer screening exam • Adult Immunizations endorsed by the Centers for Disease Control and Prevention (CDC): based on the Advisory Committee on Immunization Practices (ACIP) schedule. For a complete list of endorsed immunizations go to the Centers for Disease Control (CDC) website at https://www.cdc.gov/vaccines/imz-schedules/index.html • To build your personalized list of preventive services go to https://health.gov/myhealthfinder Note: Any procedure, injection, diagnostic service, laboratory, or x-ray service done in conjunction with a routine examination and is not included in the preventive recommended listing of services will be subject to the applicable member copayments, coinsurance, and deductible.	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Obesity counseling, screening, and referral, for those persons at or above the USPSTF obesity prevention risk factor level, to intensive nutrition and behavioral weight-loss therapy, counseling, or family centered programs under the USPSTF A and B recommendations are covered as part of prevention and treatment of obesity as follows: • Nutrition and behavioral weight-loss counseling therapy when ordered by your physician for obesity.	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Preventive care, adult - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Preventive care, adult (cont.)	Elevate Plus	Elevate
- Family centered programs when medically identified to support obesity prevention and management by an in-network provider. Programs must be ordered by a physician for treatment of your own obesity, for education and support of a family member with obesity. - Obesity screening for those persons below the USPSTF obesity prevention risk factor level. Notes: - Nutritional counseling, see Section 5(a), <i>Educational classes and programs</i>. - When anti-obesity medication is prescribed, see Section 5(f). - When Bariatric or Metabolic surgical treatment or intervention is indicated, see Section 5(b). - Also, for Weight Management, see <i>Wellness and Other Special Features</i>, Section 5(h).	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> <i>Professional fees for automated lab tests</i> <i>Physical, psychiatric, or psychological exams and testing required for obtaining or continuing employment or insurance, attending schools or camps, sports physicals, travel related to judicial or administrative proceedings or orders, or required to obtain or maintain a license of any type.</i> <i>Medications required for obtaining, continuing, or maintaining insurance, a license of any type, employment and/or work-related exposure, attending camps, sports physicals, or for travel; unless Section 5(a), Preventive Care coverage criteria are met.</i>	<i>All charges</i>	<i>All charges</i>
Preventive care, children	Elevate Plus	Elevate
- Well-child visits, examinations, and other preventive services as described in the Bright Future Guidelines provided by the American Academy of Pediatrics. For a complete list of the American Academy of Pediatrics Bright Futures Guidelines go to https://brightfutures.aap.org. - Children's immunizations endorsed by the Centers for Disease Control (CDC) including, DTaP/Tdap, Polio, Measles, Mumps, and Rubella (MMR), and Varicella. For a complete list of immunizations go to the website at www.cdc.gov/vaccines/imz-schedules/index.html. You may also find a complete list of U.S. Preventive Services Task Force (USPSTF) A and B recommendations online at https://uspreventiveservicestaskforce.org/uspstf/recommendation-topics/uspstf-a-and-b-recommendations. - Examinations, limited to: - Examinations for amblyopia and strabismus – limited to one screening examination (ages 3 through 5)	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Preventive care, children - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Preventive care, children (cont.)	Elevate Plus	Elevate
- Examinations done on the day of immunizations (ages 3 up to age 22) • To build your personalized list of preventive services go to https://health.gov/myhealthfinder. Notes: • For preventive medications for appropriate age and dosage limits, see Section 5(f) <i>Preventive care medications</i>, or visit geha.com/Prescriptions-Summary. • Any procedure, injection, diagnostic service, laboratory, or x-ray service done in conjunction with a routine examination that is not included in the preventive recommended listing of services will be subject to the applicable member copayments, coinsurance, and deductible.	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Obesity counseling, screening, and referral for those persons at or above the USPSTF obesity prevention risk factor level, to intensive nutrition and behavioral weight-loss therapy, counseling, or family centered programs under the USPSTF A and B recommendations are covered as part of prevention and treatment of obesity as follows: • Nutrition and behavioral weight-loss counseling therapy in children and adolescents age 6 years or older with BMI greater than or equal to 95th percentile on CDC growth charts for age and sex. • Family centered programs when medically identified to support obesity prevention and management by an in-network provider in children and adolescents age 6 years or older with BMI greater than or equal to 95th percentile on CDC growth charts for age and sex. • Obesity screening for those persons below the USPSTF obesity prevention risk factor. Notes: • Nutritional counseling, see Section 5(a), <i>Educational classes and programs</i>. • When anti-obesity medication is prescribed, see Section 5(f). • When Bariatric or Metabolic surgical treatment or intervention is indicated, see Section 5(b). • Also, for Weight Management program, see <i>Wellness and Other Special Features</i>, Section 5(h).	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Professional fees for automated lab tests.</i> • <i>Physical, psychiatric, or psychological exams and testing required for obtaining or continuing employment or insurance, attending schools or camps, sports physicals, travel, related to judicial or administrative proceedings or orders, or required to obtain or maintain a license of any type.</i>	<i>All charges</i>	<i>All charges</i>

Preventive care, children - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Preventive care, children (cont.)	Elevate Plus	Elevate
Medications required for obtaining, continuing, or maintaining insurance, a license of any type, employment and/or work-related exposure, attending camps, sports physicals, or for travel; unless Section 5(a), Preventive Care coverage criteria are met.	All charges	All charges
Maternity care	Elevate Plus	Elevate
Complete maternity (obstetrical) care, such as: - Prenatal and Postpartum care - Screening for gestational diabetes - Delivery - Bacteriuria screening - Screening and counseling for prenatal and postpartum depression - Breastfeeding and lactation support, supplies and counseling for each birth Notes: As part of your coverage, you have access to in-network certified nurse midwives and board-certified lactation specialists during the prenatal and post-partum period. Your coverage also includes services provided by a certified doula as outlined below. - You do not need to precertify your vaginal delivery. See Section 3, <i>How You Get Care/Maternity care</i> for other circumstances, such as extended stays for you or your baby. - You may remain in the hospital up to 48 hours after a vaginal delivery and 96 hours after a cesarean delivery. We will cover an extended stay if medically necessary. - We cover routine nursery care of a newborn child during the covered portion of the mother's maternity stay. We will cover other care of an infant who requires non-routine treatment if we cover the infant under Self Plus One or Self and Family enrollment. - We pay hospitalization and surgeon services for non-maternity care the same as for illness and injury. - Hospital services are covered under Section 5(c) and Section 5 (b), <i>Surgical benefits</i>. - Home nursing visits, intravenous/infusion therapy, and injections are covered the same as other medical benefits (not maternity) for diagnostic and treatment services as outlined in Section 5(a), <i>Home health services</i>. Note: When a newborn requires definitive treatment during or after the mother's hospital stay, the newborn is considered a patient in their own right. If the newborn is eligible for coverage, regular medical or surgical benefits apply rather than maternity benefits. In addition, circumcision is covered at the same rate as for regular medical or surgical benefits.	In-network: Nothing for preventive prenatal and postpartum care (no deductible) Out-of-network: You pay all charges	In-network: Nothing for preventive prenatal and postpartum care (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Maternity care - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Maternity care (cont.)	Elevate Plus	Elevate
• Maternity care expenses incurred by a Plan member serving as a surrogate are covered by the Plan subject to reimbursement from the other party according to the surrogacy contract or agreement. The involved Plan member must execute our Reimbursement Agreement against any payment that may be received under a surrogacy contract or agreement. Expenses of the newborn child are not covered under this or any other benefit in a surrogate mother situation. • Refer to Section 5(a), <i>Educational classes</i> and programs for information on Childbirth Education classes. • See Section 5(h), for more information on G.E.H.A's Family Planning Care Program. • Refer to Section 5(a), <i>Durable medical equipment (DME)</i>, for obtaining breast pump and supplies from a contracted provider. • See Section 5(e), for more information on <i>Mental Health and Substance Use Disorders</i> for treatment or call SAMHSA's National Helpline, 800-662-4357 or go to www.findtreatment.gov • For more information, visit geha.com/Healthy-Pregnancy.	In-network: Nothing for preventive prenatal and postpartum care (no deductible) Out-of-network: You pay all charges	In-network: Nothing for preventive prenatal and post-partum care (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
A doula is a non-medical trained professional who provides emotional, physical, and informational support during pregnancy, labor/delivery, and post-partum periods. See Section 10, <i>Definitions of Terms</i>, for additional information. Benefits are allowable for services of a certified doula providing support for pregnancy-related care. Coverage is limited to \$1,000 per pregnancy and must include in-person support during labor and delivery when pregnancy results in birth. Services provided by a certified doula limited to: • Prenatal visits • Labor and delivery support • Postpartum visits for up to one year following birth or cessation of pregnancy • Support during and after miscarriage, including bereavement support	All charges in excess of \$1,000 (no deductible)	All charges in excess of \$1,000 (no deductible)
<i>Not covered:</i> • <i>Home uterine monitoring devices</i> • <i>Services for birth coaching or labor support, except when provided by a certified doula. See Section 10, Definitions of Terms</i>	<i>All charges</i>	<i>All charges</i>

Benefits Description	You pay After the calendar year deductible...	
Family planning	Elevate Plus	Elevate
Contraceptive counseling on an annual basis. A range of voluntary family planning services, without cost sharing, that includes at least one form of contraception in each of the categories in the HRSA supported guidelines. This list includes: • Voluntary female sterilization • Surgically implanted contraceptives • Injectable contraceptive drugs (such as Depo Provera) • Intrauterine devices (IUDs) • Diaphragms For additional Family Planning and Prescription drug coverage, see Section 5(f). Note: Your plan offers some type of voluntary female sterilization surgery coverage at no cost to members. The contraceptive benefit includes at least one option in each of the HRSA-supported categories of contraception (as well as the screening, education, counseling, and follow-up care). Any type of voluntary female sterilization surgery that is not already available without cost sharing can be accessed through the contraceptive exceptions process described below. If you have difficulty accessing contraceptive coverage or other reproductive healthcare, you can contact contraception@opm.gov.	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Voluntary male sterilization	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Reversal of voluntary surgical sterilizations</i> • <i>Genetic testing and counseling</i>	<i>All charges</i>	<i>All charges</i>
Infertility services	Elevate Plus	Elevate
Infertility definition can be found in Section 10, <i>Definitions of Terms</i>. Diagnosis and treatment of infertility specific to: • Artificial insemination (AI) • Intravaginal insemination (IVI) • Intracervical insemination (ICI) • Intrauterine insemination (IUI)	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Infertility services - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Infertility services (cont.)	Elevate Plus	Elevate
Fertility preservation for iatrogenic infertility: • Procurement of sperm or eggs including medical, surgical, and pharmacy claims associated with retrieval; • Cryopreservation of sperm and mature oocytes; and • Cryopreservation storage costs for 12 months. Notes: • All Infertility and related services within this section, Preauthorization is required. See Section 3, <i>Other services that require preauthorization.</i> • See Section 5(b), <i>Surgical procedures</i> for cost share associated with covered surgical services. • Fertility drugs (See Section 5(f), Prescription Drug Benefits, Plan limits apply.)	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance (deductible applies) and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Assisted reproductive technology (ART), See Section 10, Definitions of Terms.</i> - <i>In vitro fertilization (IVF), See Section 10, Definitions of Terms.</i> - <i>Embryo transfer and gamete intrafallopian transfer (GIFT) and zygote intrafallopian transfer (ZIFT)</i> • <i>Charges for gestational carrier or surrogacy, including antenatal appointments and labor/delivery services</i> • <i>Charges for procedures to collect, analyze, manipulate, or otherwise treat gametes (sperm and ova) when the partner or donor who produces the gamete is not a covered patient on the plan</i> • <i>Collection of unfertilized eggs/sperm and storage of fertilized and/or unfertilized eggs/sperm except in cases of iatrogenic infertility</i> • <i>Cost of donor egg</i> • <i>Cost of donor sperm</i> • <i>Elective preservation for reasons other than listed above, such as egg freezing sought due to natural aging</i> • <i>Fertility drugs, provided by facilities or physicians, including ovulation induction cycles while on injectable medication to stimulate the ovaries. Fertility drugs must be obtained through the pharmacy benefits, see Section 5(f), Prescription Drug Benefits and Specialty Drug Benefits. Medications will not be covered when dispensed by other sources, including physician offices, home health agencies and outpatient hospitals.</i> • <i>Genetic counseling</i> • <i>Infertility services after voluntary sterilizations</i> • <i>Preimplantation diagnosis, testing, and/or screening of eggs, sperm, or embryos</i>	<i>All charges</i>	<i>All charges</i>

Infertility services - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Infertility services (cont.)	Elevate Plus	Elevate
• Reversal of voluntary surgical sterilizations • Services and supplies related to non-covered ART procedures • Treatments such as artificial insemination, assisted reproductive technology, and/or in vitro fertilization prior to establishing diagnosis of infertility. See Section 10, <i>Definitions of Terms</i>	All charges	All charges
Allergy care	Elevate Plus	Elevate
• Testing and treatment, including materials (such as allergy serum) • Allergy injections	In-network: \$30 copayment for primary care providers (no deductible); \$50 copayment for specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for primary care providers (no deductible); \$30 copayment for specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • Clinical ecology and environmental medicine • Provocative food testing • Sublingual allergy desensitization drugs	All charges	All charges
Treatment therapies	Elevate Plus	Elevate
• Intravenous (IV)/Infusion Therapy - Home IV/infusion and antibiotic therapy - See Section 5(c) for Outpatient therapy services • Total Parenteral Nutrition (TPN) • Intrathecal pump refills • Enteral/Tube feeding nutrition, including Medical Foods for Inborn Errors of Metabolism (IEM). See Section 10, <i>Definitions of Terms</i>. • Chemotherapy and radiation therapy - Preauthorization is required. See Section 3, <i>Other services that require preauthorization</i>. Note: High-dose chemotherapy in association with autologous bone marrow transplants is limited to those transplants listed in Section 5(b), <i>Surgical and Anesthesia Services</i> and Section 5 (f), <i>Prescription Drug Benefits</i>. • Respiratory and inhalation therapies • Growth hormone therapy (GHT)	In-network: \$30 copayment for primary care providers (no deductible); \$50 copayment for specialists (no deductible) Out-of-network: You pay all charges You pay 10% of the Plan allowance for drugs and/or medical nutrition administered or obtained in connection with your treatment (excludes drugs obtained under the <i>Prescription drug benefit</i> in Section 5(f)).	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Treatment therapies - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Treatment therapies (cont.)	Elevate Plus	Elevate
Note: Growth hormone therapy (GHT) is covered under the prescription drug benefit. We only cover GHT when we preauthorize the treatment. Call 800-821-6136 for preauthorization. We will ask you to submit information that establishes GHT is medically necessary. Ask us to authorize GHT before you begin treatment; otherwise, we will only cover GHT services from the date you submit the information. If you do not ask or if we determine GHT is not medically necessary, we will not cover the GHT or related services and supplies. See <i>Other services that require preauthorization</i> in Section 3. Notes: • Most medications required for treatment therapies are available under Section 5(f), <i>Specialty drug benefits</i>. • Applied Behavioral Analysis Therapy is available under the <i>Mental Health and Substance Use Disorder Benefits</i> in Section 5(e). • Outpatient therapy (including cardiac and pulmonary rehabilitation). See Section 5(c), <i>Outpatient hospital services</i> for applicable outpatient facility charges.	In-network: \$30 copayment for primary care providers (no deductible); \$50 copayment for specialists (no deductible) Out-of-network: You pay all charges You pay 10% of the Plan allowance for drugs and/or medical nutrition administered or obtained in connection with your treatment (excludes drugs obtained under the <i>Prescription drug benefit</i> in Section 5(f)). Note: Specialty drugs obtained outside of the pharmacy benefit from other sources (physician offices, home health agencies, outpatient hospitals) may be subject to additional cost share as outlined in Section 5(f), <i>Specialty drug benefits</i>.	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Dialysis • Dialysis - hemodialysis and peritoneal dialysis • Initial home dialysis training for the member and a helper is covered. Notes: • Notification for all dialysis requests is required, even when an extension is being requested. Refer to the dialysis notification form located at geha.com/Dialysis-Notification. • See Section 5(f), <i>Prescription drug benefits</i>, for available medications required for treatment therapies.	In-network: \$30 copayment for primary care providers (no deductible); \$50 copayment for specialists (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Chelation therapy except for acute arsenic, gold or lead poisoning</i> • <i>"Grocery" food items that can routinely be obtained online or in stores (e.g., gluten-free breads)</i> • <i>Maintenance cardiac and pulmonary rehabilitation</i> • <i>Topical hyperbaric oxygen therapy</i> • <i>Prolotherapy</i> • <i>On-going licensed/unlicensed dialysis assistance in the home after initial dialysis training</i>	<i>All charges</i>	<i>All charges</i>

Benefits Description	You pay After the calendar year deductible...	
Physical, occupational, speech, habilitative and rehabilitative therapy	Elevate Plus	Elevate
Outpatient therapy visits are covered, per person, per calendar year for the combined services when a physician: • orders the care; • identifies the specific professional skills the patient requires and the medical necessity for skilled services; and • indicates the length of time the services are needed. Notes: • Inpatient therapy services are not applied to the visit limits, see Section 5(c), <i>Inpatient hospital</i> • Rehabilitative and Habilitative therapy, see Section 10, <i>Definitions of Terms</i>	In-network: \$30 copayment per visit for primary care providers (no deductible); \$50 copayment per visit for specialists (no deductible), for up to 60 therapy visits Out-of-network: You pay all charges	In-network: \$10 copayment per visit for primary care providers (no deductible); \$30 copayment per visit for specialists (no deductible), for up to 30 therapy visits Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Exercise programs</i> • <i>Long-term rehabilitation therapy</i> • <i>Maintenance therapy-measurable improvement is not expected, or progress is no longer demonstrated</i> • <i>Computers, tablets, wearable or digital devices, software or application programs (“apps”, including phone apps), and subscriptions are not covered (even if ordered by a doctor) unless explicitly stated as a covered service or as offered through participation in a clinical program.</i> • <i>Hot and cold packs</i> • <i>Hippotherapy</i> • <i>Sensory, Auditory, or Sensory Integration Therapy</i> • <i>Biofeedback, educational, recreational or milieu therapy</i>	<i>All charges</i>	<i>All charges</i>
Cognitive rehabilitation	Elevate Plus	Elevate
Provided when medically necessary following brain injury or traumatic brain injury. Services will only be covered when provided by the following while practicing within their scope of care: • Speech, occupational and/or physical therapists • Psychologists • Physicians	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Hearing services (testing, treatment, and supplies)	Elevate Plus	Elevate
- For treatment related to illness or injury, including evaluation and diagnostic hearing tests performed by an M.D., D.O., or audiologist. - Implanted hearing-related devices, such as bone anchored hearing aids Notes: - For routine hearing screening performed during a child's preventive care visit, see Section 5(a), <i>Preventive care, children</i>. - For benefits for the devices, see Section 5(a), <i>Orthopedic and prosthetic devices</i>	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
External hearing aids Note: See <i>Non-FEHB Benefits Available to Plan Members</i> for additional hearing aid discount program information.	All charges in excess of \$1,500 (no deductible) Payable per person every 36 months for adults, and every 12 months for children up to age 22.	<i>All charges</i>
<i>Not covered:</i> <i>Hearing services that are not shown as covered</i> <i>Over-the-counter hearing aids, enhancement devices, accessories or supplies</i>	<i>All charges</i>	<i>All charges</i>
Vision services (testing, treatment, and supplies)	Elevate Plus	Elevate
One pair of eyeglasses or contact lenses to correct an impairment directly caused by accidental ocular injury or intraocular surgery (such as for cataracts)	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
- Diagnosis and treatment of diseases of the eye - Outpatient vision therapy for treatment of convergence insufficiency up to a maximum of 24 visits per year for ages 5-18	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> <i>Eyeglasses or contact lenses and examinations for them, except as shown above</i> <i>Radial keratotomy and other refractive surgeries</i> <i>Special multifocal ocular implant lenses</i>	<i>All charges</i>	<i>All charges</i>

Vision services (testing, treatment, and supplies) - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Vision services (testing, treatment, and supplies) (cont.)	Elevate Plus	Elevate
- Computers, tablets, wearable or digital devices, software or application programs (“apps”, including phone apps), and subscriptions are not covered (even if ordered by a doctor) unless explicitly stated as a covered service or as offered through participation in a clinical program. - Vision therapy except as noted above - Eye exercises and orthoptics	All charges	All charges
Foot care	Elevate Plus	Elevate
Routine foot care only when you are under active treatment for a metabolic or peripheral vascular disease, such as diabetes	In-network: \$30 copayment for office visits to primary care providers (no deductible); \$50 copayment for office visits to specialists (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible); \$30 copayment for office visits to specialists (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Diabetic shoes and shoe inserts individually designed and fitted to offload pressure points on the diabetic foot	In-network: All charges in excess of \$150 (no deductible) Out-of-network: You pay all charges	All charges in excess of \$150 (no deductible) for both in- and out-of-network
<i>Not covered:</i> - Cutting, trimming, or removal of corns, calluses, or the free edge of toenails, and similar routine treatment of conditions of the foot, except as stated above - Treatment of weak, strained or flat feet or bunions or spurs; and of any instability, imbalance or subluxation of the foot (except for surgical treatment)	All charges	All charges
Orthopedic and prosthetic devices	Elevate Plus	Elevate
- Artificial limbs and eyes - Orthopedic braces - Prosthetic sleeve or sock - Externally worn breast prostheses and surgical bras, including necessary replacements following a mastectomy - Corrective orthopedic appliances for non-dental treatment of temporomandibular joint (TMJ) pain dysfunction syndrome - Internal prosthetic devices, such as artificial joints, pacemakers, and surgically implanted breast implant following mastectomy - Bioelectric, computer programmed prosthetic devices	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Orthopedic and prosthetic devices - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Orthopedic and prosthetic devices (cont.)	Elevate Plus	Elevate
- Implanted hearing-related devices, such as bone anchored hearing aids and cochlear implants Notes: - For information on the professional charges for the surgery to insert an implant, see Section 5(b), <i>Surgical procedures</i>. For information on the hospital and/or ambulatory surgery center benefits, see Section 5(c), <i>Services provided by a Hospital or Other Facility, and Ambulance Services</i>. - We will pay only for the cost of the standard item. Coverage for specialty items is limited to the cost of the standard item. Preauthorization is required, see Section 3, <i>Other services that require preauthorization</i>, for orthopedic and prosthetic devices with a retail price of \$1,000 or more.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> <i>Orthopedic and corrective shoes, arch supports, foot orthotics, heel pads and heel cups</i> <i>Over-the-counter hearing aids, enhancement devices, accessories or supplies</i> <i>Lumbosacral supports</i> <i>Corsets, trusses, elastic stockings, support hose, and other supportive devices</i>	<i>All charges</i>	<i>All charges</i>
Durable medical equipment (DME)	Elevate Plus	Elevate
We cover rental or purchase of prescribed durable medical equipment, at our option, including repair and adjustment. Covered items include: - Oxygen - Certain medical supplies - Rental of dialysis equipment - Standard hospital beds - Standard wheelchairs - Crutches - Walkers - Continuous Positive Airway Pressure (CPAP) machine - Braces including necessary adjustments to shoes to accommodate braces, which are used for the purpose of supporting a weak or deformed body part - Braces restricting or eliminating motion in a diseased or injured part of the body - Bone growth stimulators Notes:	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Durable medical equipment (DME) - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Durable medical equipment (DME) (cont.)	Elevate Plus	Elevate
• Call us at 800-821-6136 as soon as your physician prescribes this equipment. We arrange with a healthcare provider to rent or sell you durable medical equipment at discounted rates and will tell you more about this service when you call. • Preauthorization is required, See Section 3, <i>Other services that require preauthorization</i>, for Durable Medical Equipment that has a cumulative rental and/or retail purchase price of \$1,000 or more. • See Section 10, <i>Definitions of Terms</i> for Medical Necessity • Refer to Section 5(f), <i>Prescription drugs/Covered medications and supplies</i>, to obtain glucose meter and diabetic supplies.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Breast pump and supplies: • One personal use, double channel electric breast pump with double suction capability is purchased for pregnant or nursing members every 12-months with birth/delivery. A prescription is required when requesting a pump. - An initial all-inclusive supply kit is provided with a new pump order. Replacement supplies and supply kits are allowed when necessary for pump operation. - There is no cost to the member when the designated pump is obtained through a contracted provider. For more information visit geha.com/Healthy-Pregnancy. Note: Refer to Section 5(a), <i>Maternity Care</i>, for information on Breastfeeding support and counseling.	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Speech Generating Devices Notes: • For definition, see <i>Definitions of Terms</i>, Section 10. • Preauthorization is required, See Section 3, <i>Other services that require preauthorization</i> • Used for patients suffering from severe expressive speech disorders and have a medical condition that warrants the use of such device • Requires a formal speech and language evaluation by licensed speech therapist	In-network: All charges in excess of \$1,250 per calendar year (no deductible) Out-of-network: You pay all charges	All charges in excess of \$1,250 per calendar year (no deductible), in- and out-of-network
Wigs/cranial hair prosthesis used for hair loss due to the treatment of cancer. Note: One wig/cranial hair prosthesis per lifetime.	All charges in excess of \$350 (no deductible), in- and out-of-network	All charges in excess of \$350 (no deductible), in- and out-of-network
<i>Not covered:</i> • <i>Motorized wheelchairs and other power operated vehicles unless meeting ACA requirements and medical necessity</i> • <i>Deluxe or upgraded equipment and supplies</i>	<i>All charges</i>	<i>All charges</i>

Durable medical equipment (DME) - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Durable medical equipment (DME) (cont.)	Elevate Plus	Elevate
• Air purifiers, air conditioners, heating pads, cold therapy units, whirlpool bathing equipment, sun and heat lamps, exercise devices (even if ordered by a doctor), and other equipment that does not meet the definition of durable medical equipment (see Section 10, Definitions of Terms) • Lifts, such as seat, chair, hydraulic, or van lifts • Replacement of the wig/cranial hair prosthesis, maintenance and supplies • Hair transplants or surgical procedures that involve the attachment of hair or a wig/cranial hair prosthesis to the scalp • Devices or programs to eliminate bed wetting • If a member is a patient in a facility other than the member's primary residence, or in a distinct part of a facility that provides services such as skilled nursing, rehabilitation services, or provides medical or nursing, DME will not be covered separately for rental or purchase.	All charges	All charges
Home health services	Elevate Plus	Elevate
Services are limited to 50 visits per calendar year as defined by service type billed by the home health agency. Notes: • Medically necessary part-time or intermittent skilled nursing care by a licensed Home Health agency, when ordered by an appropriate medical professional, for services such as: - Wound care for pressure sores or a surgical wound - Patient and caregiver education - Intravenous or nutrition therapy - Injections - Monitoring serious illness and unstable health status • Please refer to the Specialty drug benefits in Section 5(f), <i>Prescription Drug Benefits</i>, for information on benefits for home infusion therapy medications. • See <i>Durable medical equipment (DME)</i> above for coverage of DME benefit services received in the home.	In-network: \$30 copayment (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • Nursing care requested by, or for the convenience of, the patient or the patient's family • Services primarily for hygiene, feeding, exercising, moving the patient, homemaking, companionship or giving oral medications • Home care primarily for personal assistance that does not include a medical component and is not diagnostic, therapeutic, or rehabilitative • Custodial care (see Section 10, Definitions of Terms) • Long-term care (see Section 10, Definitions of Terms)	All charges	All charges

Home health services - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Home health services (cont.)	Elevate Plus	Elevate
- Services or supplies furnished by immediate relatives or household members, such as spouse, parents, children, brothers or sisters by blood, marriage or adoption - Private duty nursing	All charges	All charges
Chiropractic services	Elevate Plus	Elevate
Chiropractic services limited to: - Chiropractic spinal and extremities manipulative treatment - Adjunctive procedures such as ultrasound, electrical muscle stimulation, and vibratory therapy - X-rays used to detect and determine nerve interferences due to spinal subluxations or misalignments	In-network: \$30 copayment (no deductible), for up to 15 visits, per person, per calendar year Out-of-network: You pay all charges	In-network: \$10 copayment (no deductible), for up to 12 visits, per person, per calendar year Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> - Any treatment not specifically listed as covered, including acupressure, aroma therapy, biofeedback, clinical ecology, cupping, dry needling, environmental medicine, hypnotherapy, massage therapy, cold pack application, naturopathic services and rolfing - Maintenance therapy - measurable improvement is not expected or progress is no longer demonstrated	All charges	All charges
Alternative treatments	Elevate Plus	Elevate
Acupuncture Benefits are limited to 20 visits per person, per calendar year for medically necessary acupuncture treatments by a Doctor of Medicine or osteopathy, or licensed or certified acupuncture practitioner	In-network: \$30 copayment (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> - Any treatment not specifically listed as covered, including acupressure, aroma therapy, biofeedback, clinical ecology, cupping, dry needling, environmental medicine, hypnotherapy, massage therapy, naturopathic services, and rolfing. - Services provided by Christian Science practitioners or facilities	All charges	All charges

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible...	
Educational classes and programs	Elevate Plus	Elevate
Coverage is limited to: - Tobacco Cessation programs - We cover counseling sessions including, proactive phone counseling, group counseling and individual counseling. Benefits are payable for up to two attempts per person, per calendar year, with up to four counseling sessions per attempt. - In addition, we cover over-the-counter (with a physician's prescription) and prescription tobacco cessation drugs approved by the FDA to treat tobacco dependence. Note: The quantity of drugs reimbursed will be subject to recommended courses of treatment. You may obtain tobacco cessation drugs with your plan identification card, through a participating network retail Pharmacy or Elevate Plus members may obtain through CVS Caremark Mail Service Pharmacy. (See filing instructions in Section 5(f), <i>Prescription drug benefits</i>.)	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: Nothing, except any difference between our Plan allowance and the billed amount (no deductible)
Diabetes self-management The following program criteria needs to be met: - Consists of services by healthcare professionals (physicians, registered dietitians, registered nurses, registered pharmacists); - Designed to educate the member about medically necessary diabetes self-care upon initial diagnosis	In-network: Nothing up to the Plan allowance (no deductible) Out-of-network: You pay all charges	In-network: Nothing up to the Plan allowance (no deductible) Out-of-network: Nothing up to the Plan allowance and any difference between our allowance and the billed amount (no deductible)
Nutritional counseling Note: Provided by a dietitian with a state license or statutory certification. Nutritional counseling must be ordered by a physician.	In-network: Nothing up to the Plan allowance (no deductible) Out-of-network: You pay all charges	In-network: Nothing up to the Plan allowance (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Childbirth education classes One series of childbirth education classes per pregnancy, only when provided by a covered provider, see Section 3, <i>How You Get Care/Plan providers</i>. For more information visit geha.com/Healthy-Pregnancy.	In-network: All charges in excess of \$150 (no deductible) Out-of-network: You pay all charges	All charges in excess of \$150 (no deductible) for both in- and out-of-network
<i>Not covered:</i> <i>Weight loss programs, except as specified by the brochure</i>	<i>All charges</i>	<i>All charges</i>

Section 5(b). Surgical and Anesthesia Services Provided by Physicians and Other Healthcare Professionals

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
 - The in-network benefits are the standard benefits of the Plan.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). Applicable coinsurance applies after you met your Deductible until you've reached your Out-of-Pocket Maximum. We added "(no deductible)" to show when the calendar year deductible does not apply.
 - In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply.
- When you visit an in-network facility, the professionals who provide services to you may not all be in-network providers. If you receive out-of-network services at an in-network facility, we will pay up to the Plan allowable according to the No Surprises Act.
- Medications may be available under the Prescription drug benefit and may require prior authorization.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy under the pharmacy benefit even if you obtain these medications from your provider.
- **YOU MUST GET PREAUTHORIZATION FOR SOME SERVICES.** Please refer to the preauthorization information shown in Section 3 or call customer service to be sure which services require preauthorization.
- Be sure to read Section 4, Your Costs for Covered Services, for valuable information about how cost-sharing works. Also read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- The coverages and cost-sharing listed below are for the charges billed by a physician or other healthcare professional for your surgical care. See Section 5(c) for charges associated with a facility (i.e., hospital, surgical center, etc.).

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible	
Note: The calendar year deductible applies to almost all benefits in this Section. We say “(No deductible)” when it does not apply.		
Surgical procedures	Elevate Plus	Elevate
A comprehensive range of services, such as • Operative procedures • Treatment of fractures, including casting • Routine pre- and post-operative care by the surgeon • Correction of amblyopia and strabismus • Endoscopy and non-routine colonoscopy procedures • Biopsy procedures • Removal of tumors and cysts • Correction of congenital anomalies - limited to children under the age of 18 unless there is a functional deficit (see <i>Reconstructive surgery</i> below) • Insertion of internal prosthetic devices (see Section 5(a), <i>Orthopedic and prosthetic devices</i> for device coverage information) • Treatment of burns • Assistant surgeons are covered up to 20% of our allowance for the surgeon's charge for procedures medically necessary. Registered nurse first assistants and certified surgical assistants are covered up to 15% of our allowance if medically necessary. • Surgical treatment of severe obesity (bariatric surgery). Preauthorization is required, see Section 3, <i>Other services that require preauthorization</i>. See geha.com/Provider-Partner-Tools for additional information on policy and criteria. When multiple surgical procedures performed during the same operative session add time or complexity to patient care, our benefits are available on our website at www.geha.com/Provider-Policies. Note: Multiple surgical procedures performed through the same incision are “incidental” to the primary surgery. That is, the procedure would not add time or complexity to patient care. We do not pay extra for incidental procedures. Note: For female and male surgical family planning procedures, see Section 5(a), <i>Family Planning</i>.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance, for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Reversal of voluntary sterilization</i> • <i>Routine treatment of conditions of the foot (see Section 5 (a), Foot care)</i> • <i>Surgical treatment of hyperhidrosis unless alternative therapies such as botox injections or topical aluminum chloride and pharmacotherapy have been unsuccessful</i> • <i>Removal of excess skin unless medically necessary</i>	<i>All charges</i>	<i>All charges</i>

Benefits Description	You pay After the calendar year deductible	
Reconstructive surgery	Elevate Plus	Elevate
• Surgery to correct a functional defect • Surgery to correct a condition caused by injury or illness if: - the condition produced a major effect on the member's appearance; and - the condition can reasonably be expected to be corrected by such surgery • Surgery to correct a condition that existed at or from birth and is a significant deviation from the common form or norm. Examples of congenital anomalies are: cleft lip; cleft palate; birth marks; and webbed fingers and toes. • Surgeries to correct congenital anomalies for individuals age 18 and older unless there is a functional deficit Note: Covered for children under the age of 18 regardless of functional deficit. • All stages of breast reconstruction surgery following a mastectomy or lumpectomy, such as: - surgery to produce a symmetrical appearance of breasts - treatment of any physical complications, such as lymphedemas - breast prostheses; and surgical bras and replacements (see Section 5(a), <i>Orthopedic and prosthetic devices</i> for coverage) Notes: • We pay for internal breast prostheses as hospital benefits if billed by a hospital. If included with the surgeon's bill, surgery benefits will apply. • If you need a mastectomy, you may choose to have the procedure performed on an inpatient basis and remain in the hospital up to 48 hours after the procedure. • Preauthorization is required, see Section 3, <i>Other services that require preauthorization</i>.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance, for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Cosmetic surgery – any surgical procedure (or any portion of a procedure) performed primarily to improve physical appearance through change in bodily form, except repair of accidental injury, see section 5(d), Accidental injury.</i> • <i>Surgeries related to sexual dysfunction</i> • <i>Surgery for Sex-Trait Modification to treat gender dysphoria</i>	<i>All charges</i>	<i>All charges</i>

Reconstructive surgery - continued on next page

Benefits Description	You pay After the calendar year deductible	
Reconstructive surgery (cont.)	Elevate Plus	Elevate
<i>If you are mid-treatment under this Plan, within a surgical or chemical regimen for Sex-Trait Modification for diagnosed gender dysphoria, for services for which you received coverage under the 2025 Plan brochure, you may seek an exception to continue care for that treatment. Ongoing sex-trait modification hormone treatment is not a covered benefit under this plan as of 1/1/2026. Our exception process is as follows: To initiate the exception process, contact Customer Care at 800-821-6136, Monday-Friday, 8 a.m. - 8 p.m. E.T. If you disagree with our decision on your exception, please see Section 8 of this brochure for the disputed claims process.</i> <i>Individuals under age 19 are not eligible for exceptions related to services for ongoing surgical or hormonal treatment for diagnosed gender dysphoria.</i>	All charges	All charges
Oral and maxillofacial surgery	Elevate Plus	Elevate
Oral surgical procedures, limited to: • Reduction of fractures of the jaws or facial bones • Surgical correction of cleft lip, cleft palate, and severe functional malocclusion • Excision of cysts and incision of abscesses unrelated to tooth structure • Extraction of impacted (unerupted or partially erupted) teeth • Partial or radical removal of the lower jaw with bone graft • Excision of tori, tumors, leukoplakia, premalignant and malignant lesions, and biopsy of hard and soft oral tissues when unrelated to teeth and supporting structures • Open reduction of dislocations and excision, manipulation, aspiration or injection of temporomandibular joints • Removal of foreign body, skin, subcutaneous areolar tissue, reaction-producing foreign bodies in the musculoskeletal system and salivary stones and incision/excision of salivary glands and ducts • Repair of traumatic wounds • Incision of the sinus and repair of oral fistulas • Surgical treatment of trigeminal neuralgia • Orthognathic surgery for the following conditions only: - Moderate or severe sleep apnea only after conservative treatment of sleep apnea has failed - Craniofacial congenital anomalies - Severe functional malocclusion not able to be corrected by conservative treatment options - Orthognathic procedures used for reconstruction following injury or illness causing a functional deficit • Frenectomy, frenotomy, or frenuloplasty when the patient has a functional deficit unrelated to teeth and their supporting structures • Other oral surgery procedures that do not involve the teeth or their supporting structures	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible	
Oral and maxillofacial surgery (cont.)	Elevate Plus	Elevate
Notes: - Accidental dental injury benefit is covered see Section 5(g), <i>Dental benefits</i>. Preauthorization is required, see Section 3, <i>Other services that require preauthorization</i>.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> <i>Oral implants and transplants; including for the treatment of accidental injury</i> <i>Procedures that involve the teeth or their supporting structures (such as the periodontal membrane, gingiva, and alveolar bone) including removal of tori for placement of dentures</i> <i>Orthodontic treatment</i> <i>Any oral or maxillofacial or orthognathic surgery not specifically listed as covered</i> <i>Apicoectomy, Alveolectomy, root canal therapy</i>	<i>All charges</i>	<i>All charges</i>
Organ/tissue transplants	Elevate Plus	Elevate
These solid organ and tissue transplants are covered and limited to: - Allogeneic islet - Autologous pancreas islet cell transplant (as an adjunct to total or near total pancreatectomy) only for patients with chronic pancreatitis - Cornea - Heart - Heart/Lung - Intestinal transplants - Isolated small intestine - Small intestine with the liver - Small intestine with multiple organs, such as the liver, stomach, and pancreas - Kidney - Kidney/Pancreas - Liver	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Organ/tissue transplants - continued on next page
Elevate Plus and Elevate Options Section 5(b)

Elevate Plus and Elevate Options

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
• Lung single/bilateral/lobar • Pancreas Note: All transplants and related services within this section, Preauthorization is required. See Section 3, <i>Refer to Other services that require preauthorization</i>.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
These tandem blood or marrow stem cell transplants for covered transplants are subject to medical necessity review by the Plan. • Autologous tandem transplants for: - AL Amyloidosis - Multiple myeloma (de novo and treated) - Recurrent germ cell tumors (including testicular cancer)	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Blood or marrow stem cell transplants: The Plan extends coverage for the diagnoses as indicated below. • Allogeneic transplants for: - Acute lymphocytic or non-lymphocytic (i.e., myelogenous) leukemia - Acute myeloid leukemia - Advanced Hodgkin's lymphoma with recurrence (relapsed) - Advanced Myeloproliferative Disorders (MPDs) - Advanced neuroblastoma - Advanced non-Hodgkin's lymphoma with recurrence (relapsed) - Amyloidosis - Beta Thalassemia Major - Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings

Organ/tissue transplants - continued on next page
Elevate Plus and Elevate Options Section 5(b)

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
- Chronic lymphocytic leukemia/small lymphocytic lymphoma (CLL/SLL) - Chronic myelogenous leukemia - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Hemoglobinopathy - Immune deficiency diseases other than Severe Combined Immunodeficiency (SCID, e.g., Wiskott-Aldrich syndrome, Kostmann's syndrome, Leukocyte Adhesion Deficiencies) not amendable to more conservative treatments - Infantile malignant osteopetrosis - Kostmann's syndrome - Leukocyte adhesion deficiencies - Marrow failure and related disorders (i.e., Fanconi's, Paroxysmal Nocturnal Hemoglobinuria, Pure Red Cell Aplasia) - Mucopolysaccharidosis (e.g., Hunter's syndrome, Hurler's syndrome, Sanfilippo's syndrome, Maroteaux-Lamy syndrome variants) - Mucopolysaccharidosis (e.g., Gaucher's disease, metachromatic leukodystrophy, adrenoleukodystrophy) - Mucopolysaccharidosis (e.g., Hunter's syndrome, Hurler's syndrome, Sanfilippo's syndrome, Maroteaux-Lamy syndrome variants) - Multiple myeloma - Multiple sclerosis - Myelodysplasia/Myelodysplastic syndromes - Myeloproliferative disorders - Paroxysmal Nocturnal Hemoglobinuria - Phagocytic/Hemophagocytic deficiency diseases (e.g., Wiskott-Aldrich syndrome) - Severe combined immunodeficiency - Severe or very severe aplastic anemia - Sickle cell anemia - X-linked lymphoproliferative syndrome • Autologous transplants for: - Acute lymphocytic or non-lymphocytic (i.e., myelogenous) leukemia - Advanced childhood kidney cancers - Advanced Ewing sarcoma - Advanced Hodgkin's lymphoma with recurrence (relapsed) - Advanced non-Hodgkin's lymphoma with recurrence (relapsed) - Aggressive non-Hodgkin's lymphomas (Mantle Cell lymphoma, adult T-cell leukemia/lymphoma, peripheral T-cell lymphoma and aggressive Dendritic Cell neoplasms) - Amyloidosis - Breast cancer	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
- Childhood rhabdomyosarcoma - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Ependyoblastoma - Epithelial ovarian cancer - Mantle Cell (non-Hodgkin lymphoma) - Medulloblastoma - Multiple myeloma - Multiple sclerosis - Neuroblastoma - Pineoblastoma - Scleroderma - Scleroderma - SSC (severe, progressive) - Systemic sclerosis - Testicular, mediastinal, retroperitoneal and ovarian germ cell tumors - Waldenstrom's macroglobulinemia	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Mini-transplants performed in a clinical trial setting (non-myoablative, reduced intensity conditioning or RIC) for members with a diagnosis listed below are subject to medical necessity review by the Plan. • Allogeneic transplants for: - Acute lymphocytic or non-lymphocytic (i.e., myelogenous) leukemia - Acute myeloid leukemia - Advanced Hodgkin's lymphoma with recurrence (relapsed) - Advanced Myeloproliferative Disorders (MPDs) - Advanced non-Hodgkin's lymphoma with recurrence (relapsed) - Amyloidosis - Chronic lymphocytic leukemia/small lymphocytic lymphoma (CLL/SLL) - Chronic myelogenous leukemia - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Hemoglobinopathy - Marrow failure and related disorders (i.e., Fanconi's, Paroxysmal Nocturnal Hemoglobinuria, Pure Red Cell Aplasia) - Multiple myeloma - Multiple sclerosis - Myelodysplasia/Myelodysplastic syndromes - Severe combined immunodeficiency - Severe or very severe aplastic anemia	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Organ/tissue transplants - continued on next page

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
- Sickle cell disease • Autologous transplants for: - Acute lymphocytic or non-lymphocytic (i.e., myelogenous) leukemia - Advanced Hodgkin's lymphoma with recurrence (relapsed) - Advanced non-Hodgkin's lymphoma with recurrence (relapsed) - Amyloidosis - Neuroblastoma	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
These blood or marrow stem cell transplants are covered in-network at a Plan-designated National Cancer Institute or National Institutes of Health approved clinical trial facility if approved by the Plan's medical director in accordance with the Plan's protocols. These transplants include but are not limited to the diagnoses below. If you are a participant in a clinical trial, the Plan will provide benefits for related routine care that is medically necessary (such as doctor visits, lab tests, x-rays and scans, and hospitalization related to treating the patient's condition) if it is not provided by the clinical trial. Section 9 has additional information on costs related to <i>clinical trials</i>. We encourage you to contact the Plan to discuss specific services if you participate in a clinical trial. • Allogeneic transplants for: - Advanced Hodgkin's lymphoma - Advanced non-Hodgkin's lymphoma - Beta Thalassemia Major - Chronic inflammatory demyelination polyneuropathy (CIDP) - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Multiple myeloma - Multiple sclerosis - Sickle cell anemia • Mini-transplants (non-myeloablative allogeneic, reduced intensity conditioning or RIC) for: - Acute lymphocytic or non-lymphocytic (i.e., myelogenous) leukemia - Advanced Hodgkin's lymphoma - Advanced non-Hodgkin's lymphoma - Breast cancer	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Organ/tissue transplants - continued on next page

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
- Chronic lymphocytic leukemia - Chronic lymphocytic lymphoma/small lymphocytic lymphoma (CLL/SLL) - Chronic myelogenous leukemia - Colon cancer - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Multiple myeloma - Multiple sclerosis - Myelodysplasia/Myelodysplastic Syndromes - Myeloproliferative disorders (MDDs) - Non-small cell lung cancer - Ovarian cancer - Prostate cancer - Renal cell carcinoma - Sarcomas - Sickle cell anemia • Autologous Transplants for: - Advanced childhood kidney cancers - Advanced Ewing sarcoma - Advanced Hodgkin's lymphoma - Advanced non-Hodgkin's lymphoma - Aggressive non-Hodgkin lymphomas - Breast cancer - Childhood rhabdomyosarcoma - Chronic lymphocytic lymphoma/small lymphocytic lymphoma (CLL/SLL) - Chronic myelogenous leukemia - Early stage (indolent or non-advanced) small cell lymphocytic lymphoma - Epithelial Ovarian Cancer - Mantle Cell (non-Hodgkin lymphoma) - Multiple sclerosis - Small cell lung cancer - Systemic lupus erythematosus - Systemic sclerosis	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: \$250 copayment (no deductible) per performing surgeon, for surgical procedures performed in an inpatient setting In-network: 25% of the Plan allowance for surgical procedures performed in all other settings Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Organ/tissue transplants - continued on next page

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
Transportation Benefit - We will also provide up to \$10,000 per covered transplant, excluding cornea, for transportation (mileage or airfare) to a plan-designated facility and reasonable temporary living expenses (i.e., lodging and meals) for the recipient and one other individual (or in the case of a minor, two other individuals), if the recipient lives more than 100 miles from the designated transplant facility. - Transportation benefits are only payable when we are the primary payor. - Transportation benefits are payable for follow-up care up to one year following the transplant. - You must contact Customer Care at 800-821-6136 for what are considered reasonable temporary living expenses.	All charges in excess of \$10,000 (no deductible)	All charges in excess of \$10,000 (no deductible)
National Transplant Program (NTP) Note: We cover related medical and hospital expenses of the donor when we cover the recipient. We cover donor testing for the actual solid organ donor or up to four bone marrow/stem cell transplant donors in addition to the testing of family members. Notes: - If you are a participant in a clinical trial, please see Section 9, <i>Clinical Trials</i>, for coverage details. - The process for preauthorizing transplants is more extensive than the normal process. Before your initial evaluation as a potential candidate for a transplant procedure, you or your doctor must contact G.E.H.A's Medical Management Department so we can arrange to review the clinical results of the evaluation and determine if the proposed procedure meets our definition of "medically necessary" and is on the list of covered transplants. Coverage for the transplant must be authorized in advance, in writing. - The transplant must be performed at a Plan-designated transplant facility to receive maximum benefits. G.E.H.A uses a defined transplant network which may be different than the Preferred Provider Network. - If benefits are limited to \$100,000 per transplant, included in the maximum are all charges for hospital, medical and surgical care incurred while the patient is hospitalized for a covered transplant surgery and subsequent complications related to the transplant. Outpatient expenses for chemotherapy and any process of obtaining stem cells or bone marrow associated with bone marrow transplant (stem cell support) are included in benefits limit of \$100,000 per transplant. - If precertification is not obtained or a Plan-designated transplant facility is not used, our allowance will be limited for hospital and surgery expenses up to a maximum of \$100,000 per transplant. If we cannot refer a member in need of a transplant to a designated facility, the \$100,000 maximum will not apply.	Services are paid at regular Plan benefits. Note: See Sections 5(a) through 5(f) for applicable services and benefits	Services are paid at regular Plan benefits. Note: See Sections 5(a) through 5(f) for applicable services and benefits

Organ/tissue transplants - continued on next page

Benefits Description	You pay After the calendar year deductible	
Organ/tissue transplants (cont.)	Elevate Plus	Elevate
<i>Not covered:</i> • <i>Services or supplies for or related to surgical transplant procedures (including administration of high-dose chemotherapy) for artificial or human organ/tissue transplants not listed as specifically covered</i> • <i>Donor screening tests and donor search expenses, except as listed above</i> • <i>Implants for artificial organs</i>	<i>All charges</i>	<i>All charges</i>
Anesthesia	Elevate Plus	Elevate
Professional fees for the administration of anesthesia in: • Hospital (inpatient) • Hospital outpatient department • Skilled nursing facility • Ambulatory surgical center • Office Notes: • We cover anesthesia services related to dental procedures when necessitated by a non-dental physical impairment and the patient qualifies for dental treatment in a <i>hospital or outpatient facility</i> (see Section 5(c) for facility coverage). • We do not cover the dental procedures.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Anesthesia related to non-covered surgeries or procedures</i>	<i>All charges</i>	<i>All charges</i>

Section 5(c). Services Provided by a Hospital or Other Facility, and Ambulance Services

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
 - The in-network benefits are the standard benefits of the Plan.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). Applicable coinsurance applies after you met your Deductible until you've reached your Out-of-Pocket Maximum.
 - In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply.
- When you visit an in-network facility, the professionals who provide services to you may not all be in-network providers. If you receive out-of-network services at an in-network facility, we will pay up to the Plan allowable according to the No Surprises Act.
- Medications may be available under the Prescription drug benefit and may require prior authorization.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- **YOU MUST GET PREAUTHORIZATION FOR INPATIENT STAYS UNLESS DUE TO A MEDICAL EMERGENCY. FAILURE TO DO SO WILL RESULT IN A FINANCIAL PENALTY.** Please refer to the preauthorization information shown in Section 3 to be sure which services require preauthorization. Confinements which are considered not medically necessary will not be covered. Penalties are not subject to the catastrophic limit.
- Be sure to read Section 4, *Your Costs for Covered Services*, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- The amounts listed below are for the charges billed by the facility (i.e., hospital or surgical center) or ambulance service for your surgery or care. Any costs associated with the professional charge (i.e., physicians, etc.) are in Sections 5(a) or 5(b).

Benefits Description	You pay After the calendar year deductible...	
Note: The calendar year deductible applies to almost all benefits in this Section. We say “(No deductible)” when it does not apply.		
Inpatient hospital	Elevate Plus	Elevate
Room and board, such as: • Ward, semiprivate, or intensive care accommodation • General nursing care • Meals and special diets Note: We only cover a private room if we determine it to be medically necessary. Otherwise, we will pay the hospital's average charge for semiprivate accommodations. The remaining balance is not a covered expense. If the hospital only has private rooms, we will cover the private room rate. Other hospital services and supplies, such as: • Operating, recovery, maternity and other treatment rooms • Prescribed drugs and medications • Diagnostic laboratory tests and x-rays • Blood or blood plasma, if not donated or replaced • Dressings, splints, casts, and sterile tray services • Medical supplies and equipment, including oxygen • Anesthetics, including nurse anesthetist services • Take-home items • Medical supplies, appliances, medical equipment, and any covered items billed by a hospital for use at home - See Section 5(a) for any applicable <i>Durable Medical Equipment (DME) cost share</i>. Notes: • Precertification is required in advance of admission, see Section 3, <i>Other services that require preauthorization</i>. • We base payment on whether the facility or a healthcare professional bills for the services or supplies. For example, when the hospital bills for its nurse anesthetists’ services, we pay hospital benefits and when the anesthesiologist bills, we pay surgery benefits.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> • <i>Any part of a hospital admission that is not medically necessary, see Section 10, Definitions of Terms, such as when you do not need acute hospital inpatient (overnight) care, but could receive care in some other setting without adversely affecting your condition or the quality of your medical care. Note: In this event, we pay benefits for services and supplies other than room and board and in-hospital physician care at the level they would have been covered if provided in an alternative setting.</i> • <i>Any part of a hospital admission that is related to a non-covered surgery or procedure</i>	<i>All charges</i>	<i>All charges</i>

Inpatient hospital - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Inpatient hospital (cont.)	Elevate Plus	Elevate
• Custodial care (see Section 10, Definitions of Terms) • Long-term care (see Section 10, Definitions of Terms) • Non-covered facilities such as nursing homes or schools • Personal comfort items such as phone, television, barber services, guest meals and beds • Private nursing care	<i>All charges</i>	<i>All charges</i>
Outpatient hospital or ambulatory surgical center	Elevate Plus	Elevate
• Operating, recovery, observation, and other treatment rooms • Prescribed drugs and medications • Administration of blood, blood plasma, and other biologicals • Blood or blood plasma, if not donated or replaced • Pre-surgical testing • Dressings, casts, and sterile tray services • Medical supplies, including oxygen • Anesthetics and anesthesia service • Outpatient cardiac and pulmonary rehabilitation, 36 visit maximum each per year • Respiratory and inhalation therapies Notes: • Surgical benefits, not maternity benefits, apply to circumcision. • We cover hospital services and supplies related to dental procedures when necessitated by a non-dental physical impairment. We do not cover the dental procedures.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Outpatient diagnostic and treatment services performed and billed by a facility, such as but not limited to: • Blood tests • Urinalysis • Non-routine Pap tests • Pathology	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Outpatient diagnostic testing and treatment services performed and billed by a facility, such as but not limited to: • X-rays • Non-routine mammograms • Ultrasound • Electrocardiogram and EEG	In-network: \$50 copayment per day per facility (no deductible) Out-of-Network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Outpatient diagnostic testing and treatment services performed and billed by a facility, such as but not limited to: • CT/CAT Scan • MRI	In-network: \$75 copayment per day per facility (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance

Outpatient hospital or ambulatory surgical center - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Outpatient hospital or ambulatory surgical center (cont.)	Elevate Plus	Elevate
Note: Preauthorization is required for these tests	In-network: \$75 copayment per day per facility (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
<i>Not covered:</i> <i>Maintenance cardiac and pulmonary rehabilitation</i> <i>Services that are related to a non-covered surgery or procedure</i>	<i>All charges</i>	<i>All charges</i>
Treatment Therapies - Preauthorization is required, see Section 3, <i>Other services that require preauthorization.</i> - Intravenous (IV)/Infusion Therapy - Outpatient IV/infusion and antibiotic therapy - Intrathecal pumps refills - Note: The per diem (daily) rate for intrathecal pump refill will only be reimbursed on the day of the refill. No daily per diems will be allowed. - Chemotherapy and radiation therapy - Preauthorization required for chemotherapy and radiation therapy - Note: High-dose chemotherapy in association with autologous bone marrow transplants is limited to those transplants listed in Section 5(b), <i>Services Provided by a Hospital or Other Facility, and Ambulance Services.</i> Note: - Medications required for treatment therapies may be available under the <i>Prescription drug</i> benefits in Section 5(f).	In-network: 20% of the Plan allowance You pay 10% of the Plan allowance (deductible applies) for drugs administered or obtained in connection with your treatment (excludes drugs obtained under the Prescription drug benefit in Section 5(f)). Note: Specialty drugs obtained outside of the pharmacy benefit from other sources (physician offices, home health agencies, outpatient hospitals) may be subject to additional cost share as outlined in Section 5(f), Specialty drug benefits. Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount.

Benefits Description	You pay After the calendar year deductible...	
Extended care benefits/Skilled nursing care facility benefits	Elevate Plus	Elevate
Skilled nursing facility (SNF): We cover semiprivate room, board, services and supplies in a SNF for up to 50 days per confinement when: • SNF care is medically appropriate (prior authorization is required, See Section 3).	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	<i>All charges</i>
Hospice/End of life care	Elevate Plus	Elevate
Hospice is a coordinated program of maintenance and supportive care for the terminally ill provided by a medically supervised team under the direction of a Plan-approved independent hospice administration. • We pay up to \$30,000 for hospice care provided in an outpatient setting, or for room, board, and care while receiving hospice care in an inpatient setting. Services may include a combination of inpatient and outpatient care up to a maximum of \$30,000. These benefits will be paid if the hospice care program begins after a person's primary doctor certifies terminal illness and life expectancy of six months or less and any services or inpatient hospice stay that is part of the program is: • Provided while the person is covered by this Plan • Ordered by the supervising doctor • Charged by the hospice care program • Provided within six months from the date the person entered or re-entered (after a period of remission) a hospice care program Remission is the halt or actual reduction in the progression of illness resulting in discharge from a hospice care program with no further expenses incurred. A readmission within three months of a prior discharge is considered as the same period of care. A new period begins after three months from a prior discharge with maximum benefits available. Note: See Section 5(a), Medical services and supplies provided by physicians and other health care professionals, diagnostic and treatment services, professional services of physicians.	In-network: Charges in excess of \$30,000 (deductible applies) Out-of-network: You pay all charges	Charges in excess of \$30,000 (deductible applies)
<i>Not covered:</i> • <i>Charges incurred during a period of remission</i> • <i>Bereavement counseling, pastoral counseling, financial or legal counseling</i> • <i>Funeral arrangements</i> • <i>Homemaker or caretaker services</i>	<i>All charges</i>	<i>All charges</i>

Benefits Description	You pay After the calendar year deductible...	
Ambulance	Elevate Plus	Elevate
Ambulance (Local and Air): Please see Section 5(d), <i>Ambulance</i> for complete ambulance benefit coverage information. Note: Medical Necessity review is required for all air ambulance transportation.	Services are paid at the regular Plan benefits.	Services are paid at the regular Plan benefits.

Section 5(d). Emergency Services/Accidents

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
 - The in-network benefits are the standard benefits of the Plan.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply. Applicable coinsurance applies after you meet your Deductible until you've reached your Out-of-Pocket Maximum.
 - In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply.
- We will provide in-network benefits if you are admitted to an out-of-network hospital due to a medical emergency. We will also provide in-network benefits if you receive care from professionals who provide services in an out-of-network hospital, when admitted due to a medical emergency.
- Under the Elevate Option, the out-of-network benefits are the standard benefits of the Plan. In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply. Under the Elevate Plus Option, the in-network benefits are the standard benefits of the Plan.
- When you visit an in-network facility, the professionals who provide services to you in a facility may not all be preferred providers. If you receive out-of-network services at an in-network facility, we will pay up to the Plan allowance according to the No Surprises Act.
- Medications may be available under the Prescription drug benefit and may require prior authorization.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- **YOU MUST GET PREAUTHORIZATION FOR SOME SERVICES.** Please refer to the preauthorization information shown in Section 3 to be sure which services require preauthorization.
- Be sure to read Section 4, *Your Costs for Covered Services*, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.

What is an Accidental Injury?

An accidental injury is a bodily injury sustained solely through violent, external, and accidental means, such as broken bones, animal bites, and poisonings.

Benefits Description	You pay After the calendar year deductible...	
Note: The calendar year deductible applies to almost all benefits in this Section. We say “(No deductible)” when it does not apply.		
Accidental injury	Elevate Plus	Elevate
If you receive care for your accidental injury within 48 hours, we cover: • Non-surgical physician services and supplies • Related outpatient hospital services Note: We pay hospital benefits if you are admitted.	In-network: 25% of the Plan allowance Note: Out-of-network emergency care is paid at the in-network level	In-network: 35% of the plan allowance Note: Out-of-network emergency care is paid at the in-network level
If you receive care for your accidental injury after 48 hours, we cover: • Non-surgical physician services and supplies • Surgical care Note: We pay hospital benefits if you are admitted.	In-network: 25% of the Plan allowance Note: Out-of-network emergency care is paid at the in-network level	In-network: 35% of the plan allowance Note: Out-of-network emergency care is paid at the in-network level
Medical emergency	Elevate Plus	Elevate
Outpatient medical or surgical services and supplies billed by a hospital or physician for emergency room treatment. Notes: • We pay hospital benefits if you are admitted. • We will also provide in-network benefits for professionals who provide services in an out-of-network hospital, when admitted due to a medical emergency.	In-network: 25% of the Plan allowance Note: Out-of-network emergency care is paid at the in-network level	In-network: 35% of the plan allowance Note: Out-of-network emergency care is paid at the in-network level
Urgent Care Facility	Elevate Plus	Elevate
Outpatient medical services and supplies billed by an urgent care facility Note: This applies only to urgent care facilities, not providers that offer urgent care or after-hours services.	In-network: \$50 copayment (no deductible) Out-of-network: You pay all charges	In-network: \$50 copayment (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Ambulance	Elevate Plus	Elevate
Local ambulance service, within 100 miles*, only when medically necessary and the patient cannot be transported by other means to: • to the first hospital where treated • from the first hospital to the next nearest hospital or other medical facility only, if necessary, treatment is unavailable or medically unsuitable at the first hospital • the home, only when the patient requires the assistance of medically trained personnel during transportation * Member is responsible for all charges for 100 miles or greater when medically necessary treatment is available within 100 miles.	In-network: 20% of the Plan allowance Note: Out-of-network emergency care and transport is paid at the in-network level	In-network: 25% of the Plan allowance Out-of-network: 25% of the Plan allowance and any difference between our allowance and the billed amount

Ambulance - continued on next page

Benefits Description	You pay After the calendar year deductible...	
Ambulance (cont.)	Elevate Plus	Elevate
Air ambulance to nearest hospital is only covered when medically necessary, and the severity of the member's condition warrants immediate evacuation, and: - the pick-up location is inaccessible by other means, or - transportation by any other means could further endanger the member's health, and - the patient is transported to the nearest facility where medically necessary treatment is available. Note: Medical Necessity review is required for all air ambulance transportation.	In-network: 20% of the Plan allowance Note: Out-of-network emergency care and transport is paid at the in-network level	In-network: 25% of the Plan allowance Note: Out-of-network emergency care and transport is paid at the in-network level
<i>Not covered:</i> <i>Ambulance transportation when the patient does not require the assistance of medically trained personnel and can be safely transferred (or transported) by other means</i> <i>All ambulance charges for 100 miles or greater when medically necessary treatment is available within 100 miles</i> <i>Non-ambulance transportation including wheelchair van, gurney van, commercial air flights, or any other vehicle not licensed as ambulance</i> <i>Air ambulance will not be covered if transport is beyond the nearest available medically suitable facility, but is requested by patient or physician for continuity of care or other reasons</i>	<i>All charges</i>	<i>All charges</i>

Section 5(e). Mental Health and Substance Use Disorder Benefits

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
 - The in-network benefits are the standard benefits of the Plan.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). Applicable coinsurance applies after you met your Deductible until you've reached your Out-of-Pocket Maximum. We added "(no deductible)" to show when the calendar year deductible does not apply.
 - In-network benefits apply only when you use an in-network provider. When no in-network provider is available, out-of-network benefits apply.
- When you visit an in-network facility, the professionals who provide services to you in a facility may not all be in-network providers. If you receive out-of-network services at an in-network facility, we will pay up to the plan allowance according to the No Surprises Act.
- Medications may be available under the Prescription drug benefit and may require prior authorization.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- **YOU MUST GET PREAUTHORIZATION FOR SOME SERVICES.** Please refer to the preauthorization information shown in Section 3 to be sure which services require preauthorization.
- Be sure to read Section 4, *Your Costs for Covered Services*, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- The coverage and cost-sharing listed below are for services provided by physicians and other healthcare professionals for your medical care. See Section 5(c), for cost-sharing associated with the facility (i.e., hospital, surgical center, etc.).

Benefits Description		You pay After the calendar year deductible...
Note: The calendar year deductible applies to almost all benefits in this Section. We say "(No deductible)" when it does not apply.		
Professional services	Elevate Plus	Elevate
We cover professional services by licensed professional mental health and substance use disorder treatment practitioners when acting within the scope of their license, such as psychiatrists, psychologists, clinical social workers, licensed professional counselors, or marriage and family therapists.	Your cost-sharing responsibilities are no greater than for other illnesses or conditions.	Your cost-sharing responsibilities are no greater than for other illnesses or conditions.
Diagnosis or treatment of behavioral health conditions including psychiatric conditions, mental illness or disorders, and substance use disorders. Services include:	In-network: \$30 copayment per office visit (no deductible)	In-network: \$10 copayment per office visit (no deductible)

Benefits Description	You pay After the calendar year deductible...	
Professional services (cont.)	Elevate Plus	Elevate
- Diagnostic evaluation - Crisis intervention and stabilization for acute episodes - Medication evaluation and management (pharmacotherapy) - Treatment and counseling (including individual, group, or in-home therapy visits) - Diagnosis and treatment of substance use disorders including detoxification, treatment and counseling - Professional charges for intensive day treatment in a provider's office or other professional setting Preauthorization is required, see Section 3, <i>Other services that require preauthorization.</i>	In-network: \$30 copayment per office visit (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment per office visit (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
- Electroconvulsive therapy (ECT) - facility fees may also apply - Inpatient professional fees	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
One visit to a primary care or specialist for the management of a mental health condition or substance use disorder as a follow up within 30 days of: - Inpatient confinement - Emergency room visit	In-network: Nothing (no deductible) Out-of-network: You pay all charges	In-network: Nothing (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Professional services for the first 5 visits per year, per pregnancy for office-based treatment of prenatal and postpartum depression. Services include: - Diagnostic evaluation - Medication evaluation and management (pharmacotherapy) - Treatment and counseling (including individual, group, or in-home therapy visits)	In-network: Nothing for the first 5 visits for treatment of prenatal and postpartum depression, after which the \$30 copayment applies (no deductible) Out-of-network: You pay all charges	In-network: Nothing for the first 5 visits for treatment of prenatal and postpartum depression, after which the \$10 copayment applies (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Benefits Description	You pay After the calendar year deductible...	
Applied Behavioral Analysis Therapy	Elevate Plus	Elevate
Note: Here are some things to keep in mind: • Preauthorization is required, see Section 3, <i>Other services that require preauthorization</i>. • Required diagnosis of ASD (Autism Spectrum Disorder) by a provider qualified to make the diagnosis: Board Certified Behavior Analyst (BCBA), psychiatrist, or pediatrician. • Initiation of treatment and on-going treatment and intensity of treatment must be medically necessary and appropriate for the child. • A Functional Behavioral Assessment must be submitted prior to treatment and must demonstrate appropriateness of ABA Therapy. • Services must be directed by a Board-Certified Behavior Analyst (BCBA) and services may be provided by Board Certified Assistant Behavior Analysts (BCaBA) or Registered Behavior Technicians (RBTs). • Approval of on-going services requires demonstrated involvement by family.	In-network: \$30 copayment per office visit (no deductible) Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Diagnostics	Elevate Plus	Elevate
• Outpatient diagnostic tests provided and billed by a licensed mental health and substance use disorder treatment practitioner • Outpatient diagnostic tests provided and billed by a laboratory, hospital or other covered facility Note: Certain diagnostic tests are not subject to deductible. See Section 5(a), <i>Lab, x-ray and other diagnostic tests</i>, for applicable cost share	Your cost sharing responsibilities are no greater than for other illnesses or conditions. See diagnostic tests, Section 5(a) and 5(c).	Your cost sharing responsibilities are no greater than for other illnesses or conditions. See diagnostic tests, Section 5(a) and 5(c).
• Psychological and neuropsychological testing necessary to determine the appropriate psychiatric treatment Notes: • Preauthorization required for testing exceeding 8 hours/calendar year • See Section 5(c), for any applicable outpatient facility charges.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Benefits Description	You pay After the calendar year deductible...	
Telehealth services	Elevate Plus	Elevate
Behavioral health professional services through the Plan's administrative partner for: • Mental health counseling • Substance use disorder counseling Note: • For more information on telehealth benefits, please see Section 5(h), <i>Wellness and Other Special Features</i>, or visit geha.com/Telehealth-Visit.	Nothing (no deductible)	Nothing (no deductible)
Telehealth professional services provided by a healthcare provider other than the Plan's telehealth administrative partner.	In-network: \$30 copayment for office visits to primary care providers (no deductible) Out-of-network: You pay all charges	In-network: \$10 copayment for office visits to primary care providers (no deductible) Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Inpatient hospital or other covered facilities	Elevate Plus	Elevate
Precertification is required in advance of admission. Inpatient services provided and billed by a hospital or other covered facility • Room and board, such as semiprivate or intensive accommodations, general nursing care, meals and special diets, and other hospital services Note: We limit covered facilities for medically necessary treatment to a hospital and/or Residential Treatment Center.	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount
Outpatient hospital or other covered facilities	Elevate Plus	Elevate
Preauthorization required, see Section 3, <i>Other services that require preauthorization</i>. Outpatient services provided and billed by a hospital or other covered facility, services such as: • Services such as partial hospitalization, residential treatment, full-day hospitalization, or facility-based intensive outpatient treatment • Electroconvulsive therapy • Transcranial Magnetic Stimulation (TMS)	In-network: 20% of the Plan allowance Out-of-network: You pay all charges	In-network: 25% of the Plan allowance Out-of-network: 50% of the Plan allowance and any difference between our allowance and the billed amount

Benefits Description	You pay After the calendar year deductible...	
Emergency room	Elevate Plus	Elevate
See Section 5(d), <i>Medical emergency</i>	Services are paid at the regular Plan benefits.	Services are paid at the regular Plan benefits.
Services we do not cover	Elevate Plus	Elevate
<i>Not covered:</i> • <i>ABA therapy services provided by the school</i> • <i>Pastoral, marital, educational counseling or training services</i> • <i>Therapy for sexual dysfunction or inadequacy</i> • <i>Services performed by a non-covered provider</i> • <i>Treatment for learning and intellectual disabilities</i> • <i>Services rendered or billed by schools, halfway houses, or sober homes</i> • <i>Hypnotherapy</i> • <i>Respite care, outdoor residential programs, recreational therapy, educational therapy or classes</i> • <i>Outward Bound programs, including equine therapy provided during the approved stay</i> • <i>Personal comfort items, such as guest meals and beds, telephone, television, beauty and barber services</i> • <i>Custodial or long-term care</i> • <i>Testing ordered by or on behalf of third parties (e.g., schools, courts, employers, etc.)</i> • <i>Physical, psychiatric, or psychological exams and testing required for obtaining or continuing employment or insurance, attending schools or camps, sports physicals, travel related to judicial or administrative proceedings or orders, or required to obtain or maintain a license of any type</i>	<i>All charges</i>	<i>All charges</i>

Section 5(f). Prescription Drug Benefits

Important things you should keep in mind about these benefits:

- We cover prescribed drugs and medication, as described in this section.
- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- Members must make sure their prescribers obtain prior approval/authorizations for certain prescription drugs and supplies before coverage applies. Prior approval/authorizations must be renewed periodically.
- Federal Law prohibits the return of prescription medications. Medication cannot be returned to dispensing pharmacies, and you will be responsible for the cost. Be sure to check the cost of your medication before filling the prescription.
- There is no calendar year deductible for prescription drugs processed under the prescription benefit. Copayments and coinsurance for prescription drugs go toward the annual out-of-pocket limit except for the difference between the cost of the generic and brand name medication.
- The non-PPO benefits are the standard benefits of this Plan. PPO benefits apply only when you use a PPO provider. When no PPO provider is available, non-PPO benefits apply.
- Be sure to read Section 4, *Your Costs for Covered Services*, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- **The exclusion for hormone treatments for Sex-Trait Modification for gender dysphoria** only pertains to chemical and surgical modification of an individual's sex traits (including as part of "gender transition" services). **We do not exclude coverage for entire classes of pharmaceuticals**, e.g., GnRH agonists may be prescribed during IVF, for reduction of endometriosis or fibroids, and for cancer treatment or prostate cancer/tumor growth prevention.

There are important features you should be aware of. These include:

- **How to obtain preauthorization:** If you are filling a medication requiring a preauthorization for medical necessity, please call 855-240-0536.
- **Your plan participates in the Exclusive Choice Pharmacy Network.** All prescriptions need to be purchased through a participating network retail pharmacy. **Out-of-network claims are not eligible for benefits.**
- **Drug coupon/copay cards:** We do not coordinate benefits with drug coupon/copay cards.
- **Who can write your prescription:** A licensed physician or dentist, and in states allowing it, licensed or certified providers with prescriptive authority prescribing within their scope of practice.
- **Where you can obtain them:** You may fill the prescription at a participating network retail pharmacy or for Elevate Plus Option members, CVS Caremark Mail Service Pharmacy. CVS Specialty Pharmacy is the exclusive provider for specialty medications. You may contact the Specialty Pharmacy at 800-237-2767.
- **We have a managed formulary.** If your physician believes a name brand product is necessary or there is no generic available, your physician may prescribe a name brand drug from a formulary list. This list of name brand drugs is a preferred list of drugs that we selected to meet patient needs at a lower cost.
 - **Please see our online formulary and drug pricing search tools at the following geha.com/Prescriptions-Summary.**
- **These are the dispensing limitations.**
 - We use a formulary drug list that excludes coverage for certain medications unless we determine they are medically necessary. Refer to geha.com/Prescriptions-Summary for a list of drugs that require preauthorization for medical necessity.
 - CVS Specialty Pharmacy is the exclusive provider for specialty medications. You may contact the Specialty Pharmacy at 800-237-2767.

- Refills cannot be obtained until 80% of the drug has been used. Next available refill date may be provided; however, the date is an estimate. Cumulative "refill too soon" logic also applies, which looks back at prescription history and considers the amount of medication on hand. Refills for maintenance medications are not considered new prescriptions except when the doctor changes the strength, or the prescription has expired.
- Recurring oral non-specialty and specialty medications must be obtained through the pharmacy benefit. Medications will not be covered when dispensed by other sources, including physician offices, home health agencies and outpatient hospitals.
- Select therapies are included in the Starter Fill Program. For these medications, you will receive 14- or 15- day supplies for the first 2 months of therapy. Your coinsurance will be prorated based on the days of therapy.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- Some specialty and non-specialty medications may not be available in a 30-day supply, your coinsurance will be based on days of therapy.
- **A generic equivalent will be dispensed if it is available**, unless your physician specifically requires a name brand. If you receive a name brand drug when a FDA approved generic drug is available, and your physician has not specified Dispense as Written for the name brand drug, you have to pay the difference in cost between the name brand drug and the generic.
- **Why use generic drugs?** Generic drugs are FDA approved prescription medications. They are chemically and therapeutically equivalent to the corresponding brand name drug but are available at a lower price. The FDA requires that generic equivalent medications contain the same active ingredients and be equivalent in strength and dosage to brand name drugs.
- **When you do have to file a claim.** You may need to file a claim for services you received overseas or when another group health plan is primary or if you use a non-Network pharmacy. Visit www.geha.com/Claim for more information on how to file.
- **How to submit a prescription reimbursement claim:**
 - Include original drug receipts and submit to:

CVS Caremark
P.O. Box 52136
Phoenix, AZ 85072-2136

You may also submit prescription reimbursement requests online via Caremark's web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple).

Reimbursement will be based on G.E.H.A's costs had the participating pharmacy submitted the claim electronically. Members are responsible for their applicable copayment and/or coinsurance, and the difference between our allowance and the cost of the drug.

- **Our prescription benefit includes a step therapy program.** G.E.H.A's preauthorization process may include a step therapy which requires you to use a generic/preferred medication(s) before a brand and/or non-preferred medication is covered. If you are filling a brand and/or non-preferred medication and have already tried a generic/preferred medication(s), the brand and/or non-preferred medication will be dispensed for the applicable plan copayment/coinsurance.
 - When you try to fill a brand and/or non-preferred medication and you have not tried a generic/preferred medication(s), the pharmacist will contact your physician to notify them of the generic/preferred alternative. If the physician approves, a generic/preferred medication will be dispensed for the applicable plan copayment/coinsurance. If the physician does not approve, a preauthorization review will be initiated to determine the medical necessity of the brand and/or non-preferred drug. If the preauthorization for the brand and/or non-preferred medication is approved, you will be responsible for the applicable plan copayment/coinsurance.
 - Unless there are documented clinical reasons why you cannot take the generic/preferred drug, you may still obtain the brand and/or non-preferred drug, but you will be responsible for 100% of the cost of the brand and/or non-preferred drug, which will not apply to your annual out-of-pocket maximum.
- **Compound Medication:** A compound drug is a medication made by combining, mixing or altering ingredients in response to a prescription, to create a customized drug that is not otherwise commercially available. Some ingredients often found in compounds including, but not limited to, over the counter (OTC) products, experimental or investigational agents, bulk powders, bulk chemicals, and certain bases, are not covered through the prescription benefit. Coverage for other ingredients commonly found in compound prescriptions may also **require preauthorization** before coverage is allowed.

- Claim pricing is based on the contractual discounts plus a professional fee and any applicable sales tax. Pharmacies must submit all ingredients in a compound prescription for online and paper claim submissions. At least one of the ingredients must require a physician's prescription in order to be covered by the Plan. You are responsible for the appropriate brand or generic copay/coinsurance based on ingredients. **Preauthorization may be required.** If the compound includes an experimental or investigational drug, the compound will not be covered. Compound medications are limited to a 30-day supply.
- Ask the pharmacist to submit your claim electronically or online. If the participating retail pharmacy is unable to submit the compound medication claim electronically to CVS Caremark, you will pay the full cost of the medication and must submit the claim for reimbursement. Make sure the pharmacy provides a list of the National Drug Codes (NDCs), quantity and cost for every ingredient in the compound medication and include this information on your claim.
- Claim calculations, copayments, and reimbursement for direct claims are performed using an industry standard reimbursement method for compounds.
- **We divide prescription drugs into categories or tiers:** generic, preferred, and non-preferred medications. Please note specialty medications can be considered either preferred or non-preferred. When an approved generic equivalent is available, that is the drug you will receive, unless you or your physician specifies the prescription must be dispensed as written. When an approved generic equivalent is not available, you will pay the brand name drug's applicable plan copayment/coinsurance. If an approved generic equivalent is available, but you or your physician specifies that the prescription must be dispensed as written with the brand name medication, you will pay the generic copayment plus the difference between the cost of the generic drug and the brand name drug dispensed. Your physician may request the brand name drug be reviewed and if approved as medically necessary, you will pay the applicable brand name copayment/coinsurance.
 - **Generic drugs** are FDA approved prescription medications. They are chemically and therapeutically equivalent to the corresponding brand name drug but are available at a lower price. The FDA requires that generic equivalent medications contain the same active ingredients and be equivalent in strength and dosage to brand name drugs.
 - **Preferred drugs** are FDA approved prescription medications included on the Preferred Drug List developed by CVS Caremark. This list is developed by an independent panel of doctors and pharmacists who ensure the medications are clinically appropriate and cost-effective.
 - **Non-Preferred drugs** are FDA approved prescription medications that may or may not be covered by G.E.H.A, however they are not included on the CVS Caremark Preferred Drug List. Most commonly utilized medications have generic or preferred medications available.

CVS Caremark Formulary

Your prescription drug program includes use of the CVS Caremark Formulary which is developed by an independent panel of doctors and pharmacists who ensure the medications are clinically appropriate and cost-effective. Formularies are reviewed quarterly; medications may change formulary status. You will receive notification if your cost share increases due to a formulary change. In an effort to continue to help promote affordable and clinically appropriate products, there are a select number of drugs that are excluded from the formulary and are not covered by the Plan. For these drugs, generics and/or therapeutic alternative medications in the same drug class are readily available. If one of these excluded drugs is medically necessary, a **preauthorization for medical necessity is required**. We do not cover excluded drugs unless we determine the medical necessity to treat a medical condition based on objective clinical data. New drugs and supplies may be added to the list as they are introduced and may require a review for medical necessity until the formulary status is determined. Please visit our website at geha.com/Prescriptions-Summary to view formulary medications and the most current list of specialty drugs. You may also call CVS Specialty at 800-237-2767.

The Elevate Plus Option includes the Advanced Control Specialty Formulary (ACSF). The ACSF may reduce your out-of-pocket costs yet may limit your options due to a strict formulary. The ACSF focuses on specialty medications that are similar to one another, with similar effectiveness and safety. The Elevate Option includes a Value Formulary. The Value Formulary is a closely managed drug list and an array of utilization management (UM) strategies, including classes largely managed with specialty medications. Both Elevate and Elevate Plus formularies incorporate step therapy, where a generic/preferred medication is used prior to a brand and/or non-preferred medication.

CVS Caremark Formulary - continued on next page

CVS Caremark Formulary (cont.)

Notes:

- **Changes to the formulary are not considered benefit changes.**
- Your physician may be contacted to discuss your prescriptions for drugs that are excluded by the Plan's formulary. No change in the medication prescribed will be made without your physician's approval.
- Please visit our website at geha.com/Prescriptions-Summary to view the most current list of specialty drugs. You may also call CVS Caremark at 844-443-4279 or CVS Specialty at 800-237-2767.
- **Any rebates or savings received by the Plan on the cost of drugs purchased under this Plan from drug manufacturers are credited to the health plan and are used to reduce healthcare costs.**

Coordinating with other drug coverage

For other commercial coverage: If you also have drug coverage through another group health insurance plan and we are your secondary insurance, follow these procedures:

If you obtain your prescription from a retail pharmacy using your primary insurance plan:

1. Present prescription ID cards from both your primary insurance plan and G.E.H.A.
2. If able, the pharmacy will electronically process both your primary and secondary claims, and the pharmacist will tell you if you have any remaining copay/coinsurance to pay.
3. If the pharmacy cannot electronically process the secondary claim, purchase your prescription using the prescription ID card issued by your primary insurance carrier and pay any copay/coinsurance required by the primary insurance. Then, mail your pharmacy receipt and primary Explanation of Benefits (EOB) to CVS Caremark for consideration of possible reimbursement through your G.E.H.A. secondary benefit. Submit these claims to CVS Caremark, PO Box 52136, Phoenix, AZ 85072-2136 or submit for reimbursement online via Caremark web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple).

If you obtain your prescription from a mail service pharmacy using your primary insurance plan, your G.E.H.A. reimbursement will be based on the G.E.H.A. retail Plan benefit:

1. Purchase your prescription using the prescription ID card issued by your primary insurance carrier and pay any copay/coinsurance required by the primary insurance.
2. Then, mail your pharmacy receipt and primary EOB to CVS Caremark for consideration of possible reimbursement through your G.E.H.A. secondary benefit. Submit these claims to CVS Caremark, PO Box 52136, Phoenix, AZ 85072-2136 or submit for reimbursement online via Caremark web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple).

If your primary insurance does not provide a prescription ID card:

1. Purchase your drug from the pharmacy and submit the bill to your primary insurance.
2. When the primary insurance has made payment, file the claims and the primary EOB with CVS Caremark for consideration of possible reimbursement using your secondary benefit. Submit these claims to CVS Caremark, PO Box 52136, Phoenix, AZ 85072-2136 or submit for reimbursement online via Caremark web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple).

In any event, if you use G.E.H.A.'s plan ID card when another insurance plan is primary, you will be responsible for reimbursing G.E.H.A. any amount in excess of our secondary benefit. If another insurance plan is primary, you should use their drug benefit.

When coordination of benefits apply, reimbursement is based on G.E.H.A.'s retail Plan allowable benefit. Our secondary and tertiary claim payment is the lesser of:

- what G.E.H.A. would have paid in the absence of other primary coverage, or
- the balance due after the primary carrier's payment.

Coordinating with other drug coverage - continued on next page

Coordinating with other drug coverage (cont.)

Note: G.E.H.A secondary and tertiary member responsibility could be higher than G.E.H.A's primary copay/coinsurance, depending upon the primary plan's allowable and the primary payment.

Should Medicare rules change on prescription drug coverage, we reserve the right to require you to use your Medicare coverage as the primary insurance for these drugs.

For Medicare Part B insurance coverage: If Medicare Part B is primary, discuss with the retail pharmacy and/or CVS Caremark the options to submit Medicare covered medications and supplies to allow Medicare to pay as the primary carrier. Prescriptions typically covered by Medicare Part B include diabetes supplies (test strips, meters), specific medications used to aid tissue acceptance from organ transplants, certain oral medications used to treat cancer, and ostomy supplies.

Retail - When using a retail pharmacy for eligible Medicare Part B medication or supplies, present the Medicare ID card. Request the retail pharmacy bill Medicare as primary. Most independent pharmacies and national chains are Medicare providers. To locate a retail pharmacy that is a Medicare Part B participating provider, visit the Medicare website at www.medicare.gov/supplier/home.asp or call Medicare Customer Service at 800-633-4227.

Mail Order – Elevate Plus Option members also have the opportunity to receive Medicare Part B-eligible medications by mail. The CVS Caremark Mail Service Pharmacy will review the prescriptions to determine whether it could be eligible for Medicare Part B coverage and submit to Medicare if appropriate.

Note: The CVS Caremark Mail Service Pharmacy is not a Medicare Part B provider for diabetic supplies. You must use a retail pharmacy willing to bill Medicare as primary.

For Medicare Part D insurance coverage: G.E.H.A supplements the coverage you get with your Medicare Part D prescription drug plan. Your Medicare drug plan provides your primary prescription drug benefit. G.E.H.A provides your secondary prescription drug benefit. To ensure that you maximize your benefits, use a pharmacy in network for both the G.E.H.A Plan and your Medicare Part D plan, and provide both the plan ID cards when filling a prescription allowing the pharmacy to coordinate coverage on your behalf.

Patient Safety

G.E.H.A has several programs to promote patient safety. Through these programs, we work to ensure safe and appropriate quantities of medication are being dispensed. The result is improved care and safety for our members. Patient safety programs include:

- Preauthorization - Approval must be obtained for certain prescription drugs and supplies before providing benefits for them.
- Quantity allowances - Specific allowances are in place for certain medications, based on manufacturer and FDA recommended guidelines.
- Pharmacy utilization - G.E.H.A reserves the right to maximize your quality of care as it relates to the utilization of pharmacies.

G.E.H.A will participate in other approved managed care programs, as deemed necessary, to ensure patient safety.

How to use participating network retail pharmacies

You must fill your prescription at a participating network retail pharmacy. To locate participating pharmacies, call CVS Caremark at 844-443-4279 or visit geha.com/Prescriptions-Summary. To receive maximum savings, you must present your plan ID card at the time of each purchase, and your enrollment information must be current and correct. In most cases, you simply present the plan ID card together with the prescription to the pharmacist.

Claims (electronic or paper) processed through a non-network pharmacy will not be paid unless they are filled outside of the United States or United States territories.

How to use CVS Caremark Mail Service Pharmacy for Elevate Plus

Through this service, Elevate Plus Option members may receive up to a 90-day supply per prescription of maintenance medications and/or supplies for drugs which require a prescription. Some medications may not be available in a 90-day supply from CVS Caremark Mail Service Pharmacy even though the prescription is for 90 days. Although insulin, syringes, diabetic supplies and ostomy supplies do not require a physician's prescription, to obtain through CVS Caremark Mail Service Pharmacy you should obtain a prescription (including the product number for ostomy and insulin pump supplies) from your physician for a 90-day supply.

Not all drugs are available through CVS Caremark. In order to use CVS Caremark Mail Service Pharmacy, your prescriptions must be written by a licensed prescriber in the United States. In addition, your mailing address must be within the United States or include an APO address.

To order new prescriptions, ask your physician to prescribe needed medication for up to a 90-day supply, plus refills, if appropriate. Complete the information on the Ordering Medication Form found at geha.com/Mail-Order-Rx; enclose your prescription and the correct copayment/coinsurance.

Under regular circumstances, you should receive your medication within approximately 14 days from the date you mail your prescription. If you have any questions or need an emergency consultation with a registered pharmacist, you may call CVS Caremark at 844-443-4279 available 24 hours a day, 7 days a week. Forms necessary for refills will be provided each time you receive a supply of medication.

Refilling your medication: To be sure you never run short of your prescription medication, you should re-order on or after the estimated refill date or when you have approximately 18 days of medication left.

To order by phone: Call Member Services at 844-443-4279. Have your prescription bottle with the prescription information ready.

To order by mail: Simply mail the G.E.H.A Mail Order Form and copayment/coinsurance to CVS Caremark, PO Box 659541, San Antonio, TX 78265-9541.

To order online: Go to geha.com/Prescriptions-Summary.

Fax: You can ask your physician to fax your prescriptions to CVS Caremark Mail Service Pharmacy. To do this, provide your physician with your ID number (located on your ID card) and ask him or her to fax the prescription to the CVS Caremark Mail Service Pharmacy fax number: 800-378-0323.

Electronic transmission: You can ask your physician to transmit your prescriptions electronically to CVS Caremark Mail Service Pharmacy.

Benefits Description	You pay	
Covered medications and supplies	Elevate Plus	Elevate
Network Retail Pharmacy You must fill your prescription at a participating network retail pharmacy. • Drugs and medications (including those administered during a non-covered admission or in a non-covered facility) that by Federal Law of the United States require a physician's prescription for their purchase, except those listed as not covered; • FDA approved contraceptive drugs and devices for women; • Diabetic medications and supplies, such as: - Insulin; - Needles and syringes for the administration of covered medications; - Blood glucose meter to be provided at no charge by the manufacturer.	Generic: • \$15 for up to a 30-day supply • \$30 for a 31-60 day supply • \$45 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Preferred: • \$100 for up to a 30-day supply • \$200 for a 31-60 day supply • \$300 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less	Generic: • \$10 for up to a 30-day supply • \$20 for a 31-60 day supply • \$30 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Preferred: 50% of Plan allowance up to a maximum of: • \$500 for up to a 30-day supply • \$1,000 for a 31-60 day supply • \$1,500 for a 61-90 day supply

Covered medications and supplies - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay	
Covered medications and supplies (cont.)	Elevate Plus	Elevate
- Elevate Plus Option members may contact CVS Caremark Mail Service Pharmacy: 877-418-4746. - Elevate Option members may contact their participating retail pharmacy or CVS Caremark Customer Care for specifics on how to obtain a free meter at 844-443-4279. - Drugs associated with artificial insemination and/or drugs associated with up to three (3) cycles of in vitro fertilization (IVF) treatment. Prior authorization is required. - Medications prescribed to treat obesity. Prior authorization is required. - Prescription prenatal vitamins; - Covered ostomy supplies (please include the manufacturer's product number to ensure accurate fill of the product). Notes: - If there is no generic equivalent available, you will pay the brand name copay/coinsurance. - Medications to treat some complex and chronic medical conditions are only available through CVS Specialty. See CVS Caremark formulary for the categories of drugs in this program.	Generic: \$15 for up to a 30-day supply \$30 for a 31-60 day supply \$45 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Preferred: \$100 for up to a 30-day supply \$200 for a 31-60 day supply \$300 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Non-Preferred: 50% of Plan allowance for up to a 90-day supply	Generic: \$10 for up to a 30-day supply \$20 for a 31-60 day supply \$30 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Preferred: 50% of Plan allowance up to a maximum of: \$500 for up to a 30-day supply \$1,000 for a 31-60 day supply \$1,500 for a 61-90 day supply or the retail pharmacy's usual and customary cost of the drug, whichever is less Non-Preferred: You pay 100% of all charges
Non-Network Retail Pharmacy	You pay 100% of all charges	You pay 100% of all charges
CVS Caremark Mail Service Pharmacy All copayments/coinsurance are for up to a 90-day supply per prescription. - Drugs and medications (including those administered during a non-covered admission or in a non-covered facility) that by Federal Law of the United States require a physician's prescription for their purchase, except those listed as not covered. - FDA approved contraceptive drugs and devices for women; - Diabetic medications and supplies, such as: - Insulin; - Needles and syringes for the administration of covered medications; - Blood glucose meter to be provided at no charge by the manufacturer. - Elevate Plus Option members may contact CVS Caremark Mail Service Pharmacy: 877-418-4746.	Generic: \$20 or the cost of the drug, whichever is less Preferred: \$200 or the cost of the drug, whichever is less Non-Preferred: 50% of Plan allowance	Not available

Covered medications and supplies - continued on next page

Benefits Description	You pay	
Covered medications and supplies (cont.)	Elevate Plus	Elevate
- Elevate Option members may contact their participating retail pharmacy or CVS Caremark Customer Care for specifics on how to obtain a free meter at 844-443-4279. - Prescription prenatal vitamins; - Covered ostomy supplies (please include the manufacturer's product number to ensure accurate fill of the product).	Generic: \$20 or the cost of the drug, whichever is less Preferred: \$200 or the cost of the drug, whichever is less Non-Preferred: 50% of Plan allowance	Not available
Contraceptive drugs and devices as listed in the Health Resources and Services Administration site https://www.hrsa.gov/womens-guidelines. Network and Non-Network Retail CVS Caremark Mail Service Pharmacy Contraceptive coverage is available at no cost to FEHB members. The contraceptive benefit includes at least one option in each of the HRSA-supported categories of contraception (as well as the screening, education, counseling, and follow-up care). Over the counter (prescription required) and prescription drugs approved by the FDA to prevent an unintended pregnancy are included. Any contraceptive that is not already available without cost sharing on the formulary can be accessed through the contraceptive exceptions process described on our website at geha.com/Family-Planning-Options or by calling CVS Caremark at 844-443-4279. Exception requests for contraceptive coverage will be processed within 24 hours of receiving complete information. Reimbursement for over-the-counter contraceptives (prescription required) can be submitted by sending in your original prescription receipt obtained from your pharmacy to: CVS Caremark P.O. Box 52136 Phoenix, AZ 85072-2136 You may also submit prescription reimbursement requests online via Caremark web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple). Notes: For more information regarding prescription contraceptives, please refer to <i>Preventive care medications</i> in this section. Some contraceptives and services are covered under the medical benefit; see Section 5(a), <i>Family planning</i>.	Nothing (no deductible)	Nothing (no deductible)

Covered medications and supplies - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay	
Covered medications and supplies (cont.)	Elevate Plus	Elevate
Members are encouraged not to use an HSA, health FSA, or HRA (including any related debit card) to purchase contraception for which the individual intends to seek reimbursement from their FEHB plan.	Nothing (no deductible)	Nothing (no deductible)

Specialty drug benefits

CVS Specialty Pharmacy is the exclusive provider for specialty medications. CVS Specialty Pharmacy provides not only your specialty medications, but also personalized pharmacy care management services. If you have questions, visit geha.com/Prescriptions-Summary or call Specialty Customer Care at 800-237-2767.

Specialty medications are certain pharmaceuticals which may be biotech or biological drugs. Specialty medications are oral, injectable or infused, and/or may require special handling. To maximize patient safety, most specialty medications require preauthorization. These drugs are used in the treatment of complex, chronic medical conditions which include but are not limited to hemophilia, multiple sclerosis, hepatitis, cancer, rheumatoid arthritis, pulmonary hypertension, transplant, HIV, osteoarthritis, growth hormone therapy, and immune deficiency. If you are new to select specialty therapies (i.e., oral oncology, hepatitis B, Parkinson's disease psychosis and hematological disorders), you will receive a 14- or 15-day supply for the first 2 months of therapy. Your copayment/coinsurance will be prorated. If you continue on this therapy, you may receive up to a 30-day supply of the medication.

Your benefits may include the Advanced Control Specialty Formulary (ACSF); please see *CVS Caremark formulary* for additional information. Most specialty drugs **require preauthorization**. See "How to obtain preauthorization" under *Prescription drug benefits*. For certain specialty therapies, you are required to use the generic unless your physician demonstrates medical necessity for the brand.

Outpatient, non-surgical cancer treatments **require preauthorization**. You or your provider will need to call us at 800-821-6136 or visit geha.com/Provider-Partner-Tools.

*Your specialty benefit is limited to a 30-day supply. However, some specialty medications may not be available in a 30-day supply. Your coinsurance/copayment will be based on days of therapy (length of time medication remains in your system).

Benefits Description	You pay	
Specialty drug benefits	Elevate Plus	Elevate
CVS Specialty Pharmacy All coinsurances are for up to a 30-day supply per prescription. Copay maximums apply per each 30-day supply. Specialty Plan benefits apply to limited distribution specialty medications when CVS Specialty Pharmacy does not have access to dispense. Notes: - Recurring oral medications must be obtained through the pharmacy benefit. - If Medicare denies coverage, G.E.H.A does not waive the coinsurance.	Generic and Preferred: 40% of Plan allowance up to a maximum of: \$700 for up to a 30-day supply \$1,400 for up to a 60-day supply* \$2,100 for up to a 90-day supply* Non-preferred: 50% of Plan allowance, for up to a 30-day supply*	Generic and Preferred: 50% of Plan allowance up to a maximum of: \$500 for up to a 30-day supply \$1,000 for up to a 60-day supply* \$1,500 for up to a 90-day supply* Non-preferred: You pay 100% of all charges
Specialty medications dispensed by other sources including physician offices, home health agencies, outpatient hospitals may be paid under the medical benefit. If Medicare denies coverage your cost share is not waived. Recurring oral medications must be obtained through the pharmacy benefit.	You pay after the calendar year deductible: Generic and Preferred: \$500 copayment applies per prescription fill and 40% of the Plan allowance, up to a 30-day supply.	You pay after the calendar year deductible: Generic and Preferred: \$500 copayment applies per prescription fill and 50% of the Plan allowance, up to a 30-day supply.

Specialty drug benefits - continued on next page

Elevate Plus and Elevate Options

Benefits Description	You pay	
Specialty drug benefits (cont.)	Elevate Plus	Elevate
	• Non-Preferred: \$500 copayment applies per prescription fill and 60% of the Plan allowance, up to a 30-day supply.	• Non-Preferred: You pay 100% of all charges.
Non-CVS Specialty Pharmacy retail purchase	You pay 100% of all charges	You pay 100% of all charges
Preventive care medications	Elevate Plus	Elevate
Preventive Care - The following preventive medications are covered as recommended under the Patient Protection and Affordable Care Act (ACA). Preventive medications with USPSTF A and B recommendations are covered with no cost-share at a participating pharmacy. These may include some over-the-counter vitamins, nicotine replacement medications, and low dose aspirin for certain patients. For current recommendations, go to www.uspreventiveservicestaskforce.org/BrowseRec/index/browse-recommendations. Age restrictions apply. To receive preventive care benefits, a prescription from a doctor must be presented to the pharmacy. • Aspirin - All single ingredient generic oral dosage forms less than or equal to 81mg OTC only (requires a prescription) for the prevention of pre-eclampsia after 12 weeks of gestation. Limit of 100 units per fill. • Colorectal Cancer Prevention - Bowel prep products - generic RX, and brand name only when generic or over the counter (OTC) equivalent is not available, requires a prescription, age 45-75 years. • Fluoride supplements (not toothpaste or rinses) - Single ingredient brand name and generic prescription products in an oral dosage form less than 0.5mg for children 5 years of age and younger. • Folic acid supplements - Single ingredient generic 0.4mg and 0.8mg tabs. OTC only (requires a prescription) for women 55 years of age and younger. Limit of 100 units per fill. • Generic metformin - 850mg tablets for individuals age 35-70 years with no prior use of anti-diabetic medications. • Generic tamoxifen, raloxifene, exemestane and anastrozole - with prescription for women ages 35 and over for the prevention of breast cancer. • HIV Pre-Exposure Prophylaxis - Prior authorization may be required for coverage. CVS Specialty Pharmacy is G.E.H.A's exclusive Specialty Pharmacy. • Iron supplements - Single ingredient pediatric oral liquids (requires a prescription) for children age 6-12 months.	Nothing (no deductible)	Nothing (no deductible)

Preventive care medications - continued on next page

Benefits Description	You pay	
Preventive care medications (cont.)	Elevate Plus	Elevate
• Opioid rescue agent under this Plan with no cost share when obtained from a network pharmacy with a prescription. Limited to three doses annually (requires a prescription). For more information, consult the FDA guidance at: www.fda.gov/consumers/consumer-updates/access-naloxone-can-save-life-during-opioid-overdose. Or call SAMHSA's National Helpline at 800-662-4357 or go to www.findtreatment.samhsa.gov. Prior authorization may be required on some formulations. • Statins - Certain statins for individuals age 40-75 years. • Women's Preventive Service - Contraceptives - oral, emergency, injectable, patch, barrier, and misc. - generic Rx or OTC (requires a prescription) and brand name only when generic is not available. If the brand name is medically necessary, a preauthorization for medical necessity is required. Women only and limits may apply.	Nothing (no deductible)	Nothing (no deductible)
• Immunizations: Vaccines; childhood and adult, Rx only - Coverage dependent on vaccine type. - G.E.H.A members can go to a participating retail pharmacy to receive certain vaccinations. Influenza vaccine is commonly administered by retail pharmacies. Other vaccines, such as those for pneumococcal pneumonia (Pneumovax), varicella/shingles (Shingrix) and hepatitis B may also be available through retail pharmacies. Note: Members may call CVS Caremark at 844-443-4279 to identify a participating vaccine pharmacy or go to geha.com/Prescriptions-Summary. G.E.H.A members should check with the retail pharmacy to ensure availability of a pharmacist who can inject vaccines and availability of the vaccine product before going to the pharmacy. G.E.H.A members should also ask retail pharmacies if there is an age requirement for vaccines that can be administered at that pharmacy.	Nothing (no deductible)	Nothing (no deductible)
• Tobacco cessation - Gum, lozenge, patch, inhaler, spray and oral therapy, brand name and generic coverage, Rx and OTC (requires a prescription). - We will cover over the counter (with a physician's prescription) and prescription tobacco cessation drugs approved by the FDA. The quantity of drugs reimbursed will be subject to recommended courses of treatment. You may obtain tobacco cessation drugs with your G.E.H.A ID card, through a participating network retail pharmacy or CVS Caremark Mail Service Pharmacy (<i>Elevate Plus only</i>), (see previous section Covered medications and supplies for filing instructions). Note: For additional information on Tobacco Cessation Educational Classes and Programs, see Section 5(a).	Nothing (no deductible), day supply limits apply depending on therapy	Nothing (no deductible), day supply limits apply depending on therapy

Benefits Description	You Pay	
Non-covered medications and supplies	Elevate Plus	Elevate
<i>Not covered:</i> • <i>Drugs and supplies for cosmetic purposes</i> • <i>Drugs to enhance athletic performance</i> • <i>Drugs obtained at a non-Plan pharmacy; except for out-of-area emergencies</i> • <i>Vitamins, nutrients and food supplements (alone or in combination) not listed as a covered benefit or that do not require a prescription are not covered, including enteral formula/tube feeding nutrition available without a prescription. See Section 5(a), Treatment therapies for more information</i> • <i>Nonprescription medications unless specifically indicated elsewhere</i> • <i>Services or supplies for the administration of a non-covered medication</i> • <i>Medical devices, or supplies such as dressings and antiseptics</i> • <i>Drugs which are investigational</i> • <i>Drugs to treat impotency</i> • <i>Certain prescription drugs that have an over the counter (OTC) equivalent drug or treatment are not covered</i> • <i>Certain compounding chemicals, including, but not limited to, OTC products, experimental, investigational, bulk powders, bulk chemicals, and certain bases</i> • <i>Drugs prescribed in connection with Sex-Trait Modification for treatment of gender dysphoria</i> <i>If you are mid-treatment under this Plan, within a surgical or chemical regimen for Sex-Trait Modification for diagnosed gender dysphoria, for services for which you received coverage under the 2025 Plan brochure, you may seek an exception to continue care for that treatment. Ongoing sex-trait modification hormone treatment is not a covered benefit under this plan as of 1/1/2026. Our exception process is as follows: To initiate the exception process, contact Customer Care at 800-821-6136, Monday-Friday, 8 a.m. - 8 p.m. E.T. If you disagree with our decision on your exception, please see Section 8 of this brochure for the disputed claims process.</i> <i>Individuals under age 19 are not eligible for exceptions related to services for ongoing surgical or hormonal treatment for diagnosed gender dysphoria.</i>	<i>All charges</i>	<i>All charges</i>

Section 5(g). Dental Benefits

Important things you should keep in mind about these benefits:

- Please remember that all benefits are subject to the definitions, limitations, and exclusions in this brochure and are payable only when we determine they are medically necessary.
- **If you are enrolled in a Federal Employees Dental/Vision Insurance Program (FEDVIP) Dental Plan, your FEHB Plan will be First/Primary payor of any Benefit payments and your FEDVIP Plan is secondary to your FEHB Plan. See Section 9 Coordinating benefits with other coverage.**
- Under the **Elevate Plus** Option:
 - The calendar year deductible is \$200 per person (\$400 if enrollment is Self Plus One or Self and Family). We added "(no deductible)" to show when the calendar year deductible does not apply.
- Under the **Elevate** Option:
 - The calendar year deductible is \$750 per person (\$1,500 if enrollment is Self Plus One or Self and Family). If you use an out-of-network provider, the calendar year deductible is \$2,000 per person (\$4,000 if enrollment is Self Plus One or Self and Family). Applicable coinsurance applies after you met your Deductible until you've reached your Out-of-Pocket Maximum. We added "(no deductible)" to show when the calendar year deductible does not apply.
- Be sure to read Section 4, *Your Costs for Covered Services*, for valuable information about how cost-sharing works. Also, read Section 9, *Coordinating Benefits with Medicare and Other Coverage*, for information about how we pay if you have other coverage, or if you are age 65 or over.
- We cover hospitalization for dental procedures only when a non-dental physical impairment exists, which makes hospitalization necessary to safeguard the health of the patient. We do not cover the dental procedure. See Section 5(c), *Inpatient hospital* benefits.

Benefits Description	You pay	
Accidental injury benefits	Elevate Plus	Elevate
We cover restorative services and supplies necessary to promptly repair but not replace sound natural teeth. The need for these services must result from an accidental injury. • Repair to sound natural teeth such as expenses for x-rays, drugs, crowns, bridgework, inlays and dentures. Notes: • We may review x-rays and/or treatment records in order to determine benefit coverage. • See sections 5(a) through 5(f) for applicable services and benefits.	Services are paid at regular medical plan benefits.	Services are paid at regular medical plan benefits.
<i>Not covered:</i> • <i>Oral implants and transplants</i> • <i>Masticating (biting or chewing) incidents are not considered to be accidental injuries.</i>	<i>All charges</i>	<i>All charges</i>

Elevate Plus and Elevate Options

Benefits Description	You pay	
Dental Benefits	Elevate Plus	Elevate
We have no other dental benefits.	N/A	N/A

Section 5(h). Wellness and Other Special Features

Special features	Description
Flexible Benefits Option	Under the flexible benefits option, we determine the most effective way to provide services. - We may identify medically appropriate alternatives to regular contract benefits as a less costly alternative. If we identify a less costly alternative, we will ask you to sign an alternative benefits agreement that will include all of the following terms in addition to other terms as necessary. Until you sign and return the agreement, regular contract benefits will continue. - Alternative benefits will be made available for a limited time period and are subject to our ongoing review. You must cooperate with the review process. - By approving an alternative benefit, we do not guarantee you will get it in the future. - The decision to offer an alternative benefit is solely ours, and except as expressly provided in the agreement, we may withdraw it at any time and resume regular contract benefits. - If you sign the agreement, we will provide the agreed-upon alternative benefits for the stated time period (unless circumstances change). You may request an extension of the time period, but regular contract benefits will resume if we do not approve your request. - Our decision to offer or withdraw alternative benefits is not subject to OPM review under the disputed claims process. However, if at the time we make a decision regarding alternative benefits, we also decide that regular contract benefits are not payable, then you may dispute our regular contract benefits decision under the OPM disputed claim process (see Section 8).
Impaired Hearing Services	TTY service is available at 800-821-4833 for members who are hearing impaired.
Wellness Incentives	Earn rewards for healthy actions with G.E.H.A's Wellness Pays program. Total annual rewards are limited to \$500 each for the subscriber and covered spouse. Maximum reward amounts are not guaranteed. Rewardable activities include, but may not be limited to, the following: - Health assessment (must complete to be eligible for additional rewards) - Preventive cancer screenings (Cervical, Colorectal, and Breast) - Annual physical - Health and Wellness webinars Members will be issued a rewards account with a reloadable debit card, which can be used for eligible medical expenses. For detailed information about eligibility requirements, how to access the health assessment and all available rewards, visit geha.com/Reward-My-Health.
24-Hour Nurse Advice Line	Call the G.E.H.A 24-Hour Nurse Advice Line number 888-257-4342 and speak with a registered nurse – any time, 24 hours a day. The nurse can help you understand your symptoms and determine appropriate care for your needs. The 24-Hour Nurse Advice Line allows you to conveniently manage your symptoms and treatment anywhere you have access to a phone.
Telehealth	Telehealth is available at a reduced cost through the Plan's administrative partner. Go to geha.com/Telehealth-Visit or call 888-912-1183 to access on demand, affordable, high-quality care for adults and children experiencing non-emergency medical issues, including treatment of minor acute conditions (see Section 10, <i>Definitions of Terms</i>), dermatology conditions (see Section 10, <i>Definitions of Terms</i>), and counseling for mental health and substance use disorder. Notes: - This benefit is available at a reduced cost only through the Plan's administrative partner. - Practitioners must be licensed in the state where the patient is physically located at the time services are rendered.

Special features	Description
Member Portal	Your family's healthcare resources, in your hands whether at home or on the go. The online member portal provides instant access to your family's critical health information – anytime and anywhere. Whether you want to find a physician near you, check the status of a claim or speak directly with a healthcare professional, the portal is your go-to resource. Key features include: • Search for physicians or facilities by location or specialty • Store favorite physicians and facilities • View and share health plan ID card information • Access the Plan's telehealth administrative partner at geha.com/Telehealth-Visit • Check reward program status and activities • Can be personalized with individual member avatar, notes and reminders • Complete confidentiality • Check health-related financial account balance • Locate nearby convenience clinics, urgent care facilities, and emergency rooms • Check status of deductible and out-of-pocket spending For more information, visit geha.com/My-Member-Portal.
Weight Management	G.E.H.A offers a number of services and tools for weight management. • BMI calculation through on-line health risk assessment • Nutrition counseling (see Educational Classes and Programs, Section 5(a)) • Behavioral change programs with coaching for members who qualify • Discounts for gym memberships and other services through Connection Fitness®
G.E.H.A Maternity Program	G.E.H.A makes various maternity resources available to you or your covered dependent. Visit geha.com/Healthy-Pregnancy to order your packet on pregnancy and prenatal care and to learn more about the maternity program.
Personal Health Record	Our Personal Health Record helps you track health conditions, allergies, medications and more. This program is voluntary and confidential.
Value Added Programs and Services	G.E.H.A offers a number of programs and services to members to assist with special conditions and needs. Members with these conditions or needs can work with health professionals, such as a nurse or health coach. Visit geha.com/Your-Health-Journey for a list of programs, program criteria, and contact information. Please note that eligibility for these services may vary, and not all members will qualify.
Family Planning Care Program	G.E.H.A Care Management resources, and guidance are available to assist members or covered dependents through the infertility process. Visit geha.com/Family-Planning-Support. Note: Infertility coverage is limited. See Section 5(a), <i>Infertility services</i> for covered services.
Preconception Program	G.E.H.A Care Management resources, and guidance are available to members or covered dependents who are considering a future pregnancy and want to optimize their own health and well-being prior to conception. Visit geha.com/Pregnancy-Prep.
Clinical Assistance Programs	G.E.H.A offers several programs to support and partner with you during your healthcare journey. When you are experiencing a new diagnosis or struggling with managing your health, G.E.H.A is here to support you by offering various programs. You may find out more about our programs at geha.com/Your-Health-Journey. A nurse case manager may contact you to participate in one of these programs and provide assistance with the coordination of your care, education and clinical support. Please note that eligibility for these services may vary, and not all members will qualify.

Special features	Description
Health Education Resources	Visit our website at geha.com/Provider-Partner-Tools for information on: • General health topics • Links to healthcare news • Cancer and other specific diseases • Drugs/medication interactions • Kids' health • Patient safety information • Helpful website links
Care Support	G.E.H.A has a strong patient safety program. Pharmacy initiatives help ensure that members have fewer health complications related to prescription drugs. Disease management programs help our members with specific health conditions such as heart disease and diabetes. Medical case managers assist patients with high-risk pregnancies, durable medical equipment, transplants and other special needs. Patient safety information is available online at geha.com/Provider-Partner-Tools.

Non-FEHB Benefits Available to Plan Members

The benefits in this Section are not part of the FEHB contract or premium, **and you cannot file an FEHB disputed claim about them.** Fees you pay for these services do not count toward FEHB deductibles or catastrophic protection out-of-pocket maximums. These programs and materials are the responsibility of the Plan, and all appeals must follow their guidelines. For additional information contact the Plan at 800-821-6136 or visit our website at geha.com/Get-Support.

Connection Hearing® -844-513-0890 - geha.com/Hearing-Savings

G.E.H.A members and their families, including over-age children, domestic partners, same-sex spouses, parents, and grandparents, can save 30% to 60% off the average retail price of hearing aids through the Plan's hearing partner. Shop from a selection of more than 100 of the latest hearing aids from the top hearing aid manufacturers in the world.

Call to set up an appointment with a provider in your area or go online to learn more.

Elevate Plus only: Benefit is payable per person every 36 months for adults and every 12 months for children up to age 22.

Connection Vision® - 877-808-8538 - geha.com/Save-On-Vision

Connection Vision provides eye care to you and your covered family members at low cost and is available at no extra cost to G.E.H.A members. Connection Vision includes low copays for eye exams and discounts on lenses, frames, and contact lenses. Call, or go online to find a list of participating locations.

Connection Fitness® - 800-821-6136 - geha.com/Fit-For-Less

G.E.H.A promotes healthy lifestyles and fitness activities. G.E.H.A health plan members can take advantage of our Connection Fitness program including discounts on gym memberships, access to online tools, and activity tracking. Access to more than 12,200 nationwide participating fitness centers and more than 9,700 digital workout videos for a minimal monthly fee (plus a small, one-time enrollment fee and applicable taxes).

Connection Dental® - 800-296-0776 - geha.com/Discount-Dental

Free to G.E.H.A health plan members, Connection Dental® can reduce your costs for dental care. Connection Dental is a network of more than 190,000 provider locations nationwide. Participating providers have agreed to limit their charges to reduced fees for G.E.H.A health plan members. To find a participating Connection Dental provider in your area, call or go online.

Connection Dental Plus® - 833-434-2988 - geha.com/More-Dental-Care

Available for an additional premium, Connection Dental Plus® is a supplemental dental plan that pays benefits for a wide variety of procedures. Enrollment is open year-round to all current and former Federal employees, retirees and annuitants, including those who are not members of the G.E.H.A health plan. Learn more about eligibility and coverage by calling or going online.

Oral Care Program - 855-944-8361 - geha.com/Smile-Savings

G.E.H.A members save up to 70% off a premium electric toothbrush and 20% off of the lowest-published price for professional teeth-whitening when using our preferred partner. Supplies such as custom-fitted trays, teeth whitening gel and desensitizing gel can be ordered online. Call or go online to learn more about the brands and products offered by G.E.H.A.

Emergency Response Services - 800-640-0518 - geha.com/MedicalAlert

G.E.H.A members and their extended family (including over-age children, domestic partners, same-sex spouses, parents and grandparents) can receive discounted emergency response services. To receive this G.E.H.A discount (free activation plus a 10% discount on your monthly billing), call and request a free brochure. A representative will follow up after the brochure arrives to discuss your options.

Section 6. General Exclusions - Services, Drugs and Supplies We Do Not Cover

The exclusions in this section apply to all benefits. There may be other exclusions and limitations listed in Section 5 of this brochure. Although we may list a specific service as a benefit, we will not cover it unless we determine it is medically necessary to prevent, diagnose, or treat your illness, disease, injury, or condition. For information on obtaining preauthorization for specific services, such as transplants, see Section 3, *Other services that require preauthorization*.

We do not cover the following:

- Services, drugs, or supplies you receive while you are not enrolled in this Plan.
- Services, drugs, or supplies not medically necessary.
- Services, drugs, or supplies not required according to accepted standards of medical, dental, or psychiatric practice.
- Experimental or investigational procedures, treatments, drugs or devices (see specifics regarding transplants).
- Services, drugs, or supplies related to abortions, except when the life of the mother would be endangered if the fetus were carried to term, or when the pregnancy is the result of an act of rape or incest.
- Services, drugs, or supplies you receive from a provider or facility barred from the FEHB Program.
- Services or supplies for which no charge would be made if the covered individual had no health insurance coverage.
- Services or supplies we are prohibited from covering under the Federal Law.
- Services, drugs, or supplies you receive without charge while in active military service.
- Services, drugs or supplies furnished, ordered or billed by yourself, immediate relatives or household members, such as spouse, parents, children, brothers or sisters by blood, marriage or adoption.
- Services or supplies furnished or billed by a non-covered facility, except that medically necessary prescription drugs and physical, occupational and speech therapy rendered by a qualified professional therapist on an outpatient basis are covered subject to Plan limits.
- Charges which the enrollee or Plan has no legal obligation to pay, such as excess charges for an annuitant age 65 or older who is not covered by Medicare Parts A and/or B (see Section 9, *Coordinating Benefits with Medicare and Other Coverage*), doctor's charges exceeding the amount specified by the Department of Health & Human Services when benefits are payable under Medicare (limiting charge, see Section 9, *Coordinating Benefits with Medicare and Other Coverage*), or State premium taxes however applied.
- Any benefits or services required solely for your employment are not covered by this Plan.
- Chemical or surgical modification of an individual's sex traits through medical interventions (to include "gender transition" services), other than mid-treatment exceptions, see Section 3, *How You Get Care*.

Section 7. Filing a Claim for Covered Services

This Section primarily deals with post-service claims (claims for services, drugs or supplies you have already received).

See Section 3, *How You Get Care*, for information on pre-service claims procedures (services, drugs or supplies requiring prior plan approval), including urgent care claims procedures.

How to claim benefits

To obtain claim forms, claims questions or assistance, or answers about our benefits, contact us at 800-821-6136, or at our website at geha.com/My-Member-Portal.

In most cases, providers and facilities file HIPAA compliant electronic claims for you. In cases where a paper claim must be used, the provider must file on the form CMS-1500, Health Insurance Claim Form. Your facility will file on the UB-04 form.

Note: The Elevate Plus Option does not provide out-of-network benefits except in cases of emergency.

Submit dental claims, or out-of-network charges that you have paid in full to:

G.E.H.A Dental Claims
PO Box 21191
Eagan, MN 55121

Submit medical and Medicare primary claims, or out-of-network charges that you have paid in full to:

G.E.H.A Medical Claims
PO Box 21172
Eagan, MN 55121

When you must file a claim - such as for services you received overseas or when another group health plan is primary - submit it on the CMS-1500 or a claim form that includes the information shown below or visit geha.com/UM-Care-Form. Bills and receipts should be itemized and show:

- Patient's name, date of birth, address, phone number and relationship to enrollee;
- Patient's Plan identification number;
- Name and address of person or company providing the service or supply;
- Dates that services or supplies were furnished;
- Diagnosis;
- Type of each service or supply; itemized bill including valid codes such as ADA, CPT, HCPCS (including NDC numbers for all Drug type charges); and
- The charge for each service or supply
- Note: Canceled checks, cash register receipts, or balance due statements are not acceptable substitutes for itemized bills. Many direct-to-consumer program models do not support claim submissions to insurance carriers. They do not provide enough detailed, itemized, information to meet this claim submission criteria.

In addition:

- If another health plan is your primary payor, you must send a copy of the Explanation of Benefits (EOB) form you received from any primary payor (such as the Medicare Summary Notice (MSN)) with your claim.
- Bills for home nursing care must show that the nurse is a registered or licensed practical nurse and should include nursing notes.
- If your claim is for the rental or purchase of durable medical equipment; private duty nursing; and physical therapy, occupational therapy, or speech therapy, you must provide a written statement from the provider specifying the medical necessity for the service or supply and the length of time needed.
- Claims for prescription drugs and supplies must include receipts that show the prescription number, name of drug or supply, prescribing provider's name, date, and charge. A copy of the provider's script must be included with prescription drugs purchased outside the United States.

Post-service claims procedures	We will notify you of our decision within 30 days after we receive your post-service claim. If matters beyond our control require an extension of time, we may take up to an additional 15 days for review and we will notify you before the expiration of the original 30-day period. Our notice will include the circumstances underlying the request for the extension and the date when a decision is expected. If we need an extension because we have not received necessary information from you, our notice will describe the specific information required and we will allow you up to 60 days from the receipt of the notice to provide the information. If you do not agree with our initial decision, you may ask us to review it by following the disputed claims process detailed in Section 8 of this brochure.
Records	Keep a separate record of the medical expenses of each covered family member as deductibles and maximum allowances apply separately to each person. Save copies of all medical bills, including those you accumulate to satisfy a deductible. In most instances they will serve as evidence of your claim. We will not provide duplicate or year-end statements.
Deadline for filing your claim	Send us all of the documents for your claim as soon as possible. You must submit the claim by December 31 of the year after the year you received the service. If you could not file on time because of Government administrative operations or legal incapacity, you must submit your claim as soon as reasonably possible. Once we pay benefits, there is a three-year limitation on the re-issuance of uncashed checks.
Overseas claims	For covered services you receive by providers and facilities outside the United States and Puerto Rico, send a completed Overseas Claim Form and the itemized bills to: G.E.H.A, Medical Claims, PO Box 21172, Eagan, MN 55121. Obtain Overseas Claim Forms from geha.com/Health-Overseas-Access. Eligibility and/or medical necessity review is required when procedures are performed, or you are admitted to a hospital outside of the United States. Review includes the procedure/service to be performed, the number of days required to treat your condition, and any other applicable benefit criteria. If you have questions about the processing of overseas claims, contact us at 800-821-6136 or go to geha.com/My-Member-Portal. Covered providers outside the United States will be paid at the in-network level of benefits, subject to the Plan deductible, copayments and/or coinsurance. We will provide translation and currency conversion for claims for overseas (foreign) services. The conversion rate will be based on the date services were rendered. You may be required to pay for the services at the time you receive them and then submit a claim to us for reimbursement. Proof of payment is required to be submitted with Overseas Claim Form. Canceled checks, cash register receipts, or balance due statements are not acceptable. All foreign claim payments will be made directly to the enrollee. When members living abroad are stateside and seeking medical care, contact us at 800-821-6136, or visit geha.com/Locate-Care to locate an in-network provider. When you are seeking treatment stateside, all precertification and authorization requirements are applicable. If you utilize an out-of-network provider, out-of-network benefits would apply on the Elevate Option. The Elevate Plus Option does not provide out-of-network benefits except in cases of emergency. Note: It is recommended that any member traveling outside the United States or territories purchase Travel Insurance.
When we need more information	Please reply promptly when we ask for additional information. We may delay processing or deny benefits for your claim if you do not respond. Our deadline for responding to your claim is stayed while we await all of the additional information needed to process your claim.
Authorized Representative	You may designate an authorized representative to act on your behalf for filing a claim or to appeal claims decisions to us. For urgent care claims, a healthcare professional with knowledge of your medical condition will be permitted to act as your authorized representative without your express consent. For the purposes of this section, we are also referring to your authorized representative when we refer to you.

**Notice
Requirements**

The Secretary of Health and Human Services has identified counties where at least 10% of the population is literate only in certain non-English languages. The non-English languages meeting this threshold in certain counties are Spanish, Chinese, Navajo and Tagalog. If you live in one of these counties, we will provide language assistance in the applicable non-English language. You can request a copy of your Explanation of Benefits (EOB) statement, related correspondence, oral language services (such as phone customer assistance), and help with filing claims and appeals (including external reviews) in the applicable non-English language. The English versions of your EOBs and related correspondence will include information in the non-English language about how to access language services in that non-English language.

Any notice of an adverse benefit determination or correspondence from us confirming an adverse benefit determination will include information sufficient to identify the claim involved (including the date of service, the healthcare provider, and the claim amount, if applicable), and a statement describing the availability, upon request, of the diagnosis and procedure codes and its corresponding meaning, and the treatment code and its corresponding meaning.

Section 8. The Disputed Claims Process

You may appeal directly to the Office of Personnel Management (OPM) if we do not follow required claims processes. For more information or to make an inquiry about situations in which you are entitled to immediately appeal to OPM, including additional requirements not listed in Sections 3, 7 and 8 of this brochure, please call your plan's customer service representative at the phone number found on your enrollment card, plan brochure, or plan website.

Please follow this Federal Employees Health Benefits Program disputed claims process if you disagree with our decision on your post-service claim (a claim where services, referrals, drugs or supplies have already been provided). In Section 3, *If you disagree with our pre-service claim decision*, we describe the process you need to follow if you have a claim for services, drugs or supplies that must have prior Plan approval, such as inpatient hospital admissions.

To help you prepare your appeal, you may arrange with us to review and copy, free of charge, all relevant materials and Plan documents under our control relating to your claim, including those that involve any expert review(s) of your claim. To make your request, please contact our Customer Care Department by writing G.E.H.A Post Service Appeals, PO Box 21324, Eagan, MN 55121 or calling 800-821-6136.

For additional information on the Claims Dispute Process, go to geha.com/My-Member-Portal.

Our reconsideration will take into account all comments, documents, records, and other information submitted by you relating to the claim, without regard to whether such information was submitted or considered in the initial benefit determination.

When our initial decision is based (in whole or in part) on a medical judgment (i.e., medical necessity, experimental/investigational), we will consult with a healthcare professional who has appropriate training and experience in the field of medicine involved in the medical judgment and who was not involved in making the initial decision.

Our reconsideration will not take into account the initial decision. The review will not be conducted by the same person, or their subordinate, who made the initial decision.

We will not make our decisions regarding hiring, compensation, termination, promotion, or other similar matters with respect to any individual (such as a claims adjudicator or medical expert) based upon the likelihood that the individual will support the denial of benefits.

Step	Description
1	Ask us in writing to reconsider our initial decision. You must: a) Write to us within 6 months from the date of our decision. b) To do so you may log in at geha.com/My-Member-Portal and complete the online appeal submission form or send your request to us at: Pre-Service Appeals: G.E.H.A, PO Box 400046, San Antonio, TX 78229 or Post-Service Appeals: G.E.H.A, PO Box 21324, Eagan, MN 55121 c) Include a statement about why you believe our initial decision was wrong, based on specific benefit provisions in this brochure; and d) Include copies of documents that support your claim, such as physicians' letters, operative reports, bills, medical records, and Explanation of Benefits (EOB) forms. e) Include your email address (optional for member), if you would like to receive our decision via email. Please note that by giving us your email, we may be able to provide our decision more quickly.

	We will provide you, free of charge and in a timely manner, with any new or additional evidence considered, relied upon, or generated by us or at our direction in connection with your claim and any new rationale for our claim decision. We will provide you with this information sufficiently in advance of the date that we are required to provide you with our reconsideration decision to allow you a reasonable opportunity to respond to us before that date. However, our failure to provide you with new evidence or rationale in sufficient time to allow you to timely respond shall not invalidate our decision on reconsideration. You may respond to that new evidence or rationale at the OPM review stage described in step 4.
2	In the case of a post-service claim, we have 30 days from the date we receive your request to: a) Pay the claim; or b) Write to you and maintain our denial; or c) Ask you or your provider for more information. You or your provider must send the information so that we receive it within 60 days of our request. We will then decide within 30 more days. If we do not receive the information within 60 days, we will decide within 30 days of the date the information was due. We will base our decision on the information we already have. We will write to you with our decision.
3	If you do not agree with our decision, you may ask OPM to review it. You must write to OPM within: • 90 days after the date of our letter upholding our initial decision; or • 120 days after you first wrote to us - if we did not answer that request in some way within 30 days; or • 120 days after we asked for additional information. Write to OPM at: United States Office of Personnel Management, Healthcare and Insurance, Federal Employees Insurance Operations, FEHB 2, 1900 E Street NW, Washington, DC 20415-3620. Send OPM the following information: • A statement about why you believe our decision was wrong, based on specific benefit provisions in this brochure; • Copies of documents that support your claim, such as physicians' letters, operative reports, bills, medical records, and Explanation of Benefits (EOB) forms; • Copies of all letters you sent to us about the claim; • Copies of all letters we sent to you about the claim; • Your daytime phone number and the best time to call; and • Your email address, if you would like to receive OPM's decision via email. Please note that by providing your email address, you may receive OPM's decision more quickly. Notes: • If you want OPM to review more than one claim, you must clearly identify which documents apply to which claim. • You are the only person who has a right to file a disputed claim with OPM. Parties acting as your representative, such as medical providers, must include a copy of your specific written consent with the review request. However, for urgent care claims, a healthcare professional with knowledge of your medical condition may act as your authorized representative without your express consent. • The above deadlines may be extended if you show that you were unable to meet the deadline because of reasons beyond your control.

4	OPM will review your disputed claim request and will use the information it collects from you and us to decide whether our decision is correct. OPM will send you a final decision or notify you of the status of OPM's review within 60 days. There are no other administrative appeals. If you do not agree with OPM's decision, your only recourse is to file a lawsuit. If you decide to sue, you must file the suit against OPM in Federal court by December 31 of the third year after the year in which you received the disputed services, drugs, or supplies or from the year in which you were denied precertification or preauthorization. This is the only deadline that may not be extended. OPM may disclose the information it collects during the review process to support their disputed claim decision. This information will become part of the court record. You may not file a lawsuit until you have completed the disputed claims process. Further, Federal law governs your lawsuit, benefits, and payment of benefits. The Federal court will base its review on the record that was before OPM when OPM decided to uphold or overturn our decision. You may recover only the amount of benefits in dispute.

Note: **If you have a serious or life-threatening condition** (one that may cause permanent loss of bodily functions or death if not treated as soon as possible), and you did not indicate that your claim was a claim for urgent care, then call us at 800-821-6136. We will expedite our review (if we have not yet responded to your claim); or we will inform OPM so they can quickly review your claim on appeal. You may call OPM's Health Insurance 2 at 202-606-3818 between 8 a.m. and 5 p.m. Eastern Time.

Please remember that we do not make decisions about plan eligibility issues. For example, we do not determine whether you or a family member is covered under this plan. You must raise eligibility issues with your Agency personnel/payroll office if you are an employee, your retirement system if you are an annuitant or the Office of Workers' Compensation Programs if you are receiving Workers' Compensation benefits.

Section 9. Coordinating Benefits with Medicare and Other Coverage

When you have other health coverage or auto insurance

You must tell us if you or a covered family member has coverage under any other health plan or has automobile insurance that pays healthcare expenses without regard to fault. This is called “double coverage”.

When you have double coverage, one plan normally pays its benefits in full as the primary payor and the other plan pays a reduced benefit as the secondary payor. We, like other insurers, determine which coverage is primary according to the National Association of Insurance Commissioners’ (NAIC) guidelines. For more information on NAIC rules regarding the coordinating of benefits, visit naic.org or geha.com/Multiple-Plans.

When we are the primary payor, we will pay benefits described in this brochure.

When we are the secondary payor, we will determine our allowance. After the primary plan processes the benefit, we will pay what is left of our allowance, minus any cost share you may owe, up to our regular benefit, or up to the member’s responsibility as determined by the primary plan, whichever is less. We will not pay more than what we would have paid if we were primary. When applicable, authorization must be given to Us to obtain information about benefits or services available from the other coverage, or to recover overpayments if this Plan previously paid as primary. All terms and conditions of this Plan, as secondary payor, remain in force.

If your primary payor requires preauthorization or requires you use designated facilities or provider for benefits to be approved, it is your responsibility to comply with these requirements. In addition, you must file the claim to your primary payor within the required time period. If you fail to comply with any of these requirements and benefits are denied by the primary payor, we will pay secondary benefits based on an estimate of what the primary carrier would have paid if you followed their requirements.

Please see Section 4, *Your Costs for Covered Services*, for more information about how we pay claims.

This plan always pays secondary to:

- Any medical payment, PIP or No-Fault coverage under any automobile policy available to you.
- Any plan or program which is required by law.

You should review your automobile insurance policy to ensure that uncoordinated medical benefits have been chosen so that the automobile insurance policy is the primary payer.

• TRICARE and CHAMPVA

TRICARE is the healthcare program for eligible dependents of military persons, and retirees of the military. TRICARE includes the CHAMPUS program. CHAMPVA provides health coverage to disabled Veterans and their eligible dependents. If TRICARE or CHAMPVA and this Plan cover you, we pay first. See your TRICARE or CHAMPVA Health Benefits Advisor if you have questions about these programs.

Suspended FEHB coverage to enroll in TRICARE or CHAMPVA: If you are an annuitant or former spouse, you can suspend your FEHB coverage to enroll in one of these programs, eliminating your FEHB premium. (OPM does not contribute to any applicable plan premiums.) For information on suspending your FEHB enrollment, contact your retirement or employing office. If you later want to re-enroll in the FEHB Program, generally you may do so only at the next Open Season unless you involuntarily lose coverage under TRICARE or CHAMPVA.

- **Workers' Compensation**

Every job-related injury or illness should be reported as soon as possible to your supervisor. Injury also means any illness or disease that is caused or aggravated by the employment as well as damage to medical braces, artificial limbs and other prosthetic devices. If you are a federal or postal employee, ask your supervisor to authorize medical treatment by use of form CA-16 before you obtain treatment. If your medical treatment is accepted by the Dept. of Labor Office of Workers' Compensation (OWCP), the provider will be compensated by OWCP. If your treatment is determined not job-related, we will process your benefit according to the terms of this plan, including use of in-network providers. Take form CA-16 and form OWCP-1500/HCFA-1500 to your provider, or send it to your provider as soon as possible after treatment, to avoid complications about whether your treatment is covered by this plan or by OWCP.

We do not cover services that:

- You (or a covered family member) need because of a workplace-related illness or injury that the Office of Workers' Compensation Programs (OWCP) or a similar federal or state agency determines they must provide; or
- OWCP or a similar agency pays for through a third-party injury settlement or other similar proceeding that is based on a claim you filed under OWCP or similar laws.

- **Medicaid**

When you have this Plan and Medicaid, we pay first.

Suspended FEHB coverage to enroll in Medicaid or a similar state-sponsored program of medical assistance: If you are an annuitant or former spouse, you can suspend your FEHB coverage to enroll in one of these state programs, eliminating your FEHB premium. For information on suspending your FEHB enrollment, contact your retirement or employing office. If you later want to re-enroll in the FEHB Program, generally you may do so only at the next Open Season unless you involuntarily lose coverage under the state program.

When other Government agencies are responsible for your care

We do not cover services and supplies when a local, state, or federal government agency directly or indirectly pays for them.

When others are responsible for injuries

If G.E.H.A pays benefits for an illness or injury for which you accrue a right of action, are entitled to compensation, or receive a settlement, judgment, or recovery from another party, you must agree to the provisions below. All G.E.H.A benefit payments in these circumstances are a condition of and a limitation on the nature, provision, or extent of coverage or benefits under the Plan, and remain subject to all of our contractual benefit limitations, exclusions, and maximums. By accepting these conditional benefits, you agree to the following:

- You or your representative must contact G.E.H.A's Subrogation Vendor, The Rawlings Company, LLC, at 855-967-6609 as soon as possible after the event(s) that resulted in the illness or injury, and provide all requested information, including prompt disclosure of the terms of all settlements, judgments, or other recoveries. You must sign any releases G.E.H.A requires to obtain information about any claim(s) for compensation from other sources you may have.
- You must include all benefits paid by G.E.H.A in any claim for compensation you or your representative assert against any tortfeasor, insurer, or other party for the injury or illness, and assign all proceeds recovered from any party, including your own and/or other insurance, to G.E.H.A for up to the amount of the benefits paid.
- When benefits are payable under the Plan in relation to the illness or injury, G.E.H.A may, at its option:

Enforce its right of subrogation, that is, take over your right to receive payments from other parties. You will transfer to G.E.H.A any rights you or your representative may have to take legal action arising from the illness or injury, and to recover any sums paid on your behalf as a result of that action; or

Enforce its right of reimbursement, that is, recover any sums paid on your behalf from any payment(s) you or your representative obtain from other parties. The right of reimbursement is cumulative with and not exclusive of the right of subrogation.

You must cooperate in doing what is reasonably necessary to assist us, and you must not take any action that may prejudice these rights of recovery. It is your duty to notify the plan within 30 days of the date when notice is given to any party, including an insurance company or attorney, of your intention to pursue or investigate a claim to recover damages or obtain compensation due to your injury, illness or condition. You and your agents or representatives shall provide all information requested by the plan or its representatives. You shall do nothing to prejudice your FEHB plan's subrogation or recovery interest or to prejudice the plan's ability to enforce the terms of this provision. This includes, but is not limited to, refraining from making any settlement or recovery that attempts to reduce or exclude the full cost of all benefits provided by the plan.

- To reimburse G.E.H.A on a first priority basis (i.e., before any other party) in full, up to the amount of benefits paid, out of any and all settlements, judgments, or other recoveries that you or your representative obtain from any source and no matter how characterized, designated, or apportioned (for example, as "pain and suffering only"). G.E.H.A enforces this right of reimbursement by asserting a lien against any and all recoveries obtained, including, but not limited to, first party Medpay, Personal Injury Protection, No-Fault coverage, Third-Party liability coverage, Uninsured and Underinsured coverage, personal liability umbrella coverage, and a workers compensation program or insurance policy. G.E.H.A's lien consists of the total benefits paid to diagnose or treat the illness or injury. G.E.H.A's lien applies first, regardless of the "make whole" and "common fund" doctrines. Your plan is not required to participate in or pay court costs or attorney fees to any attorney hired by you to pursue your damage claims.

G.E.H.A's lien extends to all expenses incurred prior to the settlement or judgment date, even if those expenses were not submitted to G.E.H.A for payment at the time you reimbursed G.E.H.A. The lien remains your obligation until it is satisfied in full. Failure to refund G.E.H.A or cooperate with our recovery efforts may result in an overpayment that can be collected from you.

The provisions of this section apply to all current or former plan participants and also to the parents, guardian, or other representative of a dependent child who incurs claims and is or has been covered by the plan. The plan's right to recover (whether by subrogation or reimbursement) shall apply to the personal representative of your estate, heirs or beneficiaries, administrators, legal representatives, successors, assignees, minors, and incompetent or disabled persons. "You" or "your" includes anyone on whose behalf the plan pays benefits. No adult covered person hereunder may assign any rights that it may have to recover medical expenses from any tortfeasor or other person or entity to any minor child or children of said adult covered person without the prior express written consent of the plan.

When you have Federal Employees Dental and Vision Insurance Plan (FEDVIP)

Some FEHB plans already cover some dental and vision services. When you are covered by more than one vision/dental plan, coverage provided under your FEHB plan remains as your primary coverage. FEDVIP coverage pays secondary to that coverage. When you enroll in a dental and/or vision plan on [BENEFEDS.gov](https://www.benefeds.gov) or by phone at 1-877-888-3337, (TTY 1-877-889-5680), you will be asked to provide information on your Plan so that your plans can coordinate benefits. Providing your Plan information may reduce your out-of-pocket cost.

Clinical trials

An approved clinical trial includes a phase I, phase II, phase III, or phase IV clinical trial that is conducted in relation to the prevention, detection, or treatment of cancer or other life-threatening disease or condition, and is either Federally-funded; conducted under an investigational new drug application reviewed by the Food and Drug Administration (FDA); or is a drug trial that is exempt from the requirement of an investigational new drug application.

If you are a participant in a clinical trial, this health plan will provide related care as follows, if it is not provided by the clinical trial:

- Routine care costs – costs for routine services such as doctor visits, lab tests, x-rays and scans, and hospitalizations related to treating the patient's condition, whether the patient is in a clinical trial or is receiving standard therapy.
- Extra care costs – costs related to taking part in a clinical trial such as additional tests that a patient may need as part of the trial, but not as part of the patient's routine care.
- Research costs – costs related to conducting the clinical trial such as research physician and nurse time, analysis of results, and clinical tests performed only for research purposes. These costs are generally covered by the clinical trials. This Plan **does not** cover these costs.

When you have Medicare

For more detailed information on "What is Medicare?" and "Should I Enroll in Medicare?" please contact Medicare at 800-633-4227, (TTY 877-486-2048) or at www.medicare.gov.

• The Original Medicare Plan (Part A or Part B)

The Original Medicare Plan (Original Medicare) is available everywhere in the United States. It is the way everyone used to get Medicare benefits and is the way most people get their Medicare Part A and Part B benefits now. You may go to any doctor, specialist, or hospital that accepts Medicare. The Original Medicare Plan pays its share, and you pay your share.

All physicians and other providers are required by law to file claims directly to Medicare for members with Medicare Part B, when Medicare is primary. This is true whether or not they accept Medicare.

When you are enrolled in Original Medicare along with this Plan, you still need to follow the rules in this brochure for us to cover your care.

Claims process when you have the Original Medicare Plan - You will probably not need to file a claim form when you have both our Plan and the Original Medicare Plan.

When we are the primary payor, we process the claim first.

When Original Medicare is the primary payor and you are an annuitant/ retiree, Medicare processes your claim first. In most cases, your claim will be coordinated automatically, and we will then provide secondary benefits for covered charges. To find out if you need to do something to file your claim, call us at 800-821-6136 or see our website at geha.com/Medicare-Options.

For members enrolled in the Elevate Option, we do NOT waive deductibles, coinsurance, or copayments for Medicare members.

For members enrolled in the Elevate Plus Option we waive some costs if the Original Medicare Plan is your primary payor as follows:

- **Inpatient hospital benefits:** If you are enrolled in Medicare Part A, we waive the copayment or the deductible and coinsurance.
- **Outpatient facility benefits:** If you are enrolled in Medicare Part B, we waive the copayment or the deductible and coinsurance.

- **Medical services and supplies provided by physicians and other healthcare professionals:** If you are enrolled in Medicare Part B, we waive the copayment or the deductible and coinsurance.
- **Office visits providers and MinuteClinic® (where available):** If you are enrolled in Medicare Part B, we waive the copayment or the deductible and coinsurance for office visits.

Please review the following examples which illustrates your cost share if you are enrolled in Medicare Part B and the Elevate Plus Option. If you purchase Medicare Part B, then we waive some costs because Medicare will be the primary payor.

Elevate Plus Option

Benefit Description: Deductible

You Pay without Medicare: In-Network: \$200
 You Pay without Medicare: Out-of-Network: N/A
 You Pay without Medicare Part B: In-Network: \$0
 You Pay without Medicare Part B: Out-of-Network: \$0

Benefit Description: Catastrophic Protection Out-of-Pocket Maximum

You Pay without Medicare: In-Network: \$7,000 self only/\$14,000 family
 You Pay without Medicare: Out-of-Network: N/A
 You Pay with Medicare Part B: In Network: \$7,000 self only/\$14,000 family
 You Pay with Medicare Part B: Out-of-Network: N/A

Benefit Description: Part B Premium Reimbursement Offered

You Pay without Medicare: In-Network: N/A
 You Pay without Medicare: Out-of-Network: N/A
 You Pay with Medicare Part B: In Network: N/A
 You Pay with Medicare Part B: Out-of-Network: N/A

Benefit Description: Primary Care Provider

You Pay without Medicare: In-Network: \$30
 You Pay without Medicare: Out-of-Network: N/A
 You Pay with Medicare Part B: In Network: \$0
 You Pay with Medicare Part B: Out-of-Network: \$0

Benefit Description: Specialist

You Pay without Medicare: In-Network: \$50
 You Pay without Medicare: Out-of-Network: N/A
 You Pay with Medicare Part B: In Network: \$0
 You Pay with Medicare Part B: Out-of-Network: \$0

Benefit Description: Inpatient Hospital

You Pay without Medicare: In-Network: 20% of Plan allowance

You Pay without Medicare: Out-of-Network: N/A

You Pay with Medicare Part B: In Network: \$0

You Pay with Medicare Part B: Out-of-Network: \$0

Benefit Description: Outpatient Hospital

You Pay without Medicare: In-Network: 20% of Plan allowance

You Pay without Medicare: Out-of-Network: N/A

You Pay with Medicare Part B: In Network: \$0

You Pay with Medicare Part B: Out-of-Network: \$0

Benefit Description: Incentives Offered

You Pay without Medicare: In-Network: N/A

You Pay without Medicare: Out-of-Network: N/A

You Pay with Medicare Part B: In Network: N/A

You Pay with Medicare Part B: Out-of-Network: N/A

You can find more information about how our Plan coordinates benefits with Medicare as outlined in our Medicare Benefits Guide at geha.com/Medicare-Options.

- **Tell us about your Medicare coverage**

You must tell us if you or a covered family member has Medicare coverage, and let us obtain information about services denied or paid under Medicare if we ask. You must also tell us about other coverage you or your covered family members may have, as this coverage may affect the primary/secondary status of this Plan and Medicare.

- **Private contract with your physician**

If you are enrolled in Medicare Part B, a physician may ask you to sign a private contract agreeing that you can be billed directly for services ordinarily covered by Original Medicare. Should you sign an agreement, Medicare will not pay any portion of the charges, and we will not increase our payment. Regardless of whether the physician requires you to sign an agreement, we will still limit our payment to the coinsurance amount we would have paid after Original Medicare's payment based on our Plan allowable and the type of service you receive. You may be responsible for paying the difference between the billed amount and the amount we paid.

- **Medicare Advantage (Part C)**

If you are eligible for Medicare, you may choose to enroll in and get your Medicare benefits from a Medicare Advantage plan. These are private healthcare choices (like HMOs and regional PPOs) in some areas of the country. To learn more about Medicare Advantage plans, contact Medicare at 800-633-4227, TTY: 877-486-2048 or at www.medicare.gov.

If you enroll in a Medicare Advantage plan, the following options are available to you;

This Plan and another plan's Medicare Advantage plan: You may enroll in another plan's Medicare Advantage plan and also remain enrolled in our FEHB plan. We will still provide benefits when your Medicare Advantage plan is primary, even out of the Medicare Advantage plan's network and/or service area (if you use our Plan providers). However, if you do go outside the Medicare Advantage plan's network and/or service area, we will not waive any of our copayments, coinsurance, or deductibles. If you enroll in a Medicare Advantage plan, tell us. We will need to know whether you are in the Original Medicare Plan or in Medicare Advantage plan so we can correctly coordinate benefits with Medicare.

Suspended FEHB coverage to enroll in a Medicare Advantage plan: If you are an annuitant or former spouse, you can suspend your FEHB coverage to enroll in a Medicare Advantage plan, eliminating your FEHB premium. (OPM does not contribute to your Medicare Advantage plan premium.) For information on suspending your FEHB enrollment, contact your retirement or employing office. If you later want to re-enroll in the FEHB Program, generally you may do so only at the next Open Season, unless you involuntarily lose coverage or move out of the Medicare Advantage plan's service area.

- **Medicare prescription drug coverage (Part D)** When we are the primary payor, we process the claim first. If you enroll in Medicare Part D and we are the secondary payor, we will review claims for your prescription drug costs that are not covered by Medicare Part D and consider them for payment under the FEHB plan.

Medicare always makes the final determination as to whether they are the primary payor. The following chart illustrates whether Medicare or this Plan should be the primary payor for you according to your employment status and other factors determined by Medicare. It is critical that you tell us if you or a covered family member has Medicare coverage so we can administer these requirements correctly. **(Having coverage under more than two health plans may change the order of benefits determined on this chart.)**

Primary Payor Chart		
A. When you - or your covered spouse - are age 65 or over and have Medicare and you...	The primary payor for the individual with Medicare is...	
	Medicare	This Plan
1) Have FEHB coverage on your own as an active employee		✓
2) Have FEHB coverage on your own as an annuitant or through your spouse who is an annuitant	✓	
3) Have FEHB through your spouse who is an active employee		✓
4) Are a reemployed annuitant with the Federal government and your position is excluded from the FEHB (your employing office will know if this is the case) and you are not covered under FEHB through your spouse under #3 above	✓	
5) Are a reemployed annuitant with the Federal government and your position is not excluded from the FEHB (your employing office will know if this is the case) and...		
• You have FEHB coverage on your own or through your spouse who is also an active employee		✓
• You have FEHB coverage through your spouse who is an annuitant	✓	
6) Are a Federal judge who retired under title 28, U.S.C., or a Tax Court judge who retired under Section 7447 of title 26, U.S.C. (or if your covered spouse is this type of judge) and you are not covered under FEHB through your spouse under #3 above	✓	
7) Are enrolled in Part B only, regardless of your employment status	✓ for Part B services	✓ for other services
8) Are a Federal employee receiving Workers' Compensation		✓*
9) Are a Federal employee receiving disability benefits for six months or more	✓	
B. When you or a covered family member...		
1) Have Medicare solely based on end stage renal disease (ESRD) and...		
• It is within the first 30 months of eligibility for or entitlement to Medicare due to ESRD (30-month coordination period)		✓
• It is beyond the 30-month coordination period and you or a family member are still entitled to Medicare due to ESRD	✓	
2) Become eligible for Medicare due to ESRD while already a Medicare beneficiary and...		
• This Plan was the primary payor before eligibility due to ESRD (for 30 month coordination period)		✓
• Medicare was the primary payor before eligibility due to ESRD	✓	
3) Have Temporary Continuation of Coverage (TCC) and...		
• Medicare based on age and disability	✓	
• Medicare based on ESRD (for the 30 month coordination period)		✓
• Medicare based on ESRD (after the 30 month coordination period)	✓	
C. When either you or a covered family member are eligible for Medicare solely due to disability and you...		
1) Have FEHB coverage on your own as an active employee or through a family member who is an active employee		✓
2) Have FEHB coverage on your own as an annuitant or through a family member who is an annuitant	✓	
D. When you are covered under the FEHB Spouse Equity provision as a former spouse	✓	

*Workers' Compensation is primary for claims related to your condition under Workers' Compensation.

When you are age 65 or over and do not have Medicare

Under the FEHB law, we must limit our payments for inpatient hospital care and physician care to those payments you would be entitled to if you had Medicare. Your physician and hospital must follow Medicare rules and cannot bill you for more than they could bill you if you had Medicare. You and the FEHB benefit from these payment limits. Outpatient hospital care and non-physician based care are not covered by this law; regular Plan benefits apply. The following chart has more information about the limits.

If you:

- are age 65 or over; and
 - do not have Medicare Part A, Part B, or both; and
 - have this Plan as an annuitant or as a former spouse, or as a family member of an annuitant or former spouse; and
 - are not employed in a position that gives FEHB coverage. (Your employing office can tell you if this applies.)
-

Then, for your inpatient hospital care:

- The law requires us to base our payment on an amount - the "equivalent Medicare amount" - set by Medicare's rules for what Medicare would pay, not on the actual charge.
- You are responsible for your applicable deductibles and coinsurance under this Plan.
- You are not responsible for any charges greater than the equivalent Medicare amount; we will show that amount on the Explanation of Benefits (EOB) form that we send you.
- The law prohibits a hospital from collecting more than the "equivalent Medicare amount".

When inpatient claims are paid according to a Diagnostic Related Group (DRG) limit (for instance, for admissions of certain retirees who do not have Medicare), we will pay 30% of the total covered amount as room and board charges and 70% as other charges and will apply your coinsurance accordingly.

And, for your physician care, the law requires us to base our payment and your coinsurance on:

- an amount set by Medicare and called the "Medicare approved amount," or
- the actual charge if it is lower than the Medicare approved amount.

If your physician: Participates with Medicare or accepts Medicare assignment for the claim and is a member of our network,
Then you are responsible for: your deductibles, coinsurance, and copayments.

If your physician: Participates with Medicare and is **not** in our network,
Then you are responsible for: your deductibles, coinsurance, copayments, and any balance up to the Medicare approved amount.

If your physician: Does not participate with Medicare,
Then you are responsible for: your deductibles, coinsurance, copayments, and any balance up to 115% of the Medicare approved amount.

If your physician: Does not participate with Medicare and is not a member of our network,
Then you are responsible for: your out-of-network deductibles, coinsurance, copayments and any balance up to 115% of the Medicare approved amount.

If your physician: Opts-out of Medicare via private contract,
Then you are responsible for: your deductibles, coinsurance, copayments, and any balance your physician charges.

It is generally to your financial advantage to use a physician who participates with Medicare. Such physicians are permitted to collect only up to the Medicare approved amount.

Physicians Who Opt-Out of Medicare

A physician may have opted-out of Medicare and may or may not ask you to sign a private contract agreeing that you can be billed directly for services ordinarily covered by Original Medicare. This is different than a non-participating doctor, and we recommend you ask your physician if they have opted-out of Medicare. Should you visit an opt-out physician, the physician will not be limited to 115% of the Medicare approved amount. You may be responsible for paying the difference between the billed amount and our regular in-network/out-of-network benefits.

Our Explanation of Benefits (EOB) form will tell you how much the physician or hospital can collect from you. If your physician or hospital tries to collect more than allowed by law, ask the physician or hospital to reduce the charges. If you have paid more than allowed, ask for a refund. If you need further assistance, call us.

When you have the Original Medicare Plan (Part A, Part B, or both)

We limit our payment to an amount that supplements the benefits that Medicare would pay under Medicare Part A (Hospital insurance) and Medicare Part B (Medical insurance), regardless of whether Medicare pays. Note: We pay our regular benefits for emergency services to an institutional provider, such as a hospital, that does not participate with Medicare and is not reimbursed by Medicare.

We use the Department of Veterans Affairs (VA) Medicare-equivalent Remittance Advice (MRA) when the statement is submitted to determine our payment for covered services provided to you if Medicare is primary, when Medicare does not pay the VA facility.

If you are covered by Medicare Part B and it is primary, your out-of-pocket costs for services that both Medicare Part B and we cover depend on whether your physician accepts Medicare assignment for the claim.

For Elevate Plus, if your physician **accepts** Medicare assignment, we waive some of your deductibles, copayments and coinsurance for covered charges.

If your physician **does not accept** Medicare assignment, you pay the difference between the “limiting charge” or the physician’s charge (whichever is less) and our payment combined with Medicare’s payment.

It is important to know that a physician who does not accept Medicare assignment may not bill you for more than 115% of the amount Medicare bases its payment on, called the “limiting charge.” The Medicare Summary Notice (MSN) that Medicare will send you will have more information about the limiting charge. If your physician tries to collect more than allowed by law, ask the physician to reduce the charges. If the physician does not, report the physician to the Medicare carrier that sent you the MSN form. Call us if you need further assistance.

Section 10. Definitions of Terms We Use in This Brochure

Accidental injury	An injury caused by an external force or element such as a blow or fall that requires immediate medical attention. Also included are animal bites, poisonings, and dental care required to repair injuries to sound natural teeth as a result of an accidental injury, not from biting or chewing.
Admission	The period from entry (admission) into a hospital or other covered facility until discharge. In counting days of inpatient care, the date of entry and the date of discharge are counted as the same day.
Advance care planning	The process of making decisions about future healthcare options in the event of a medical crisis. This might involve the appointment of a substitute decision maker or the completion of an advance care directive or similar document.
Artificial insemination	Artificial insemination is a surgical procedure for the introduction of sperm or semen into the vagina, cervix, or uterus to produce pregnancy.
Assignment	An authorization by you (the enrollee or covered family member) that is approved by us (the Carrier), for us to issue payment of benefits directly to the provider. • We reserve the right to pay you directly for all covered services. Benefits payable under the contract are not assignable by you to any person without express written approval from us, and in the absence of such approval, any assignment shall be void. • Your specific written consent for a designated authorized representative to act on your behalf to request reconsideration of a claim decision (or, for an urgent care claim, for a representative to act on your behalf without designation) does not constitute an Assignment. • OPM's contract with us, based on federal statute and regulation, gives you a right to seek judicial review of OPM's final action on the denial of a health benefits claim but it does not provide you with authority to assign your right to file such a lawsuit to any other person or entity. Any agreement you enter into with another person or entity (such as a provider, or other individual or entity) authorizing that person or entity to bring a lawsuit against OPM, whether or not acting on your behalf, does not constitute an Assignment, is not a valid authorization under this contract, and is void.
Assisted reproductive technology	Assisted reproductive technology (ART) includes all fertility treatments in which either eggs or embryos are handled. In general, ART procedures involve surgically removing the eggs from the ovaries, combining them with sperm in the laboratory, and returning them to the birthing person's body or donating them to another person. They do NOT include treatments in which only sperm are handled (i.e., intrauterine - or artificial - insemination) or procedures in which a birthing person takes medicine only to stimulate egg production without the intention of having eggs retrieved.
Calendar year	January 1 through December 31 of the same year. For new enrollees, the calendar year begins on the effective date of their enrollment and ends on December 31 of the same year.
Calendar year deductible	There is an in-network deductible and an out-of-network deductible for the entire Plan year for covered services - medical, prescription, inpatient, outpatient, mental health and chiropractic services - you must incur for almost all covered services and supplies before we start paying benefits.
Clinical trials cost categories	An approved clinical trial includes a phase I, phase II, phase III, or phase IV clinical trial that is conducted in relation to the prevention, detection, or treatment of cancer or other life-threatening disease or condition, and is either federally-funded; conducted under an investigational new drug application reviewed by the Food and Drug Administration (FDA); or is a drug trial that is exempt from the requirement of an investigational new drug application. If you are a participant in a clinical trial, this health plan will provide related care as follows, if it is not provided by the clinical trial:

- Routine care costs – costs for routine services such as doctor visits, lab tests, x-rays and scans, and hospitalizations related to treating the patient’s condition whether the patient is in a clinical trial or is receiving standard therapy.
- Extra care costs – costs related to taking part in a clinical trial such as additional tests that a patient may need as part of the trial, but not as part of the patient’s routine care.
- Research costs – costs related to conducting the clinical trial such as research physician and nurse time, analysis of results, and clinical tests performed only for research purposes are generally covered by the clinical trials.

Coinsurance See Section 4, *Your Costs for Covered Services*.

Compound medications A compound medication includes more than one ingredient and is custom made by a pharmacist according to your doctor's instructions. Compound prescriptions must contain a federal legend drug and the ingredients must be covered by the G.E.H.A benefit.

Congenital anomaly A condition existing at or from birth which is a significant deviation from the common form or norm. For purposes of this Plan, congenital anomalies include cleft lips, cleft palates, birthmarks, webbed fingers or toes and other conditions that the Plan may determine to be congenital anomalies. Surgical correction of congenital anomalies is limited to children under the age of 18 unless there is a functional deficit. In no event will the term congenital anomaly include conditions relating to teeth or intra-oral structures supporting the teeth.

Copayment See Section 4, *Your Costs for Covered Services*.

Cosmetic Any procedure or any portion of a procedure performed primarily to improve physical appearance and/or treat a mental condition through change in bodily form.

Cost-sharing See Section 4, *Your Costs for Covered Services*.

Covered services Services we provide benefits for, as described in this brochure.

Custodial care We do not provide benefits for custodial care, regardless of who recommends the care or where it is provided. The Carrier or its delegated medical professionals determine which services are custodial care.

Custodial care includes treatment, supplies or services, that are provided to a patient mainly to help with activities of daily living. These activities include but are not limited to:

- Service, supplies, and treatment that are designed mainly to train or assist the patient in personal hygiene or other activities of daily living rather than provide therapeutic treatment; or
- Personal care such as help ambulating, getting in and out of bed, eating by spoon, tube or gastrostomy, exercise, and dressing;
- Homemaking, such as preparing meals or special diets;
- Acting as companion or sitter;
- Supervising medication that can usually be self-administered;
- Physical, emotional, or behavioral treatment or services that can be provided by non-licensed caregivers with minimal instruction, including but not limited to recording temperature, pulse, and respirations, or administration and monitoring of feeding systems; and
- Services or treatment where further medical professional intervention is not expected to result in significant improvement in the member's condition. The member's condition is no longer demonstrating measurable progress towards established treatment goals that have been documented in the patient's treatment record.

Deductible See Section 4, *Your Costs for Covered Services*.

Dermatology conditions (Telehealth services)	Under the telehealth benefit, dermatologic conditions seen and treated include but are not limited to acne, rashes, eczema, suspicious spots/moles, warts and other abnormal bumps, rosacea, inflamed or enlarged hair follicles, psoriasis, cold sore, alopecia, insect bites.
Doula	A doula is a non-medical trained professional who provides emotional, physical, and informational support during pregnancy, labor/delivery, and post-partum periods. Doulas must be certified to provide doula services to meet the Plan requirements of a covered provider. Doulas eligible to provide services for any state Medicaid program in the United States, or certified by any organization recognized as providing acceptable training by any state Medicaid program will be considered a certified doula and eligible for reimbursement for services from the Plan. Training organizations include, but are not limited to: • Childbirth and Postpartum Professional Association (CAPPA) • Childbirth International • Commonsense Childbirth Institute • Doulas of North America (DONA) • Doula Trainings International (DTI) • International Childbirth Education Association • National Black Doulas Association Doula services do not include diagnosis of medical conditions, provision of medical advice, or any type of clinical assessment, exam, or procedure.
Durable medical equipment	Equipment and supplies that: • Are prescribed by your attending physician (i.e., the physician who is treating your illness or injury) • Are medically necessary • Are primarily and customarily used only for a medical purpose • Are generally useful only to a person with an illness or injury • Are designed for prolonged use • Serve a specific therapeutic purpose in the treatment of an illness or injury.
Effective date	The date the benefits described in this brochure are effective: • January 1 for continuing enrollments and for all annuitant enrollments; • The first day of the first full pay period of the new year for enrollees who change plans or options or elect FEHB coverage during the open season for the first time; and • For new enrollees during the calendar year, but not during the open season, the effective date of enrollment as determined by the employing office or retirement system.
Elective surgery	Any non-emergency surgical procedure that may be scheduled at the patient's convenience without jeopardizing the patient's life or causing serious impairment to the patient's bodily functions.
Expense	An expense is "incurred" on the date the service or supply is rendered.
Experimental or investigational services	A medical, surgical, diagnostic, psychiatric, mental health, substance use disorder, or other health care service, technology, supply, treatment, procedure, drug, medication, device, or biological product (referred to as "service") is experimental or investigational if the Plan determines any of the following to apply: 1. A service which cannot be lawfully marketed without approval of the U.S. Food and Drug Administration (FDA) and approval for marketing has not been given for the proposed use or patient-specific diagnosis or condition at the time it is furnished.

2. Reliable evidence shows that it is the subject of ongoing phase I, II, or III clinical trials or under study to determine its maximum tolerated dose, its toxicity, its safety, its efficacy, or its efficacy as compared with the standard means of treatment or diagnosis for the proposed use or patient-specific diagnosis or condition.
3. Reliable evidence shows that the consensus of opinion among experts is that further studies or clinical trials are necessary to determine its maximum tolerated dose, its toxicity, its safety, its efficacy, or its efficacy as compared with the standard means of treatment or diagnosis.
4. Reliable evidence shows that the healthcare service does not improve health outcome, is not more beneficial as established alternative(s) for the proposed use or patient specific condition or diagnosis, service does not produce improvement outside of the research setting or is otherwise not consistent with generally accepted standards of medical practice in the United States.
5. Reliable evidence shall mean credible scientific evidence published in peer-reviewed medical literature generally recognized by the relevant medical community, nationally recognized evidence-based clinical guidelines and criteria from independent sources, national physician specialty society recommendations, and the views of medical practitioners practicing in relevant clinical areas, and any other relevant factors. Please refer to geha.com/Coverage-Criteria for details on the literature used in medical decision making.

Determination of experimental/investigational status may require review of appropriate government publications such as those of the National Institute of Health, National Cancer Institute, Agency for Healthcare Policy and Research, Food and Drug Administration, National Library of Medicine, the American Hospital Formulary Service, or the United States American Hospital Pharmacopoeia Dispensing Information. We reserve the right to consult expert opinion in determining whether healthcare services are Experimental or Investigational.

Group health coverage	Healthcare coverage that a member or covered dependent is eligible for because of employment by, membership in, or connection with, a particular organization or group that provides payment for hospital, medical, dental or other healthcare services or supplies, including extension of any of these benefits through COBRA.
Habilitative	Therapy is initiated to address a genetic, congenital, or early acquired disorder resulting in significant deficit of Activities of Daily Living (ADL), fine motor, or gross motor skills. Therapy services are provided to enhance functional status and is focused on developing skills that were never present.
Healthcare professional	A physician or other healthcare professional licensed, accredited, or certified to perform specified health services consistent with state law.
Home health services	Medically necessary part-time or intermittent skilled nursing care by a licensed Home Health agency, when ordered by an appropriate medical professional, for services such as: • Wound care for pressure sores or a surgical wound • Patient and caregiver education • Intravenous or nutrition therapy • Injections • Monitoring serious illness and unstable health status
Iatrogenic infertility	An impairment of fertility by surgery, radiation, chemotherapy, or other medical treatment affecting reproductive organs including gonadotoxic therapies, or ovary or testicle removal for treatment of disease.
Infertility	Infertility is defined as the inability to conceive pregnancy within a 12-month period for individuals under age 35 (6 months for persons aged 35 or older) through unprotected intercourse or artificial insemination. Infertility may also be established through evidence of medical history and diagnostic testing. Infertility includes the need for medical intervention to conceive pregnancy either as an individual or with a partner, except following voluntary sterilization.

Inpatient care	Inpatient care is care rendered to a person who has been admitted to a hospital for bed occupancy for purposes of receiving inpatient hospital services. Generally, a patient is considered an inpatient if formally admitted as an inpatient with the expectation that he or she will remain at least overnight and occupy a bed even if it later develops that the patient can be safely discharged or transferred to another hospital and not actually use a hospital bed overnight. See Section 3, <i>How You Get Care, Covered facilities</i> , for the definition of an Acute Inpatient and Residential Treatment Center.
In vitro fertilization	In vitro fertilization (IVF) is a method of assisted reproduction that involves combining an egg with sperm in a laboratory dish. If the egg fertilizes and begins cell division, the resulting embryo may be transferred into the uterus where it may implant in the uterine lining and further develop, or be cryopreserved for later transfer. A cycle of IVF is defined as stimulation of ovaries, oocyte retrieval, and embryo transfer or preservation.
Long-term acute care	Often referred to as LTCH or LTAC, these facilities provide services for patients who need extended intensive or critical, hospital-level of care beyond that of the traditional short hospital stay. LTCH's specialize in treating patients who have more than one serious condition yet have the potential to improve with time and care and return to their previous health status. Generally, services are focused on respiratory therapy, head trauma treatment, and pain management.
Long-term care	We do not provide benefits for long-term care, regardless of who recommends the care or where it is provided. The Carrier or its delegated medical professionals determine which services are long-term care. A range of services and support provided to meet personal care needs on a long-term basis. While some medical care may be necessary, most of the care provided is not and does not require a licensed caregiver. This encompasses a spectrum of services provided in a variety of settings for people who do not have full independence because of a medical condition, injury, or chronic and/or behavioral illness. Long-term care is often used to provide custodial care as well as instrumental activities of daily living necessary for safety and health. Long-term care is usually custodial care that has lasted for 90 days or more yet can begin prior to 90 days dependent on the member's response to professional intervention. Long-term care and long-term acute care are not one and the same. See the definition of long-term acute care for more information about those services.
Medical emergency	A medical emergency is the sudden and unexpected onset of a condition or an injury that you believe endangers your life or could result in serious injury or disability and requires immediate medical, surgical, or behavioral health care (includes mental health and substance use disorders).
Medical foods for a diagnosis of Inborn Errors of Metabolism (IEM)	Inborn errors of metabolism are rare genetic (inherited) disorders in which the body cannot properly turn food into energy. The disorders are usually caused by defects in specific proteins (enzymes) that help break down (metabolize) parts of food. G.E.H.A will cover medical food for a diagnosis of IEM. Medical Food is defined as a food which is recommended by a physician after an evaluation and is intended to provide for the dietary management of a disease or condition that has specific nutritional requirements. G.E.H.A will not cover "grocery" food items that can routinely be obtained online or in stores (e.g., gluten-free breads).
Medical necessity	Healthcare services, drugs, supplies or equipment provided by a hospital or covered provider of the healthcare services for the purpose of preventing, evaluating, diagnosing, or treating a Medical or Behavioral Health Condition, that the Plan determines are all of the following: • Are clinically appropriate to diagnose or treat the patient's condition, illness or injury in terms of type, frequency, extent, site and duration of treatment, and treatment setting are considered effective for your condition; • Are consistent with generally accepted standards of medical practice in the United States.

- Generally accepted standards of medical practice are based on credible scientific evidence published in peer-reviewed medical literature generally recognized by the relevant medical community, nationally recognized evidence-based clinical guidelines and criteria from independent sources, national physician specialty society recommendations and the views of medical practitioners practicing in relevant clinical areas, and any other relevant factors. Please refer to geha.com/Coverage-Criteria for details on the literature used in medical decision making.
- Are not primarily for the personal comfort or convenience of the patient, the family, or the provider;
- Are not a part of or associated with the scholastic education or vocational training of the patient;
- Are not more costly than an alternate drug, service(s), or supply that is at least as likely to produce equivalent therapeutic or diagnostic results for the diagnosis or treatment of your condition;
- In the case of inpatient care, cannot be provided safely as observation or on an outpatient basis;
- Is not custodial or long-term care (see the Plan's definition).

The fact that a covered provider has prescribed, recommended, or approved a service, supply, drug or equipment does not, in itself, make it medically necessary. We reserve the right to consult expert opinion in determining whether healthcare services are Medically Necessary.

Medicare Part A	Part A helps cover inpatient hospital stays, skilled nursing facility care, hospice care, and some home health care.
Medicare Part B	Part B covers medically necessary services like doctors' services and tests, outpatient care, home health services, durable medical equipment, and other medical services.
Medicare Part C	Part C is a Medicare Advantage plan that combines the coverage of Medicare Part A and Part B. Part C typically also covers additional benefits like dental, vision, and hearing services. Part C plans also include Medicare Part D coverage.
Medicare Part D	Medicare Part D plans provide coverage for prescription drugs. Private insurers contract with CMS on an annual basis for the right to offer Part D plans. Part D can be offered as a standalone Prescription Drug Plan (PDP) or as part of a Medicare Advantage Prescription Drug Plan (MAPD).
Mental health/ substance use disorder	Conditions and diseases listed in the most recent edition of the International Classification of Diseases (ICD) as psychoses, neurotic disorders, or personality disorders; other nonpsychotic mental disorders listed in the ICD, to be determined by the Plan; or disorders listed in the ICD requiring treatment for use or dependence upon substances such as alcohol, narcotics, or hallucinogens; may also be collectively referred to as Behavioral Health conditions. Prior authorization is required for all of the following services and must be provided by a covered facility or covered provider as defined in Section 3, <i>How You Get Care</i> .

Inpatient Behavioral Health (includes mental health and substance use disorders):

- Acute Care Hospital: See Section 3, *Covered Facilities*.
- Residential Treatment Center (RTC): See Section 3, *Covered Facilities*.

Intensive Day Treatment:

Intensive day treatment programs are outpatient services that must be rendered on an outpatient basis. Regardless of where services are rendered, the provider and/or the facility, must be licensed to provide intensive day mental health and/or substance use treatment and must meet G.E.H.A's definition of a covered provider in Section 3. G.E.H.A does not cover room and board during intensive day treatment programs. Under the direction of a physician, services must be provided for at least two hours per day and may include group, individual, and family therapy along with psychoeducational services and adjunctive psychiatric medication management.

- **Partial Hospitalization Program (PHP):** A facility-based outpatient treatment program for mental health and/or substance use disorder conditions not requiring 24-hour care. Twenty or more hours of care per week is usually delivered at a minimum of four hours a day, five days a week. Time frames and frequency will vary based on condition, severity, and individual treatment plan.
- **Intensive Outpatient Program (IOP):** A comprehensive, structured outpatient treatment program that includes extended periods of individual or group therapy sessions for mental health and/or substance use disorders conditions. It is an intermediate level of care between traditional outpatient therapy and partial hospitalization, delivered in an outpatient facility or outpatient professional office setting. Nine or more hours of care per week is usually delivered at a minimum of three hours a day, three days a week. Time frames and frequency will vary based on condition, severity, and individual treatment plan.

Minor acute conditions Common, non-emergent conditions. Examples of common conditions include sinus problems, rashes, allergies, cold and flu symptoms, etc.

Never event policies Federal or State policies that bar healthcare providers from charging patients for care that is attributable to certain avoidable complications or errors, such as wrong site surgery.

Observation care Observation care is a well-defined set of specific, clinically appropriate services, which include ongoing short-term treatment, assessment, and reassessment, that are furnished while a decision is being made regarding whether patients will require further treatment as hospital inpatients or if they are able to be discharged from the hospital. Observation services are commonly ordered for patients who present to the emergency department and who then require a significant period of treatment or monitoring in order to make a decision concerning their admission or discharge.

Plan A Fee-for-Service Elevate and Elevate Plus health plan with a Preferred Provider Network administered by G.E.H.A as permitted through contract with Office of Personnel Management (OPM).

Plan allowance Our Plan allowance is the amount we use to determine our payment and your coinsurance for covered services. Fee-for-service plans determine their allowances in different ways.

Allowable expense (plan allowance) is a healthcare expense, including deductibles, coinsurance and copayments, that is covered at least in part by any plan covering the person. When a plan provides benefits in the form of services, the reasonable cash value of each service will be considered an allowable expense and a benefit paid.

We determine our Plan allowance as follows:

In-network providers: Our network allowances are negotiated with each provider who participates in the network. Network allowances may be based on a standard reduction or on a negotiated fee schedule. For these allowances, the in-network provider has agreed to accept the negotiated reduction and you are not responsible for this discounted amount. In these instances, the benefit paid plus your coinsurance equals payment in full.

Out-of-network providers: Applies to Elevate only; Elevate Plus does not offer out-of-network benefits except in cases of emergency.

We will determine the out-of-network Plan allowance by applying the following rules:

1. For emergent services, air ambulance, and services performed by certain out-of-network providers rendered at in-network facilities, the Plan allowance will be the "recognized amount" as defined by federal law.
2. The Plan allowance for covered services received from out-of-network providers, including Physicians or health care facilities, are determined by the ClearHealth CRS program and is based on a methodology which considers the following:

- The amount that is usually accepted by health care providers in the same geographical area (or greater area, if necessary) for the same services, treatment, or devices received by the member; or
- Current publicly available data (including but not limited to pricing data published by the US Department of Veteran Affairs, RJ Health, and Medicare) reflecting the costs for health care providers providing the same or similar services, treatment, or materials adjusted for geographical differences plus a margin factor above cost.
- Or, fee(s) that are negotiated with the physician or facility.

To estimate our maximum Plan allowance for a non-network provider before you receive services from them, call us at 800-821-6136. For more information, see *Differences between our allowance and the bill* in Section 4.

You should also see *Important Notice About Surprise Billing - Know Your Rights* in Section 4 that describes your protections against surprise billing under the No Surprises Act.

Post-service claims	Any claims that are not pre-service claims. In other words, post-service claims are those claims where treatment has been performed and the claims have been sent to us in order to apply for benefits.
Pre-service claims	Those claims 1) that require precertification or preauthorization and 2) where failure to obtain precertification or preauthorization results in a reduction of benefits.
Preauthorization	A decision made by your health plan that a healthcare service, treatment plan, drug, surgery, or durable medical equipment is medically necessary after review of medical information. Sometimes called prior approval.
Precertification	The process of collecting information and obtaining authorization from the health plan prior to an inpatient admission or other selected ambulatory procedures and services.
Primary care provider	Physician that focuses on a range of prevention, wellness, and treatment for common illnesses. Primary care providers include doctors, nurses, nurse practitioners, and physician assistants who are practicing within the scope of their license and are credentialed and designated by the Plan to be a primary care provider. They often maintain long-term relationships with patients and treat a range of health-related issues. Doctors listed in provider directories or advertisements under any other medical specialty or sub-specialty area (such as internal medicine doctors also listed under cardiology, or pediatric sub-specialties such as pediatric allergy) are considered specialists, not primary care providers. Chiropractors, eye doctors, dentists and audiologists, are not considered primary care providers.
Rehabilitative	Therapy is initiated to restore bodily function when there has been a total or partial loss of bodily function due to illness, surgery, or injury.
Reimbursement	A carrier's pursuit of a recovery if a covered individual has suffered an illness or injury and has received, in connection with that illness or injury, a payment from any party that may be liable, any applicable insurance policy, or a workers' compensation program or insurance policy, and the terms of the carrier's health benefits plan require the covered individual, as a result of such payment, to reimburse the carrier out of the payment to the extent of the benefits initially paid or provided. The right of reimbursement is cumulative with and not exclusive of the right of subrogation.
Sound natural tooth	A sound natural tooth is a whole or properly restored tooth that has no condition that would weaken the tooth or predispose it to injury prior to the accident, such as decay, periodontal disease, or other impairments. For purposes of the Plan, damage to a restoration, such as a prosthetic crown or prosthetic dental appliance (i.e., bridgework), would not be covered as there is no injury to the natural tooth structure.
Speech generating devices	Electronic voice output communication aids, which are electronic augmentative and alternative communication systems used to supplement or replace speech or writing for individuals with severe speech impairments.

Specialty medication	Specialty medications are biotech or biological drugs that are oral, injectable or infused, or may require special handling. To maximize patient safety, all specialty medications require preauthorization. These drugs are used in the treatment of complex, chronic medical conditions such as hemophilia, multiple sclerosis, hepatitis, cancer, rheumatoid arthritis, pulmonary hypertension, osteoarthritis, and immune deficiency.
Subrogation	A carrier's pursuit of a recovery from any party that may be liable, any applicable insurance policy, or a workers' compensation program or insurance policy, as successor to the rights of a covered individual who suffered an illness or injury and has obtained benefits from that carrier's health benefits plan.
Surgery	Surgery may include procedures such as but not limited to cutting (incision); removing (excision); abrading; manipulating (e.g., setting bones); stitching; probing; injections (e.g., intraarticular, trigger point); exposing to heat, cold, chemicals, light/laser energy, or certain forms of radiation (e.g., radiofrequency ablation, gamma knife); or other techniques designed to structurally alter tissue within the body for the purpose of diagnosing and treating diseases, injuries, or deformities.
Surprise bill	An unexpected bill you receive for: • Emergency care- when you have little or no say in the facility or provider from whom you receive care, or for • Non-emergency services furnished by nonparticipating providers with respect to patient visits to participating facilities, or for • Air ambulance services furnished by nonparticipating providers of air ambulance services.
Telehealth	Online/virtual doctor visits provided remotely by means of telecommunications technology.
Urgent care claims	A claim for medical care or treatment is an urgent care claim if waiting for the regular time limit for non-urgent care claims could have one of the following impacts: • Waiting could seriously jeopardize your life or health; • Waiting could seriously jeopardize your ability to regain maximum function; or • In the opinion of a physician with knowledge of your medical condition, waiting would subject you to severe pain that cannot be adequately managed without the care or treatment that is the subject of the claim. Urgent care claims usually involve pre-service claims and not post-service claims. We will determine whether or not a claim is an urgent care claim by applying the judgment of a prudent layperson who possesses an average knowledge of health and medicine. If you believe your claim qualifies as an urgent care claim, please contact our Customer Service Department at 800-821-6136. You may also prove that your claim is an urgent care claim by providing evidence that a physician with knowledge of your medical condition has determined that your claim involves urgent care.
Us/We	Us and We refer to Government Employees Health Association, Inc.
You	You refers to the enrollee and each covered family member.

Index

Do not rely on this page; it is for your convenience and may not show all pages where the terms appear.

Acupuncture

24-Hour Nurse Advice Line.....92

Abortion.....96

Accidental injury.....55-56, 70-72, 90

Admission.....15-23, 64-69, 76

Advanced Control Specialty Formulary (ACSF)

Allergy care.....42

Ambulance.....27-28, 69

Ambulatory surgical center.....16-17, 66-67

Anesthesia.....63, 66-67

Applied Behavioral Therapy (ABA).....20, 75

Artificial insemination.....20, 40-42

Assignment.....103-112

Assisted reproductive technology.....40-42

Biopsy.....52-63

Blood or blood plasma.....64-69

Bone marrow transplant...19-20, 42-43, 56-63, 66-67

Breast prostheses.....46-47, 54-55

Breast pump and supplies.....32-51

Calendar year.....113

Cardiac rehabilitation.....66-67

Casts.....64-69

Catastrophic out-of-pocket max...24-28, 124-125

Chemotherapy.....19-20, 42-43, 66-67

Chiropractic.....50

Circumcision.....38-39, 66-67

Claims.....100-102, 113-121

Clinical trials.....20, 106, 113-121

Coinsurance.....114

Compound medication

Congenital anomalies.....52-63, 96

Contraceptive devices and drugs...40, 83-88

Coordination of benefits.....103-112

Copayment.....24

Cosmetic.....20, 54-55, 96

Cost-sharing.....24

Covered facilities.....16-17

Covered providers.....15-16

Covered services.....114

Cryopreservation.....40-42

Custodial care.....114

Deductible.....24-25

Definitions.....113-121

Dental care.....91, 95

Diabetic supplies.....78-89

Disputed claims review.....100-102

Doula.....38-39, 115

Durable medical equipment.....20, 47-49

Educational classes and programs.....51

Effective date.....10, 113, 115

Emergency room.....70-72

Environmental medicine.....96

Experimental or investigational...96, 115-116

Family planning.....40, 93

Flexible benefits option.....92

Foot care.....46

Foster children.....7-11

Fraud.....3

Gamete intrafallopian transfer.....40-42

General exclusions.....96

Group health coverage.....116

Healthcare professional.....116

Hearing services.....45

Home health services.....49-50

Home uterine devices.....38-39

Hospice care.....68

Iatrogenic Infertility.....40-42, 116

Immunizations.....35-38, 87-88

Infertility.....40-42

Insulin

Intensive day treatment.....73-77, 118-119

Lab and pathological services.....34

Magnetic resonance imaging (MRI)...34, 66-67

Mail Service Pharmacy.....83-86

Mammograms.....35-36, 66-67

Maternity.....22, 38-39, 65-67, 93

Medicaid.....104

Medical necessity.....117-118

Medicare.....106-109, 112

Members.....7-9

Mental Health/Substance use disorder...118-119

Never Events.....5-6

Newborn.....8-9, 38-39, 66-67

No Surprises Act.....27-28

No-fault.....103

Non-FEHB Benefits.....95

Nurse.....92-93

Observation care.....119

Obstetrical care.....38-39, 93

Ocular injury.....45-46

Office visits.....32-51

Oral and maxillofacial surgery.....55-56

Orthopedic devices.....46-47

Ostomy and catheter supplies

Out-of-pocket expenses.....24-28

Overseas claims.....23, 98

Oxygen.....32-51, 64-69

Pap test.....34, 66-67

Physical and occupational therapies.....44

Physical examinations.....96

Physical therapy.....44

Physician.....12

Plan allowance.....119-120

Post-service claims.....98, 120

Pre-service claims.....120

Preauthorization.....19-20, 120

Precertification.....120

Preferred Provider.....15

Prescription Drugs.....78-89

Preventive care.....35-38, 87-88

Adult.....35-36

Children.....36-38, 87-88, 95

Primary care provider.....120

Private room.....18-19, 65-66

Prosthetic devices.....46-47, 53

Psychologist.....44

Radiation therapy.....42-43, 66-67

Radiology/Imaging.....20, 34, 66-67

Renal dialysis.....42-43, 47-49

Room and board.....65-66

Second surgical opinions.....33

Skilled nursing care facility.....68

Sleep Studies.....20, 34

Social worker.....73-74

Sound natural tooth.....120

Specialty drugs.....78-89

Speech therapy.....20, 96

Splints.....65-66

Sterilization procedures.....40

Subrogation.....121

Substance use disorder.....73-77, 118-119

Summary of benefits.....123-126

Surgery.....52-69, 121

Anesthesia.....63, 66-67

Bariatric.....20, 53

Oral.....55-56

Outpatient.....66-67, 76

Reconstructive.....53-55

Take-home items.....65-66

Telehealth.....33, 76

Temporary Continuation of Coverage (TCC).....11

Temporomandibular Joints.....55-56

Tobacco cessation.....51, 87-88

Transitional care.....96

Transplants (organ/tissue).....56-63

Treatment therapies.....42-43, 66-67

Urgent care claims.....21, 121

Urgent care facility.....71

Us/We.....121

Vision.....45-46, 95

Vision services.....45-46

Contact lenses/Eyeglasses.....45-46

Eye examinations.....95

Vitamins.....87-89

Well-child care.....36-38, 87-88

Wheelchair.....47-49, 69

Wigs.....47-49

Workers' Compensation.....104-105

X-rays.....34, 50, 55-56, 66-67, 90

Summary of Benefits for the Elevate Plus Option of the Government Employees Health Association, Inc. 2026

Do not rely on this chart alone. This is a summary. All benefits are subject to the definitions, limitations, and exclusions in this brochure. Before making a final decision, please read this FEHB brochure. You can also obtain a copy of our Summary of Benefits and Coverage as required by the Affordable Care Act at geha.com/Plan-Summaries. On this page we summarize specific expenses we cover; for more detail, look inside.

If you want to enroll or change your enrollment in this Plan, be sure to put the correct enrollment code from the cover on your enrollment form.

Elevate Plus does not offer out-of-network benefits except in cases of emergency.

Below, an asterisk (*) means item is subject to the \$200 Self Only or \$400 Self Plus One or Self and Family calendar year deductible when you use in-network providers.

Elevate Plus Benefits	You pay	Page
Medical services provided by physicians: Diagnostic and treatment services provided in the office	In-network: Nothing for preventive care; \$30 copay primary care provider; \$50 copay specialist	35
Services provided by a hospital: Inpatient	In-network: 20%* of the Plan allowance	67
Services provided by a hospital: Outpatient	In-network: 20%* of the Plan allowance	68
Emergency benefits: Medical Emergency	In-network: 25%* of the Plan allowance Out-of-network: Emergency care is paid at the in-network level	73
Mental health and substance use disorder treatment:	Regular cost-sharing	75
Prescription drugs:	In-network benefits ONLY	
Retail pharmacy	Network pharmacy: - Generic: Member pays lesser of \$15 or pharmacy's usual and customary cost for 30-day supply - Preferred Brand: Member pays lesser of \$100 or pharmacy's usual and customary cost for 30-day supply - Non-Preferred Brand: Member pays 50% of Plan allowance for 30-day supply	86
Mail order	Mail order network: - Generic: Member pays lesser of \$20 or the cost of the drug for 90-day supply - Preferred Brand: Member pays lesser of \$200 or the cost of the drug for 90-day supply - Non-Preferred Brand: Member pays 50% of Plan allowance for 90-day supply	86
Dental care:	Accidental Dental coverage only. Services are paid at regular medical Plan benefits.	93

Elevate Plus Benefits	You pay	Page
Protection against catastrophic costs (your catastrophic protection out-of-pocket maximum):	Nothing after \$7,000 Self Only (\$14,000 Self Plus One or Self and Family) per year for in-network providers.	28

Summary of Benefits for the Elevate Option of the Government Employees Health Association, Inc. 2026

Do not rely on this chart alone. This is a summary. All benefits are subject to the definitions, limitations, and exclusions in this brochure. Before making a final decision, please read this FEHB brochure. You can also obtain a copy of our Summary of Benefits and Coverage as required by the Affordable Care Act at geha.com/Plan-Summaries. On this page we summarize specific expenses we cover; for more detail, look inside.

If you want to enroll or change your enrollment in this Plan, be sure to put the correct enrollment code from the cover on your enrollment form.

Below, an asterisk (*) means item is subject to the \$750 Self Only or \$1,500 Self Plus One or Self and Family calendar year deductible when you use in-network providers; or subject to the \$2,000 Self Only or \$4,000 Self Plus One or Self and Family calendar year deductible when you use out-of-network providers. And, for most out-of-network services, you pay 50% of the Plan allowance plus any difference between the Plan allowance and the billed amount.

Elevate Benefits	You pay	Page
Medical services provided by physicians: Diagnostic and treatment services provided in the office	In-network: Nothing for preventive care; \$10 copay primary care provider; \$30 copay specialist	35
Services provided by a hospital: Inpatient	In-network: 25%* of the Plan allowance	67
Services provided by a hospital: Outpatient	In-network: 25%* of the Plan allowance	68
Emergency benefits: Medical Emergency	In-network: 35%* of the Plan allowance	73
Mental health and substance use disorder treatment:	Regular cost-sharing	75
Prescription drugs:	In-network benefits ONLY	
Prescription drugs: Retail pharmacy	Network pharmacy: • Generic: Member pays lesser of \$10 or pharmacy's usual and customary cost for 30-day supply • Preferred Brand: Member pays 50% of Plan allowance up to a maximum of \$500 for 30-day supply • Non-Preferred Brand: Member pays 100% of all charges	86
Prescription drugs: Mail order	Not available	86
Dental care:	Accidental Dental coverage only. Services are paid at regular medical Plan benefits.	93
Protection against catastrophic costs (your catastrophic protection out-of-pocket maximum):	Nothing after \$10,600 Self Only (\$21,200 Self Plus One or Self and Family) per year for in-network providers. Nothing after \$17,000 Self Only (\$34,000 Self Plus One or Self and Family) per year for out-of-network providers.	28

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2026 Rate Information for Government Employees Health Association, Inc. Benefit Plan

To compare your FEHB health plan options, please go to www.opm.gov/fehbcompare.

To review premium rates for all FEHB health plan options please go to www.opm.gov/FEHBpremiums or www.opm.gov/Tribalpremium.

Premiums for Tribal employees are shown under the Monthly Premium Rate column. The amount shown under employee pay is the maximum you will pay. Your Tribal employer may choose to pay a higher portion of your premium. Please contact your Tribal Benefits Officer for exact rates.

Type of Enrollment	Enrollment Code	Premium Rate			
		Biweekly		Monthly	
		Gov't Share	Your Share	Gov't Share	Your Share
Elevate Plus Option- Self Only	251	\$324.76	\$205.13	\$703.65	\$444.45
Elevate Plus Option- Self Plus One	253	\$711.17	\$449.58	\$1,540.87	\$974.09
Elevate Plus Option- Self and Family	252	\$778.03	\$496.12	\$1,685.73	\$1,074.93
Elevate Option-Self Only	254	\$233.77	\$77.92	\$506.50	\$168.83
Elevate Option-Self Plus One	256	\$563.96	\$187.99	\$1,221.92	\$407.31
Elevate Option-Self and Family	255	\$686.57	\$228.85	\$1,487.56	\$495.85